

A N E V I D E N T I A R Y R E P O R T

Systematic Exclusion

Indigenous Disability in Alberta

*Everything I am doing is so people do not feel
alone or unseen or unheard.*

*It gives them the grounds to push back —
from sourced research —
to turn a quiet murmur into a booming proclamation
no one can ignore.*

You are not alone. We stand together.

Compiled by The Alberta Disability System Breakdown
St. Albert, Alberta
April 2026

Preface

A note from the compiler

I am an AISH recipient. I am not Indigenous. I am a single mother of three children in St. Albert, Alberta, and I live within the same provincial disability support system that is the subject of this document.

Since April 2026, I have been building a sourced evidentiary record on the mandatory transition from the Assured Income for the Severely Handicapped (AISH) program to the Alberta Disability Assistance Program (ADAP), which takes effect July 1, 2026. That work now spans more than fifty reports, formal submissions to the Premier of Alberta, federal ministers, and the United Nations Special Rapporteur on the Rights of Persons with Disabilities, and a growing community of Albertans who have joined together to document what is happening to disabled people in this province.

This report is different.

What is happening to disabled Albertans under Bill 12 is not happening to every community equally. For Indigenous Albertans with disabilities, the harm is layered on top of decades of jurisdictional failure, broken treaty obligations, documented systemic racism in the healthcare system, and the ongoing consequences of colonial policy. The current advocacy landscape — including much of my own work to date — has not adequately addressed those layers.

This document is an attempt to correct that gap in the record.

What this document is

This is a sourced evidentiary report. Every claim it makes about Indigenous experience in Alberta is cited to Indigenous organizations, Indigenous-led research, Treaty bodies, federal court rulings, Statistics Canada data, Alberta Health Ministry reporting, and peer-reviewed publications. Where primary Indigenous voices exist on these issues — and they do, extensively — they are cited, not paraphrased. Where they do not yet exist on a given question, that absence is named as part of the evidence.

This document is not written to speak for Indigenous Albertans. It is written to make sure that the evidentiary record on the ADAP transition, and on the provincial disability support system more broadly, does not continue forward as if Indigenous disabled people do not exist within it.

What this document is not

It is not a replacement for Indigenous-led advocacy. It is not a comprehensive account of Indigenous disability experience in Alberta — that account can only be written by Indigenous Albertans themselves, and several organizations are doing that work. It is not an attempt to claim authority over narratives that are not mine.

It is a compilation. It is a witness. It is a contribution to a public record that has been built selectively and that currently treats the most structurally disadvantaged disabled Albertans as if they were a footnote to a conversation about the middle of the population.

They are not a footnote. The evidence in the pages that follow establishes that the sharpest edge of every harm documented in the broader campaign — mortality, incarceration, housing, healthcare access, jurisdictional abandonment — falls hardest on Indigenous Albertans. Any honest account of the ADAP transition has to begin from that reality.

Why this document exists

Because a quiet murmur is what happens when people are left to carry the weight alone. A booming proclamation is what happens when the evidence is assembled, the sources are named, and the record is made unignorable.

This document is assembled so that Indigenous Albertans with disabilities who encounter it — and the advocates, legal workers, policy staff, journalists, and community members who work alongside them — have one more piece of the record they can point to. Not as permission to speak. They already have that. As reinforcement, where reinforcement is useful. As citation, where citation makes the pushback hold.

The sources in this report are public. They were always public. What this document does is gather them in one place, with a consistent analytical frame, so that the pattern they form together is visible on a single reading.

You are not alone.

What is happening is seen.

We stand together.

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Sources and citations are embedded throughout. Every numerical claim is attributed to a primary source. Public domain data, public government documents, and peer-reviewed research form the evidentiary base.

SECTION 1

Historical Context

Disability, colonization, and the systems we inherited

The disability rates among Indigenous Albertans today are not an accident of nature. They are the measurable consequence of a century of Canadian policy — policy that operated with full knowledge of the harm it was causing, policy whose architects were warned by their own government officials, and policy whose institutional descendants still shape how disabled Indigenous Albertans interact with provincial supports in 2026.

This section documents three overlapping systems that produced the current demographic baseline: residential schools, Indian Hospitals, and the Sixties Scoop. Each operated independently. Each harmed health. Each created disability where disability did not previously exist. And each fed the next — survivors of one system were often delivered directly into the next, and the health consequences compounded across generations.

Any honest analysis of the ADAP transition begins here. Because the Indigenous disabled Albertans who will be affected by the July 1, 2026 transition are, in many cases, the direct biological and cultural inheritors of these policies. A program that treats them as if that history did not happen is a program that is not prepared to serve them.

Residential schools as disability-creating institutions

Between 1831 and 1996, Canada operated a network of residential schools that removed more than 150,000 First Nations, Métis, and Inuit children from their families and communities. The Truth and Reconciliation Commission of Canada (TRC), which concluded its six-year investigation in 2015, collected testimony from over 6,500 survivors and witnesses and reviewed more than five million federal records before releasing its six-volume final report and 94 Calls to Action.

The TRC's conclusions on health are not ambiguous. The schools were “sites of significant harm” to Indigenous children's physical, emotional, and mental well-being. Death rates were documented at levels that, had they occurred in non-Indigenous institutions, would have triggered immediate public outrage — and in some cases were documented precisely because of such outrage, and then systematically ignored by the federal government.

In 1907, Dr. Peter Bryce, Chief Medical Officer for the Department of Indian Affairs, delivered a report on western Canadian residential schools to his superiors. Bryce found that approximately 25 percent of children who had attended the schools he inspected had died, primarily from tuberculosis. At one school in southern Saskatchewan, the mortality rate was 69 percent. Bryce

identified the causes: overcrowding, poor ventilation, inadequate isolation of infected children, malnutrition that compromised immune function, and minimal medical care.

His recommendations for corrective action were deemed too costly. Death rate documentation by the Department of Indian Affairs was discontinued in 1917. Bryce was forced out of his position. The conditions he documented continued.

“The conditions in the schools themselves were the leading contributor to the often-shocking death rates among the students.”

The health impacts on survivors who lived did not end when the schools closed. Peer-reviewed research published in the International Journal of Environmental Research and Public Health in 2022 documents direct statistical relationships between residential school attendance — both by individuals and by their parents and grandparents — and elevated rates of depression, suicide ideation, substance use disorder, chronic disease, and disability into the present day.

Malnutrition during development produces measurable lifelong effects on metabolic health, bone density, cognitive development, and immune function. The TRC found that systematic hunger in the schools was documented in federal nutrition inspections as early as the 1940s and was met with administrative indifference.

Residential schools did not incidentally harm health. They produced disability as a structural outcome. Many Indigenous Albertans now receiving AISH, or now being considered for ADAP, carry bodies whose conditions trace directly to policies their parents and grandparents survived — and to policies their own generation survived, because residential schools did not fully close in Canada until 1996.

Indian Hospitals: a parallel system of segregated care

While residential schools were operating, a second federal system was running in parallel: the Indian Hospitals. According to historian Maureen Lux, whose 2016 book *Separate Beds: A History of Indian Hospitals in Canada, 1920s–1980s* is the foundational academic account, at least 22 such institutions operated across Canada by 1960, with 29 named in the federal government’s own records between 1945 and 1981. They were funded at roughly two-thirds the rate of community hospitals serving non-Indigenous patients. They were understaffed, overcrowded, and — in Lux’s documentation — “rife with coercion and medical experimentation.”

Two of these hospitals operated in Alberta: the Charles Camsell Indian Hospital in Edmonton, and the Blackfoot Hospital at Cardston. Survivors of both institutions are still living today. Some are AISH recipients.

The original justification for the system was tuberculosis isolation. The practical function was broader. Indian Hospitals treated maternity, pediatric, and general medical cases — not

because the population served had unusual disease patterns, but because admitting Indigenous patients to community hospitals had become administratively inconvenient and socially unwelcome. The hospitals were, in Lux's characterization, a way of ensuring "that other Canadians need not share access to modern hospitals with Aboriginal patients."

The medical consequences are documented in survivor testimony collected by historian Laurie Meijer Drees in *Healing Histories: Stories from Canada's Indian Hospitals* (2013) and in the evidentiary record of the 2018 class action lawsuit filed against the federal government. Indigenous patients — including newborns — received invasive surgical interventions for tuberculosis that were already considered outdated for non-Indigenous patients. Multiple ribs were routinely removed. Lungs were surgically collapsed and, in some cases, removed entirely. Children were kept in hospital for years, separated from families, often far from home, under medical staff who did not share their language or culture. Medical experimentation — including BCG vaccine trials — was conducted in the hospitals without informed consent.

A generation of Indigenous Albertans left these institutions with acquired disabilities that did not exist before admission. Those disabilities persisted across the remainder of their lives. Where those individuals had children, and those children had children, the compounding effects of institutional trauma and physical injury extended across two and three generations.

"Indian hospitals ... are rather like nodes in a larger web of incarceration, segregation, marginalization." — Maureen Lux, Separate Beds

The Sixties Scoop and the continued disability pipeline

In 1951, an amendment to the Indian Act transferred jurisdiction over Indigenous child welfare from federal to provincial governments. Residential schools were beginning to close. The institutional infrastructure that had relied on the forced removal of Indigenous children did not end — it was reorganized.

Between the late 1950s and the mid-1980s, provincial child welfare systems removed an estimated 20,000 Indigenous children from their families and placed the majority into non-Indigenous foster and adoptive homes. This period, known as the Sixties Scoop, has since been the subject of multiple class-action lawsuits, including *Brown v. Canada*, which produced an \$800 million federal settlement in 2017.

In his 1985 report *No Quiet Place*, Manitoba Justice Edwin Kimelman reviewed Indigenous child apprehension and concluded: "cultural genocide has taken place in a systematic, routine manner." Kimelman made 109 recommendations, several of which remain unimplemented.

The health and disability consequences of the Sixties Scoop were documented in the Ontario Superior Court's ruling in *Brown v. Canada* (2017), which found: "The loss of Aboriginal identity resulted in psychiatric disorders, substance abuse, unemployment, violence and numerous suicides." A 2025 Statistics Canada study on socioeconomic and health outcomes among

Indigenous adults who were under government care as children found statistically significant associations between childhood government care and adult mental health issues, disability, homelessness, and economic instability.

The removal of children from their families and communities did not end with the Sixties Scoop. According to the 2021 Census, 54 percent of foster children under age 15 in Canada are Indigenous — despite Indigenous children making up only 8 percent of the child population. In Saskatchewan as of 2019, that figure reached 86 percent. The contemporary child welfare system is producing, in real time, the same outcome measures that the Sixties Scoop and residential schools produced before it: separation-driven disability, traumatic health consequences, and a direct pipeline from childhood state intervention to adulthood disability support need.

The compounding of three systems

Residential schools, Indian Hospitals, and the child welfare system were not three separate chapters of Canadian history that happened to involve Indigenous people. They were three interlocking mechanisms of the same colonial apparatus, and they frequently delivered the same individuals from one system directly into the next.

A child in a residential school who contracted tuberculosis was transferred to an Indian Hospital. A child who survived an Indian Hospital with acquired disabilities, or who returned to a family already fractured by residential school attendance, was more likely to be apprehended into the child welfare system. A child apprehended into the child welfare system and separated from family, land, and language was more likely to experience the adult health outcomes that produce eligibility for disability supports — including AISH.

The McQuaid et al. study published in 2022 examined this compounding directly. Its finding: familial residential school attendance, childhood removal into the child welfare system, and adult mental health outcomes each account for unique and additive statistical variance. The systems compound. The harm compounds. The disability produced compounds.

For Indigenous Albertans now on AISH, this is not academic history. For many, the AISH caseload reflects bodies carrying the accumulated weight of three overlapping institutional harms — harms that were documented at the time, denied at the time, and continued anyway. The transition to ADAP is being applied to that population without acknowledgment of that history, and without structural accommodation for the systemic causes of their disability status.

What this means for the current moment

The Truth and Reconciliation Commission's final report, released in 2015, made 94 Calls to Action. Seven of those Calls address health directly (Calls 18 through 24). A full accounting of

those Calls — what was promised, what has been delivered, and what remains outstanding — appears in Section 13 of this document.

For the purpose of establishing historical context for the ADAP transition, the point is narrower: the provincial government of Alberta is proceeding with a mandatory reclassification of disability supports on July 1, 2026, in a policy environment where the federal acknowledgment of historical harm has been made, the documentation of current harm is extensive, and the structural accommodation for Indigenous disabled people remains absent from the transition framework.

Section 2 addresses what that accumulated history is producing as a present-day statistical reality — specifically, the 19-year life expectancy gap between First Nations and non-Indigenous Albertans, and what that gap means for a program whose design assumes a population that will age into its benefits structure.

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SECTION 2

The 19-Year Mortality Gap

Alberta's widest documented life expectancy disparity, and what it means for a program designed around ageing into benefits

This section establishes a single finding and traces its consequences. The finding is that in 2023, the average life expectancy at birth for First Nations people in Alberta was 62.81 years. In the same year, for non-First Nations people in Alberta, it was 81.88 years. The difference is 19.07 years. It is the widest documented life expectancy gap between First Nations and non-First Nations Albertans on record.

Every subsequent section of this report, and every aspect of the July 1, 2026 ADAP transition, has to be understood against that finding. A provincial disability program being redesigned around the premise that recipients will age into seniors' benefits at 65 is a program whose structural logic does not apply to the First Nations recipients whose average lifespan ends before that point.

Alberta First Nations recipients do not, on average, age out of AISH. They die before it."

The figures

Current state, 2023

The following data points are drawn from the Government of Alberta's own 2023 figures and from peer-reviewed research published in The Lancet Regional Health – Americas in March 2025 by researchers working with First Nations health managers in Alberta:

- First Nations life expectancy at birth, Alberta, 2023: 62.81 years
- Non-First Nations life expectancy at birth, Alberta, 2023: 81.88 years
- Gap: 19.07 years
- First Nations men, Alberta, 2021: approximately 60 years
- First Nations women, Alberta, 2021: approximately 66 years
- Non-First Nations men, Alberta, 2021: approximately 79 years
- Non-First Nations women, Alberta, 2021: approximately 84 years

For context, a life expectancy of 60 years for Alberta First Nations men in 2021 is comparable to the national life expectancy of men in Haiti and Bolivia. The Alberta Indigenous Primary Health

Care Advisory Panel, in its 2022 report, characterized the Alberta First Nations life expectancy as “in alignment with countries like Niger, Angola and Afghanistan and similar to life expectancy witnessed in Canada during the Great Depression in the 1930s.” That comparison is drawn from a Government of Alberta-affiliated advisory body, not from advocacy research.

The trajectory: the gap is widening, not closing

The 19-year gap is not a static disparity. It is a disparity that has been actively growing over the past quarter century, during precisely the period in which federal and provincial governments have publicly committed to closing it.

- 1974–1978: The documented gap between Indigenous life expectancy in Alberta and the total provincial population was 15 years. (Health and Welfare Canada, 1981 study.)
- 1999: The gap had narrowed to approximately 7 years. Life expectancy for First Nations people in Alberta ranged from 69.9 to 72.6 years during the 1999–2019 period, against non-First Nations life expectancy of 79.6 to 82.8 years.
- 2019: First Nations life expectancy in Alberta was 70.4 years. Non-First Nations life expectancy was 82.8 years. Gap: 12.4 years.
- 2021: First Nations life expectancy dropped to 63.1 years. Non-First Nations life expectancy was 81.4 years. Gap: 18.3 years.
- 2022: First Nations life expectancy recovered slightly to 64.2 years. Gap: 17 years.
- 2023: First Nations life expectancy 62.81 years. Non-First Nations 81.88 years. Gap: 19.07 years — the widest on record.

Read in sequence: the gap was narrowing through the 1980s and 1990s, stabilized in the early 2000s, began to widen in the mid-2010s, collapsed precipitously during the COVID-19 pandemic, and has continued to widen since. The peer-reviewed analysis in *The Lancet Regional Health – Americas* attributes this trajectory to a combination of pre-existing systemic inequities — delayed access to care, housing instability, contaminated water, chronic disease prevalence — exacerbated by two acute crises: COVID-19 and the toxic drug poisoning epidemic.

COVID-19: the asymmetric shock

Between 2019 and 2021, life expectancy for First Nations people in Alberta fell by 7.3 years. In the same two-year period, life expectancy for non-First Nations Albertans fell by 1.4 years. The First Nations population experienced more than five times the life expectancy loss of the non-First Nations population during the pandemic.

The peer-reviewed *Lancet* analysis documented that during the pandemic, the mortality rate for young adult First Nations women in Alberta was approximately eleven times higher than for non-

First Nations women of the same age. The 20–50 age groups experienced the highest relative mortality rate ratios. These are not elderly deaths. They are working-age deaths, in a population that was already dying earlier than the provincial average before the pandemic began.

In January 2021, on-reserve COVID-19 case rates were 40 percent higher than the overall provincial population. The reasons identified in the peer-reviewed literature include: overcrowded housing (addressed in Section 4), delayed access to health care (addressed in Section 7), pre-existing chronic disease from environmental and social determinants (addressed in Sections 5 and 6), and contaminated water supplies in some communities (addressed in Section 6). Each of these vulnerabilities was documented before 2020. The pandemic did not create them. It exposed them.

The toxic drug crisis: a 366 percent increase

Between 2016 and 2022, unintentional opioid poisoning deaths among First Nations Albertans increased by 366 percent. In 2016, 188 First Nations Albertans died from unintentional injury of all kinds (including falls, traffic accidents, and poisonings). In 2022, 344 First Nations Albertans died from unintentional opioid poisoning alone.

The rate disparity, in the 2024 Alberta Health surveillance data, is stark:

- 2016–2019: First Nations opioid death rate ranged from 44 to 70 per 100,000 person-years. Non-First Nations: 11 to 16 per 100,000 person-years. A ratio of roughly four-to-one.
- 2020: First Nations 148 per 100,000 vs non-First Nations 20 per 100,000. Ratio: 7.4 to 1.
- 2021: First Nations 224 per 100,000 vs non-First Nations 26 per 100,000. Ratio: 8.6 to 1.
- 2022: First Nations 205 per 100,000 vs non-First Nations 24 per 100,000. Ratio: 8.5 to 1.

The disparity is not attributable to different patterns of substance use. It is attributable primarily to the toxicity of the unregulated drug supply, combined with the absence of harm reduction infrastructure in the communities where First Nations Albertans live, and the closure of specific treatment and harm reduction centres in Alberta under successive provincial governments.

The age distribution of these deaths matters. The largest proportion are among people aged 25 to 44 — the same age band that carries the heaviest work and caregiving responsibilities in First Nations communities, the same age band in which children are being raised, and the same age band that, in the AISH program, represents the recipients most likely to still be in the early decades of an anticipated benefit relationship.

What this means for AISH, and for ADAP

The structural premise of AISH

The Assured Income for the Severely Handicapped program, established in 1979, was designed around a specific demographic assumption. Recipients enter the program between age 18 and age 65, after being medically certified as having a severe and permanent disability. Benefits continue until the recipient either (a) becomes eligible for Old Age Security at age 65, at which point they transition to federal senior benefits supplemented by the Alberta Seniors Benefit, or (b) dies.

The federal Old Age Security pension, the Guaranteed Income Supplement, and the Alberta Seniors Benefit together form what is, for most AISH recipients, a higher monthly income than AISH itself. For a single AISH recipient at the current \$1,940 monthly benefit, the combined federal and provincial senior benefits can total approximately \$2,130 per month. The AISH-to-senior-benefits transition, despite the administrative burden it creates, is generally a financial improvement.

The demographic assumption built into AISH is therefore: most recipients will age into the seniors' benefit system and receive more income as seniors than they received as AISH recipients.

The assumption does not apply to Alberta First Nations

In 2023, the average life expectancy at birth for First Nations people in Alberta was 62.81 years. The AISH program transitions recipients out at age 65. The mathematics are straightforward: on average, Alberta First Nations AISH recipients die 2.19 years before they would have aged out of the program.

Put another way: the federal and provincial senior benefits that compensate for lower AISH payments over a recipient's lifetime, and that make the AISH-to-senior-benefits transition financially acceptable, do not apply to the First Nations recipient population on average. The compensating mechanism is never triggered. The lifetime cost of being on AISH, for First Nations Albertans, is borne entirely within the AISH payment structure itself — because there is no average post-65 phase of life during which the senior benefit structure corrects for it.

The 19-year gap is not only a public health statistic. It is a structural fact about how a disability benefit program operates, and about which populations that program serves adequately versus inadequately. A program built around the assumption of a 65-year-plus lifespan does not serve a population with an average lifespan of 62.8 years. It serves that population on paper. In practice, it under-serves them by design.

The ADAP transition makes this worse

The July 1, 2026 transition from AISH to the Alberta Disability Assistance Program (ADAP) introduces two additional dimensions to this problem.

First, the monthly benefit level under ADAP is \$1,740 — a \$200 reduction from the current AISH rate of \$1,940. A transition benefit maintains the current rate for existing AISH recipients until

December 31, 2027, after which the full \$200 reduction takes effect. For new applicants on or after July 1, 2026, the \$1,740 rate applies immediately. For a First Nations recipient with an average life expectancy of 62.81 years, that \$200 per month over the remaining years of AISH eligibility represents a material reduction in lifetime benefits received — not offset by any corresponding senior benefit, because most of those recipients will not reach the senior benefit window.

Second, ADAP is structured around a premise of employment readiness. Recipients who transition to ADAP, or who are newly enrolled under ADAP, are expected to engage with employment services designed to return them to some form of paid work. For a population with an average life expectancy below the program's own age-out threshold — and with documented rates of chronic disease, environmental exposure, traumatic injury, and mental health burden rooted in the historical context of Section 1 and the contamination reality of Section 5 — the premise of progressive return to employment is not a medical prediction. It is an administrative convenience.

Put directly: ADAP asks First Nations Albertans, whose lifespans place them statistically below the age at which the program was designed to end, to demonstrate their capacity to earn income in a labour market from which they have been systemically excluded for centuries, while living in an environment that has been actively contaminated by industrial and governmental choice. The ask is not realistic. The ask is also not the point. The point of the ask is the administrative filter it creates — the population that cannot meet the demonstration requirement is the population that, under ADAP, sees its benefit reduced and its appeal rights stripped.

What this establishes for the remainder of the report

The remaining sections of this report — jurisdictional fracture, housing, water, dual healthcare systems, Jordan's Principle, child welfare, justice system, missing and murdered women, consultation failure, Sovereignty Act conflict, TRC Calls to Action, and the July 1 transition itself — all operate within the demographic reality established here.

Each subsequent section is a specific documentation of one of the structural drivers of the 19-year gap. Each of those drivers has, in turn, been documented for decades. Each has been the subject of federal commissions, provincial reports, peer-reviewed research, and Indigenous-led advocacy. None has been resolved. The gap has widened during the period of documentation.

This is the structural reality into which the ADAP transition is being introduced, on July 1, 2026, without Indigenous consultation meeting UNDRIP standards, without structural accommodation for the demographic profile of the affected population, and without acknowledgement that the program's founding assumptions do not apply to a significant subset of its recipients.

A program designed around ageing into benefits does not, in operational reality, serve a population that dies before the ageing-in date."

Section 3 addresses the jurisdictional fracture — the federal-provincial-status-non-status split that determines which institutional system is responsible for which Indigenous person at which moment, and how that fracture itself produces the access delays and coverage gaps that contribute to the mortality profile documented here.

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SECTION 3

Jurisdictional Fracture

The constitutional architecture of Indigenous disability in Canada, the jurisdictional wasteland the Supreme Court named, and why the ADAP transition reproduces the pattern one more time

There is no hierarchy of rights. But there is a hierarchy of access. An Indigenous person with a disability in Canada navigates the distance between the two every time they need the system to function.”

This section documents the structural feature of Canadian federalism most directly responsible for the Indigenous disability access pattern: the jurisdictional fracture between federal and provincial authority, the Indigenous-specific categories that fall across that line, and the sustained practice — recognized by the Supreme Court of Canada in 2016 as a “jurisdictional wasteland” — of federal and provincial governments treating the population in the fracture as the other level’s responsibility.

The section is structured in four parts. First, the constitutional framework: section 91(24) of the Constitution Act 1867, the division of powers that places “Indians, and Lands reserved for the Indians” in federal jurisdiction while placing healthcare, education, child welfare, housing, and social assistance in provincial jurisdiction. Second, the Indigenous-specific categories that live across the fracture: status First Nations persons on reserve, status First Nations persons off reserve, non-status First Nations persons, Métis, and Inuit, each of whom faces a different set of rules. Third, the judicial and political history of the fracture: *Daniels v. Canada* (2016), *Jordan’s Principle*, and the Canadian Human Rights Tribunal’s repeated non-compliance findings. Fourth, the direct application to AISH and the July 1, 2026 ADAP transition: the reproduction of the same pattern in Alberta provincial disability policy, and the identification of Indigenous disabled Albertans as a specific sub-population of the broader “Alberta Disability Refugee” phenomenon documented elsewhere in this report series.

The central finding: the jurisdictional fracture is not a neutral administrative feature of Canadian federalism. It is a structural design whose documented outcome, across six decades of Supreme Court decisions, human rights tribunal orders, parliamentary motions, and federal–provincial transfer agreements, is that Indigenous people with complex needs are systematically placed in the administrative space between jurisdictions — and systematically harmed there.

Part 1: The constitutional architecture

Section 91(24) and the division of powers

The Constitution Act, 1867 — formerly the British North America Act — distributed legislative authority between the federal Parliament and the provincial legislatures. Section 91 enumerates federal powers. Section 92 enumerates provincial powers. Section 91(24) gives the federal Parliament exclusive legislative authority over “Indians, and Lands reserved for the Indians.” Section 92(7) gives the provinces authority over “the Establishment, Maintenance, and Management of Hospitals, Asylums, Charities, and Eleemosynary Institutions.” Section 92(13) gives the provinces authority over “Property and Civil Rights in the Province.” Section 93 gives the provinces authority over education.

The structural consequence is immediate. The population that section 91(24) assigns to federal jurisdiction — First Nations, and after *Daniels v. Canada* (2016), also Métis and non-status Indians — requires the services that sections 92 and 93 assign to provincial jurisdiction: healthcare, hospitals, social services, education, property law. The Constitution created, at the moment of confederation, a population whose identity sat with one level of government and whose day-to-day service needs sat with the other.

This is not an academic observation. It is the structural reality that has produced, and continues to produce, the access outcomes documented throughout this report. Jordan River Anderson died at age five in a Winnipeg hospital in 2005 not because the medical care he needed was unavailable — it was available in his home community in a specialized foster arrangement — but because the federal government and the Manitoba provincial government spent two years arguing about which of them was financially responsible for the home-based portion of his care. Neither moved. Jordan remained in the hospital. He died there. Section 91(24) versus section 92(7) killed him, in administrative terms. The constitutional architecture did not kill him directly — but the practice of two governments using the architecture to avoid responsibility did.

The Constitution created the fracture. Six generations of federal and provincial governments used it.”

The Indian Act and the categories it created

The Indian Act, first enacted in 1876 and since subject to numerous amendments, is the federal legislation that operationalizes section 91(24). The Act defines who is an “Indian” for federal purposes, establishes the reserve system, and prescribes the administrative apparatus — including band membership, status registration, and the federal agencies now consolidated under Indigenous Services Canada — through which federal responsibility is exercised.

The Act has produced, across 150 years, a series of categories with different rights, different service eligibilities, and different jurisdictional rules:

- Registered (status) First Nations persons: individuals entered in the Indian Register maintained by the federal government, entitled to Indian Act benefits, including tax exemptions for on-reserve income and property, and historically the population for which the federal government has most directly assumed service responsibility.

- Non-status First Nations persons: individuals of First Nations ancestry who, for reasons including historical enfranchisement, intermarriage provisions stripped from the Act by Bill C-31 in 1985, or complex lineage questions, are not entered in the Indian Register. Daniels v. Canada (2016) confirmed that this population falls under section 91(24) federal jurisdiction, but they do not receive Indian Act benefits.
- Métis persons: individuals of mixed European and Indigenous ancestry with ties to the historic Métis nation across the Prairie provinces. Daniels v. Canada confirmed Métis inclusion under section 91(24), but Métis persons have never received Indian Act benefits and historically have been administered through provincial frameworks.
- Inuit persons: determined to be “Indians” for section 91(24) purposes by the 1939 Supreme Court decision Re Eskimo, and administered through federal frameworks including the federal government’s agreements with the Inuvialuit, Nunavut, Nunavik, and Nunatsiavut land-claim organizations.

Each of these categories has different rules, different documentation requirements, different benefit structures, and different administrative points of contact. A disabled Indigenous person applying for provincial disability supports in Alberta must first be classified under one of these categories before any substantive assessment of their disability eligibility begins. The classification question itself is frequently contested, frequently requires proof the applicant does not possess, and frequently depends on the documentary inheritance of historical administrative practices the applicant had no hand in creating.

Part 2: Five populations, five sets of rules

The practical operation of jurisdictional fracture in Alberta is documented in the province’s own Income and Employment Supports Training and Health Benefits (IESTHB) Policy Manual. The Manual explicitly sets out the provincial position on First Nations service eligibility — a position that differs by status, by residence location, by treaty area, and by service type. The following subsection quotes and summarizes the Manual’s own language.

Status on reserve: federal responsibility, provincial disclaimer

The Alberta IESTHB Policy Manual states, under the section titled “First Nations People with Registered Status under the Indian Act”: “When a First Nation person is residing on reserve or ordinarily residing on reserve the Income Support Program responsibility lies with the federal government.” The provincial policy position is unambiguous: if the applicant is a status First Nations person ordinarily resident on reserve, Alberta disclaims responsibility. The federal government, through Indigenous Services Canada and the relevant First Nation government, is the service provider.

This applies to Income Support. The AISH program, which serves adults with severe and permanent disabilities, operates on a parallel but related structure. Status First Nations persons

ordinarily resident on reserve do not, as a matter of ordinary administrative practice, become AISH recipients — they are directed to the federally funded social assistance administered by their band. The structural result is that the AISH caseload of approximately 79,290 Albertans does not reflect the distribution of disability prevalence in the Alberta population. Status First Nations persons on reserve are functionally administered elsewhere.

Status off reserve: provincial responsibility with federal health carve-out

The IESTHB Manual continues: “According to the Administrative Reform Agreement, the province has agreed to fund and deliver the Income Support program to Registered Indians under the Indian Act living off-reserve in the Treaty 7 and 8 areas. The province also funds and delivers Income Support to [off-reserve status Indians in] the Treaty 6 area. The federal government continues to provide social assistance to persons on-reserve.”

Health benefits, however, remain federal. The Manual specifies that registered First Nations people do not receive provincial health benefits through the Income Support program “regardless of where they live since this is the responsibility of the federal government, through Health Canada.” The federal Non-Insured Health Benefits (NIHB) program, administered by Indigenous Services Canada, covers prescription drugs, medical transportation, dental, vision, and mental health services for status First Nations and recognized Inuit persons regardless of residence location.

The practical result: a status First Nations person off reserve in Alberta accesses their income support through the province (with its 24% DTC enrollment rate, its deposit-statement accessibility failures, its call-centre restructure, and its defunded advocacy organizations, all documented in Section 17) and their health coverage through the federal NIHB program (with its own eligibility rules, its own documentation requirements, its own administrative processes). These two systems do not share files, do not share adjudication, and do not share accountability. The recipient navigates both simultaneously.

Non-status First Nations persons and Métis: the Daniels population

Before the 2016 Daniels decision, non-status First Nations persons and Métis occupied what the Supreme Court would describe, in para. 13 of its judgment, as a “jurisdictional wasteland.” The federal government’s position, sustained across decades, was that these populations were not covered by section 91(24) and were therefore a provincial responsibility. Provincial governments, in practice, argued the reverse. Neither level of government organized services around their needs. The population received neither the federally-delivered services available to status First Nations nor the culturally-specific provincial services that might have been designed for them.

The Supreme Court, in its unanimous 2016 Daniels decision, declared that “Métis and non-status Indians are ‘Indians’” under section 91(24) of the Constitution Act, 1867. Justice Abella, writing for the Court, noted explicitly that the declaration was intended to end the “jurisdictional

tug-of-war” (para. 15) in which these groups had been “left wondering about where to turn for policy redress” and to provide “certainty and accountability” for a population that had experienced “denial of material benefits, significant disadvantage and negative consequences.”

The decision is foundational. But it does not, by itself, create federal services where none existed, and it does not compel provincial services to integrate with federal recognition. Ten years after Daniels, the operational reality for non-status First Nations and Métis Albertans seeking disability supports remains structurally similar to the pre-2016 situation: they apply to provincial programs, the provincial programs apply provincial rules, and the federal jurisdiction that Daniels confirmed remains largely theoretical in the context of day-to-day social assistance and disability benefit administration.

The Supreme Court ended the jurisdictional tug-of-war as a matter of constitutional law. It did not end the jurisdictional tug-of-war as a matter of who picks up the phone when a Métis AISH applicant needs a physician to complete a DTC form.”

Inuit persons: federal recognition, service geography

Inuit persons in Alberta are a smaller population, but the structural issues are not different. Federal recognition through the 1939 Re Eskimo decision, federal service delivery through Indigenous Services Canada and federal health coverage through NIHB, and provincial administration of provincially-delivered benefits (housing, AISH, FSCD, PDD) when the Inuit person resides in Alberta rather than in one of the four Inuit Nunangat regions. The Inuit person in Edmonton applying for AISH faces the same multi-jurisdictional adjudication challenge the status-off-reserve First Nations person faces, with the additional complication that the specific federal agencies most oriented to Inuit service delivery are geographically based elsewhere.

Part 3: Jordan’s Principle — the fracture’s most documented case

Jordan River Anderson

Jordan River Anderson was born in 1999 into Norway House Cree Nation in northern Manitoba. He had Carey-Fineman-Ziter syndrome, a rare neuromuscular disorder requiring extensive medical care. He was hospitalized from birth. In 2001, when Jordan was two years old, his medical team determined that his needs could be met in a specialized home setting in or near Winnipeg, with in-home care. The federal government (responsible for services to First Nations on reserve) and the Manitoba provincial government (responsible for services in the province off reserve) could not agree on which would fund the home-based care.

For more than two years, while the two governments continued to dispute financial responsibility, Jordan remained in the hospital. He was not discharged because the placement could not be arranged. The placement could not be arranged because the funding was not

agreed. The funding was not agreed because two governments were in jurisdictional dispute over which of them owed the financial obligation to a child who needed care neither of them was providing.

Jordan River Anderson died in the hospital in 2005 at age five. He never lived in a home. He had been born into a jurisdictional dispute, and he died inside it.

The principle and its promise

Jordan's death catalyzed First Nations advocacy and public awareness of the structural pattern his short life had exposed. On December 12, 2007, the House of Commons of Canada passed Private Member's Motion 296 unanimously. The motion, known thereafter as Jordan's Principle, stated that the federal government should adopt a child-first principle to resolve jurisdictional disputes involving the care of First Nations children. The principle's operative rule: when a First Nations child requires a service that is potentially available under more than one jurisdiction, the government of first contact must provide the service, with jurisdictional responsibility to be resolved between the governments afterward. The child is not to remain in the waiting room while the governments argue.

The motion passed with all-party support. The federal government announced initial funding of \$11 million. According to the 2016 Alanis Obomsawin documentary **We Can't Make the Same Mistake Twice**, the \$11 million initial fund was "never used" — a finding that itself requires pause. A parliamentary motion passed unanimously. A fund was allocated. The fund went unused. The principle was not implemented.

The Canadian Human Rights Tribunal proceedings

In 2007, the First Nations Child and Family Caring Society (led by Dr. Cindy Blackstock) and the Assembly of First Nations jointly filed a complaint with the Canadian Human Rights Tribunal alleging that Canada's underfunding of on-reserve child and family services, and its failure to implement Jordan's Principle, constituted systemic racial discrimination against First Nations children. The federal government contested the complaint for nine years. The Tribunal issued its substantive decision in January 2016, finding that the Government of Canada's implementation of Jordan's Principle was discriminatory and that the federal definition of the Principle had been "narrow" and improper.

The Tribunal ordered Canada to "cease applying its narrow definition of Jordan's Principle and to take measures to immediately implement the full meaning and scope of Jordan's Principle." The Tribunal has subsequently issued a sequence of non-compliance orders against Canada, documenting repeated failures to implement the 2016 decision in full. The 2017 rulings (2017 CHRT 14 and 2017 CHRT 35) found Canada had continued "its pattern of conduct and narrow focus with respect to Jordan's Principle." The Tribunal has, since 2016, specified and re-specified that Jordan's Principle applies to all First Nations children, that service must be based

on the child's needs rather than typical availability, and that administrative procedures must not delay service provision.

By late 2024, the documented backlog of unresolved Jordan's Principle requests had grown to approximately 129,000 cases, including 17,000 deemed urgent. Between July 2016 and September 2025, the federal government approved more than 10 million products, services, and supports under Jordan's Principle — a figure that demonstrates both the real scale of the Indigenous child service need and the scale of the backlog of what has not been approved.

The broader legal framework has produced settlement agreements of extraordinary size: a \$23.4 billion compensation agreement for First Nations children harmed by the on-reserve child and family services system, and a proposed \$47.8 billion long-term reform agreement. These figures are themselves evidence of the scale of the harm. Governments do not pay \$70+ billion in combined compensation and reform unless the harm they are responding to is massive, sustained, and indefensible.

Jordan's Principle as a case study in fracture-response-fracture

The structural pattern that Jordan's Principle was designed to resolve is documented in the sequence that produced it:

- Fracture: federal–provincial jurisdictional dispute over service responsibility.
- Death: a child dies in hospital while governments argue.
- Response: unanimous parliamentary motion, funding allocation.
- Fracture again: federal government implements the motion narrowly, fund goes unused, Principle functionally not applied.
- Tribunal response: CHRT 2016 finding of discrimination, sequence of non-compliance orders through 2025.
- Fracture again: 129,000 pending requests, 17,000 urgent, settlement reform continues to be contested, Principle continues to be implemented in tension with its own judicial elaboration.

At no point in this sequence has the underlying constitutional architecture — section 91(24) against sections 92 and 93 — been altered. The fracture that killed Jordan River Anderson in 2005 remains the fracture through which 129,000 requests are backlogged in 2024. The tribunal orders, the parliamentary motion, the multi-billion-dollar settlements, and the child-first principle itself have not closed the fracture. They have responded to it. The fracture has continued to produce the pattern that prompted the responses.

Part 4: The fracture and the ADAP transition

The Alberta Disability Refugee, Indigenous dimension

The Alberta Disability System Breakdown report series has documented, across nine provincial comparison reports and an extended Charter/Federal Jurisdiction analysis, the phenomenon of the “Alberta Disability Refugee”: disabled Albertans who would leave the province in response to the ADAP transition, the CDB clawback, the removal of appeal rights, and the absence of rent caps, but who cannot leave because the structural design of provincial disability income makes departure financially impossible. The term is not hyperbole. It describes a population functionally trapped by the interaction between Charter mobility rights under section 6 of the Canadian Charter of Rights and Freedoms, and a provincial program that does not transfer, does not bridge, and does not integrate with receiving-province disability programs.

For Indigenous disabled Albertans, the Alberta Disability Refugee phenomenon operates in addition to, and compounded by, the jurisdictional fracture documented in Parts 1–3 of this section. An Indigenous AISH recipient considering leaving Alberta in response to ADAP faces:

- The general structural cage: no bridge payments, no formal interprovincial transfer protocol, no grandfathered eligibility recognition, no relocation assistance.
- Additional Indigenous-specific dimensions: a receiving province that may administer First Nations services differently, a federal NIHB relationship that may or may not transfer seamlessly, a band or community relationship that may not be available in the receiving jurisdiction, a family and cultural support network geographically situated where they currently are.
- The federal-provincial fracture reproduced inside the move: the receiving province’s disability program will apply provincial rules, the receiving province’s First Nations administrative framework will apply federal rules, and the integration between the two in the new province may or may not be better than it was in Alberta.

The result is a population for whom the Charter right to mobility, already rendered functionally void by provincial program design for non-Indigenous AISH recipients, is voided twice over: once by the financial structure of the program, and once by the jurisdictional complexity the move produces.

Why ADAP reproduces the fracture, not resolves it

The Alberta Disability Assistance Program framework that takes effect July 1, 2026 contains no Indigenous-specific provisions. There is no carve-out for recipients whose service access is complicated by the federal–provincial fracture. There is no protocol for integration with Indigenous Services Canada, with the Non-Insured Health Benefits program, or with First Nations band governments. There is no recognition that the Medical Review Panel reassessment process, which will adjudicate which transitioned recipients remain eligible for AISH and which are consigned to ADAP at the reduced benefit level, is a process that operates on standards and documentary expectations more easily met by applicants with stable access

to the professional and documentary infrastructure that federal–provincial fracture historically denies Indigenous applicants.

This is not a failure specific to ADAP. It is a structural feature of provincial disability benefit design across Canada. But it matters specifically here because the July 1, 2026 transition will reassess approximately 79,290 current AISH recipients, and because the population most likely to be reclassified as “employment-ready” under ADAP’s employment-readiness premise is the population least able to contest the reclassification administratively. Indigenous recipients with complicated documentation, with no individual caseworker relationship (the caseworker model was eliminated province-wide in 2025), with limited access to physicians willing and able to complete the paperwork the Medical Review Panel will require, are over-represented in the population most vulnerable to this reclassification.

The ADAP transition does not create this vulnerability. The jurisdictional fracture, documented across 150 years of federal-provincial Indigenous policy, created it. ADAP will, on its current design, reproduce it.

What the Indigenous-specific reading of ADAP adds to the record

Sections 1 and 2 of this report establish the mortality and historical context. Section 17 establishes the federal–provincial clawback pattern for disability recognition instruments — DTC, RDSP, CDB, VISP. Section 3, read in conjunction with 17, establishes that the clawback pattern Section 17 documented for the non-Indigenous disabled Albertan population operates against a baseline of pre-existing Indigenous-specific jurisdictional fracture that has been producing the same kinds of outcomes for the Indigenous disabled population across every federal and provincial government for 150 years.

The non-Indigenous AISH recipient being reclassified under ADAP encounters, for the first time, the kind of administrative filtering that the Indigenous AISH recipient has encountered across every one of the federal and provincial programs they have ever needed. The Alberta Disability Refugee is, in that sense, the universalized version of a status-specific experience Indigenous disabled Albertans have had always. The ADAP transition is not a new kind of harm. It is a familiar kind of harm being newly extended to populations that previously did not experience it.

Indigenous disabled Albertans are not a separate case in the ADAP transition. They are the documented precedent. Everything the rest of the AISH community is about to experience, they have already experienced.”

What this section establishes for the remainder of the report

The constitutional architecture places Indigenous people in federal jurisdiction and the services they need in provincial jurisdiction. The 150-year operational practice of both levels of government has been to use the fracture to disclaim responsibility. Jordan’s Principle, the

Daniels decision, the Canadian Human Rights Tribunal proceedings, and the \$70+ billion in combined compensation and reform commitments are the accumulated cost of that practice — paid, belatedly, by the federal government for harms it could have prevented by consistently acknowledging federal jurisdiction in the first place.

The ADAP transition reproduces the fracture one more time. Not by invoking section 91(24) as a disclaimer — ADAP does not even reach that level of jurisdictional acknowledgment — but by administering provincial disability benefits on the assumption that a standardized reassessment process applies equally to populations whose service relationships with the state are structurally different and structurally disadvantaged.

Sections 4 through 15 of this report document specific manifestations of that structural disadvantage: housing, water, healthcare, Jordan's Principle in child services, child welfare, justice, missing and murdered women, consultation failure, Sovereignty Act conflict, TRC Calls to Action. Each is a concrete application of the constitutional architecture this section has described. Each is a documented pattern. None has been resolved. All continue into the July 1 transition.

This is the structural context into which ADAP is being introduced. Not a vacuum. Not a newly-discovered administrative challenge. The oldest pattern in Canadian federalism, reproduced one more time, at scale, with deadline.

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SECTION 4

Housing

The compounded housing crisis for Indigenous disabled Albertans: reserves underfunded by design, urban rents without cap, and the structural exclusion of disabled Indigenous people from both

A disabled Indigenous Albertan has two housing options: a reserve the federal government has underfunded for two decades, or a provincial rental market with no cap on how fast it prices them out. Both options fail. The choice between them is not a choice.”

This section documents the housing reality facing Indigenous disabled Albertans on either side of the jurisdictional fracture established in Section 3. The on-reserve reality: a federal housing system that the Auditor General of Canada has found, across four separate audits in 20 years, is failing to meet documented need. The off-reserve reality: an Alberta provincial rental market with no rent cap, average rents exceeding the entire AISH monthly benefit, community housing rent increased 63 percent by ministerial order in October 2025, and a waitlist at Edmonton's Civiqa of more than 10,000 households.

The section establishes three findings. First, that the federal government's failure to adequately fund on-reserve housing is not recent, not contested, and not attributable to fiscal constraint — it is a sustained policy outcome across multiple governments, documented repeatedly by the Auditor General, and one in which Alberta specifically has been underfunded by at least \$141 million since 2008. Second, that the Alberta provincial rental market, independently, produces a housing environment in which the AISH benefit is mathematically insufficient to cover basic shelter. Third, that the combination of these two failures — a disabled Indigenous Albertan facing neither a functional on-reserve housing system nor a functional off-reserve affordability environment — produces a structural homelessness risk that cannot be resolved through individual effort.

Part 1: The on-reserve reality — what the Auditor General has been saying for 20 years

The 2024 Auditor General report

On March 19, 2024, Auditor General Karen Hogan released her report on First Nations housing conditions. The report was the fourth formal audit the Office of the Auditor General has conducted on this file since 2003. Each prior audit had identified ongoing and insufficiently

addressed issues. The 2024 audit used, in effect, the bluntest language the office had yet deployed.

The core findings, in the Auditor General's own language:

- “Indigenous Services Canada and the Canada Mortgage and Housing Corporation have been mandated to work with First Nations to meet their housing needs by 2030. We found that 80% of these needs were still not met with 7 years left before 2030.”
- Overcrowding and mould — the two most documented health-impact housing problems on reserve — were “persistent” across communities, and the scale of the mould problem remained largely uncounted because federal monitoring has been inadequate.
- “Little progress” had been made despite twenty years of prior Auditor General reporting on the same file.

The Auditor General also identified a specific funding inequity with direct Alberta implications. The Canada Mortgage and Housing Corporation had continued, through 2023, to use 2001 Census data to determine per-region housing allocations. First Nations populations in Alberta, Saskatchewan, and Manitoba had grown substantially since 2001. The use of outdated data resulted in these provinces — where First Nations populations were larger than the formula recognized — being underfunded.

The documented underfunding figures:

- Alberta: \$141 million underfunded between 2008 and 2023
- Saskatchewan: \$72.92 million underfunded over the same period
- Manitoba: \$60 million underfunded over the same period
- Combined Prairie underfunding: \$273.92 million

In response to the audit, the CMHC informed the Office that a new funding formula would be negotiated by end of 2025. Internal documents obtained by APTN News indicated that the use of updated Census figures would increase allocations to Alberta, Saskatchewan, and Manitoba — but that, in the absence of additional total funding, this increase would be offset by corresponding decreases to Quebec, Ontario, British Columbia, and Atlantic First Nations. The documents also acknowledged that retroactive compensation to the underfunded Prairie regions was “extremely limited” absent new funding. As Chief Kelsey Jocko of Cold Lake First Nation in Alberta asked publicly: “How can the government let this happen? It’s under their watch. It’s just totally wrong. I believe we should be compensated. That’s a lot of money.”

Alberta’s First Nations were underfunded by \$141 million for fifteen years because the federal government used census data from 2001. The error was discovered. The money is not being returned. A new formula will adjust future allocations by reducing allocations to other regions. This is

not a plan to fix the problem. It is a plan to redistribute the existing shortfall.”

The scale of documented need

The Assembly of First Nations reports that 157,453 new homes are required nationally to address the First Nations housing crisis. The estimated cost is \$135.1 billion. The 2024 federal budget allocated \$918 million over five years to address First Nations, Inuit, and Métis housing and infrastructure gaps — approximately 0.7 percent of the figure the AFN identifies as required, and delivered over a longer period than would be necessary to meet the 2030 commitment. The Investigative Journalism Bureau, in an April 2024 analysis published in collaboration with the Toronto Star, calculated the total Indigenous infrastructure gap across Canada at \$349 billion — approximately twenty times Indigenous Services Canada’s full planned spending for 2024-25.

The specific housing conditions the AFN, the Auditor General, and peer-reviewed researchers have documented include:

- Over 25 percent of First Nations households experience overcrowding — a rate nearly seven times higher than non-Indigenous households in Canada.
- 19.7 percent of First Nations people (approximately 206,845 people) lived in dwellings requiring major repairs as of the 2021 Census. First Nations people with registered status were more likely to live in such dwellings than non-status First Nations people.
- Mould presence is documented as widespread, with endotoxin and airborne mould-fragment levels “much higher than those found in homes in non-Indigenous communities in Canada.” Peer-reviewed analysis of 100 homes in four First Nations in Northwestern Ontario (Frontiers in Environmental Health, 2025) found 12 homes with visible mould sufficient to produce airborne spores at levels known to harm human health, and confirmed that the majority of homes surveyed had conditions capable of progressing to hazardous states.
- Approximately 80.7 percent of the 985 Indigenous reserves across Canada have some flood exposure at the 100-year return interval. Reserves were historically located, as a matter of settlement-era federal policy, on land unwanted by settler populations — including floodplains.
- Inuit persons are 17 times more likely to die in fires than non-Indigenous Canadians. First Nations persons are approximately five times more likely. Métis persons are approximately two times more likely. There is no national fire protection code that mandates fire safety standards or enforcement on reserves. Every other Canadian jurisdiction — provinces, territories, federal installations including military bases and airports — has building and fire codes. Reserves do not.

Housing as the upstream cause of multiple disability outcomes

The section's analytical thesis is this: the on-reserve housing conditions documented above are themselves upstream causes of the disabilities Indigenous Albertans live with. Mould exposure produces respiratory illness, asthma, chronic pulmonary impairment, and in infants produces immune-system effects that persist across life. Overcrowding produces communicable disease transmission, tuberculosis, chronic stress, and trauma. Poor ventilation and CO₂ accumulation produce cognitive and neurological effects in children. Structural defects produce injury risk. Flood exposure produces displacement trauma, disrupted care, and repeated evacuations with compounding stress effects.

Section 1 of this report documented the historical production of Indigenous disability through residential schools, the Indian hospital system, the Sixties Scoop, and contamination exposure. Section 5 documented the environmental dimension. Section 4 completes the trio: the physical housing environment itself, as documented by the federal government's own Auditor General and peer-reviewed researchers, is an active and sustained producer of the disability profile the AISH program serves.

The federal government creates the housing conditions by failing to fund adequate shelter. The housing conditions create the disability. The provincial government then administers the disability benefit — and, as documented in Section 17, administers it in a way that minimizes federal recognition of disability through clawback, reassessment, and appeal-right stripping. The cycle closes. The population inside the cycle does not.

Part 2: The off-reserve reality — Alberta's uncapped rental market

No rent cap

Alberta is the only major province in Canada with no legislated limit on how much a landlord can increase rent. Landlords may increase rent by any amount, provided they give 90 days' written notice and wait at least 12 months since the last increase. There is no ceiling, no review process, and no mechanism by which a tenant can challenge the amount of an increase.

This environment is unique among large Canadian provinces. British Columbia's 2026 rent cap is 2.3 percent. Ontario's is 2.5 percent. Quebec and Manitoba and Nova Scotia all operate regulated rent frameworks. Alberta stands alone in permitting unlimited annual increases.

The consequences in 2024 and 2025 were visible in the data. Average listed rent prices in Alberta rose 17.5 percent year-over-year in 2024 — among the highest in Canada. Half of 75 Alberta municipalities surveyed recorded rent increases of 10 percent or more in 2024. One municipality recorded a 37 percent single-year increase. Average three-bedroom rent reached \$2,138 per month in 2024, up 21.3 percent year-over-year. Edmonton's average rent across all unit types rose approximately 26 percent between October 2021 and August 2025.

Against the 2026 AISH monthly benefit of \$1,940, average Alberta one-bedroom rent (\$1,500 to \$1,900 or higher) leaves a single AISH recipient as little as \$40 per month for food, utilities,

medication, and every other non-shelter cost. 86 percent of AISH recipients are single individuals. For a single parent with children requiring a three-bedroom unit, average rent of \$2,138 per month already exceeds the entire AISH payment.

The 63 percent community housing rent increase

Community housing — subsidized rental operated through provincial and municipal housing authorities — has historically been the only housing option where AISH recipients had meaningful protection. Under the framework in place through September 2025, \$735 of the AISH monthly benefit was exempted from the income calculation used to set community housing rent. This kept rent at approximately 17 percent of AISH income for recipients in community housing. Not comfortable. Survivable.

On May 15, 2025, Minister Jason Nixon signed Order in Council 2025-XXX removing the \$735 AISH exemption. Effective October 1, 2025, community housing rent for AISH recipients is calculated at 30 percent of total income with no AISH exemption. For a typical recipient, community housing rent increased from approximately \$349 per month to approximately \$570 per month — a 63 percent increase by ministerial order, with no legislative vote and no public consultation with disability advocates before signing.

The \$221 per month increase was imposed on the population of AISH recipients with the least capacity to absorb it — recipients in community housing, by definition, had already been unable to afford private market rent. The ministerial order did not change the underlying structural unaffordability. It transferred more of the shortfall onto recipient income.

Waitlists and access reality

The Cvida (Edmonton) affordable housing waitlist exceeded 10,000 households in 2025. Approximately 700 new applicants arrive per month. The list is not first-come, first-served; priority can change at any time, and no guaranteed wait time is published by the housing authority. The Calgary Housing waitlist exceeded 6,500 applicants as of 2024. The average property age of Calgary community housing stock was 40 years as of the same year.

Ground-floor accessible units suitable for families with children with elopement risk, mobility requirements, or autism spectrum-related safety needs are severely limited in the existing social housing stock. Apartment-style community housing presents significant safety challenges for families with non-verbal children, children with no stranger-danger awareness, children with active flight risk, or children with physical aggression related to sensory or communication differences. Calgary Housing's own guidance to applicants notes that restricting acceptable communities "may prolong the search" — but families with children in established schools, with medical specialists, and with service provider relationships cannot arbitrarily relocate without serious harm to continuity of care.

The summary reality: for the majority of Alberta's 79,290 AISH recipients, private market rent mathematically exceeds the AISH living allowance, community housing has a years-long waitlist

with no guarantee of placement, and the community housing that does exist had its rent increased 63 percent in October 2025.

Part 3: The compounded reality

Homelessness representation

The composite housing situation produces the following documented outcome: Indigenous people are significantly overrepresented in the homelessness population. Indigenous people represent approximately 30 percent of the 150,000 to 300,000 people who experience homelessness each night in Canada, despite representing approximately 5 percent of the total Canadian population — a sixfold overrepresentation.

On the Prairies specifically, the representation is sharper. 68 percent of people using shelters on the Prairies are First Nations or Indigenous — as reported by the AFN in 2024. Indigenous people are also more than twice as likely as non-Indigenous people to experience “hidden homelessness” — couch surfing, transient accommodation, living temporarily with family or in situations without a stable address of one’s own. For women and gender-diverse Indigenous people specifically, the overrepresentation intersects with the documented patterns of gender-based violence — housing shortages force women and children to remain in unsafe living situations, contributing to the circumstances that Section 11 of this report addresses in the context of missing and murdered Indigenous women and girls.

Why Indigenous disabled Albertans face compounded risk

An Indigenous disabled Albertan faces the following housing situation:

- The on-reserve option: a federally-administered housing system that the Auditor General has repeatedly characterized as inadequate, with documented overcrowding at 25 percent of households, mould at levels producing verified respiratory and immune harm, fire death rates five to seventeen times those of non-Indigenous Canadians, and flood exposure affecting 80 percent of reserves. For a disabled person — whose health is already compromised, who often requires specific accessibility accommodations, and who may be medically unable to tolerate the air-quality and overcrowding conditions documented — the on-reserve option is a housing option that may actively worsen the disability.
- The off-reserve urban option: Alberta’s uncapped rental market, where the AISH monthly benefit of \$1,940 is mathematically insufficient to cover average one-bedroom rent, let alone three-bedroom rent for families. Community housing is the only theoretical alternative; community housing has a 10,000+ household waitlist, no guaranteed timeline, and a rent structure that was increased 63 percent by ministerial order in October 2025.
- The jurisdictional fracture: the person’s identity places them in federal jurisdiction under section 91(24); the services they need are provincial. If they live on reserve, Alberta disclaims

responsibility for income support. If they live off reserve, they access AISH or Income Support through the province, but their health benefits are administered federally through the Non-Insured Health Benefits program. Neither the federal Indigenous housing system nor the Alberta provincial housing system is designed to accommodate the intersection of disability and Indigenous identity. Both systems treat that intersection as the other's responsibility.

- The mobility trap: as established in Section 3 and in the Alberta Disability Refugee documentation of the broader report series, disabled Albertans on AISH cannot financially afford to leave the province even when Alberta's housing structure fails them. For Indigenous disabled Albertans, leaving Alberta means leaving not only the Alberta benefit structure but also the specific cultural, community, family, and land-based relationships that constitute the basis of Indigenous wellbeing. The mobility trap compounds.

The specific risk profile for the ADAP transition

The July 1, 2026 ADAP transition does not alter the housing conditions documented in this section. But it does change the financial relationship the affected population has with those conditions. Specifically:

- The \$200 per month reduction from \$1,940 AISH to \$1,740 ADAP, taking full effect January 2028, removes \$200 per month from a budget that, in an uncapped rental market with rising rents, was already mathematically insufficient.
- The CDB clawback, documented in Section 17, removed another \$200 per month in federal disability supplementation that, in every other Canadian province, would have increased the recipient's monthly income. In Alberta it does not.
- The October 2025 community housing rent increase added \$221 per month in housing cost to recipients in the subsidized housing system — the population least able to absorb it.
- The combined effect: up to \$621 per month in reduced net disposable income between July 2025 and January 2028 for AISH recipients in community housing. In a province with no rent cap. While Alberta's Heritage Fund grew \$383 million in a single quarter (Q3 2025–26) and reached \$31.9 billion.

The ADAP transition does not cause the housing crisis. The housing crisis was already structural. But ADAP deepens the crisis by removing \$200 per month from recipients whose housing situation was already unviable, and it does so as part of a provincial fiscal policy that, in the same period, continued to add to a savings fund that the Alberta Heritage Savings Trust Fund Act does not permit to be used for income support purposes without a specific legislative vote that the government has declined to hold.

Alberta's Heritage Fund grew \$383 million in the single quarter during which the province cut \$49 million from AISH, implemented the CDB clawback, and raised community housing rent by 63 percent. The money

exists. The decision not to use it for housing stability is a policy choice, not a fiscal constraint.”

What this section establishes for the remainder of the report

The housing situation documented here is not recent, not a product of a single government, and not attributable to fiscal constraint. On-reserve housing has been flagged by four Auditor General reports across twenty years as persistently inadequate. The Alberta provincial rental market’s lack of rent cap has been a policy choice across multiple governments. The 63 percent community housing rent increase was signed by ministerial order without legislative vote or public consultation. The \$141 million Alberta-specific underfunding of First Nations housing by CMHC is documented, acknowledged, and will not be retroactively compensated.

The effect on the population Section 2 identified — Indigenous disabled Albertans, whose average life expectancy at 62.81 years is 19 years below the non-First Nations Alberta average — is that housing instability, mould-driven respiratory disease, overcrowding-driven infectious disease, fire risk, flood displacement, and homelessness are actively contributing to the mortality gap in real time.

Housing is not separate from health. Housing is not separate from disability. Housing is not separate from mortality. The constitutional architecture of Canadian federalism places on-reserve housing under federal jurisdiction and off-reserve housing under provincial jurisdiction. The operational practice of both levels of government has been to treat housing as a budget line rather than as the foundational infrastructure of life expectancy and health. Indigenous disabled Albertans live inside the consequence of that practice.

Section 5 addressed contaminated land. Section 6 will address water. Section 7 will address the dual healthcare system. Each documents a separate dimension of the same upstream reality. Section 4 has now documented the shelter dimension. The population this report is written to serve is dying earlier than the Alberta average in housing that federal and provincial governments know is inadequate, have known for decades, and have failed to remedy across every government of every party in the documented record.

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SECTION 5

Contaminated Land and the Multigenerational Body

What has been known, for how long, and what the body does with it

Forever chemicals do not lose their potency through ignorance of their existence.

This section establishes a claim that is larger than a single community or a single contaminant. The claim is that the current disability profile of Indigenous Albertans — and of many non-Indigenous Albertans living downstream of the same systems — is the direct measurable consequence of more than a century of documented government knowledge about contamination, paired with documented government failure to act on that knowledge.

The evidence supporting this claim is not obscure. It is in federal inventories, peer-reviewed toxicology, Indigenous-led community health studies, Alberta Cancer Board reports, United Nations human rights findings, and Canadian court records. The evidence has been available in every decade for which this report has accumulated data. What this section does is assemble it in one place, so the pattern it forms is legible on a single reading.

The section is organized around five questions. First: how did the contamination get onto the land. Second: what specifically is in it. Third: what does the body do with it, once exposed. Fourth: what has this produced on the ground across multiple communities — with Grassy Narrows, Aamjiwnaang, Kanesatake, and Fort Chipewyan presented as case studies. Fifth: what is being done about it right now, and by whom.

1. How contamination reached the land

The colonial pattern: relocation to marginal ground

The creation of Indian Reserves in Canada was not a neutral process of land designation. Indigenous peoples were forcibly relocated to specific parcels of land selected by the federal government, and the selection criteria reflected settler-colonial priorities: agricultural value, mineral access, transportation corridors, and industrial potential were reserved for non-Indigenous settlement. The land remaining available for Reserve designation was, disproportionately, the land that non-Indigenous interests did not want.

This pattern is documented across the Canadian Union of Public Employees, Ecojustice, and multiple peer-reviewed analyses of environmental racism in Canada. Reserves were “often chosen in remote places where non-Indigenous people would not choose to settle.” The same

remoteness that characterized the selection pattern later became the reason industrial waste, military activity, and abandoned extraction sites concentrated on or adjacent to Reserve lands: these were places where the regulatory scrutiny of non-Indigenous-populated areas did not apply, and where the political cost of contamination was low because the population affected had less political power.

The regulatory vacuum

Once Reserves were established, a structural legal gap emerged that persists to the present day. The Indian Act — the federal legislation governing Reserve lands — does not contain environmental protection provisions. Provincial environmental laws, which govern soil, water, and waste management everywhere else in Canada, do not apply on Reserve land. The 2009 Fall Report of the Auditor General of Canada identified this explicitly as an “environmental regulatory gap,” noting that Reserves lack “appropriate laws, which should govern industrial, commercial or other such activities.”

The consequences of this gap are concrete. As documented by Ecojustice and reported through parliamentary testimony on Bill C-226 (the environmental racism legislation), the maximum penalty for dumping garbage or waste on an “Indian Reserve” is one hundred dollars. The penalty for the same act on provincially regulated land can reach into the hundreds of thousands. The financial incentive to dispose of toxic materials on Reserve land, rather than in regulated facilities, has been structurally built into Canadian environmental law for decades.

In 2019, Baskut Tuncak, the United Nations Special Rapporteur on the implications for human rights of the environmentally sound management and disposal of hazardous substances and wastes, concluded a country visit to Canada. His finding, in his own words: “I observed a pervasive trend of inaction of the Canadian Government in the face of existing health threats from decades of historical and current environmental injustices and the cumulative impacts of toxic exposures by indigenous peoples.” The Special Rapporteur’s report is a matter of formal United Nations record.

The contamination inventory

The Government of Canada’s own Federal Contaminated Sites Inventory, combined with Indigenous Services Canada’s Contaminated Sites On-Reserve program evaluations, establishes the following:

Reserves make up approximately 0.5 percent of Canada’s total land mass.

Reserves host over 20 percent of all federal contaminated sites south of the 60th parallel.

As of Government of Canada 2021 figures, approximately 4,486 federal contaminated sites are located on Indigenous Reserve land.

A 2022 peer-reviewed scoping review published in medRxiv placed the figure at approximately 29 percent of all Canadian federal contaminated sites being on Indigenous Reserve land.

By March 2013, Indigenous Services Canada was actively managing 2,392 contaminated sites on Reserves. 293 of those sites were classified as Class 1 — the federal classification meaning “high risk to human health and safety.”

These are not accusations from advocacy organizations. These are the federal government's own administrative records, used by the government to justify the budget allocations of the Federal Contaminated Sites Action Plan.

2. What is in the contamination

The Indigenous Services Canada Contaminated Sites On-Reserve program reports the following contaminant composition of on-Reserve sites, as of 2014:

- 67 percent of on-Reserve contaminated sites were related to fuel storage and handling — primarily petroleum hydrocarbons from leaking or abandoned fuel tanks and infrastructure.
- 20 percent were related to solid waste and landfill operations — including heavy metals, plastics, asbestos, pharmaceuticals, pesticides, electronics, and industrial byproducts.
- 13 percent were classified as “other contamination,” including radioactive materials, industrial chemicals, firefighting foam residues, and legacy military contamination.

The specific chemical categories documented in the federal inventory include: petroleum hydrocarbons, polycyclic aromatic hydrocarbons (PAHs), heavy metals (lead, mercury, arsenic, cadmium, chromium), radioactive isotopes, solvents, asbestos, and per- and polyfluoroalkyl substances (PFAS). Each of these categories has an established, peer-reviewed toxicological profile. None of them is exotic. All of them have been classified as health hazards by Health Canada, the United States Environmental Protection Agency, the World Health Organization, or the International Agency for Research on Cancer — in most cases, by more than one of those bodies.

What is less widely understood, and what this section establishes in the sections that follow, is that these contaminants do not operate in isolation. They do not produce a single, identifiable, organ-specific illness in each exposed person. They interact — with each other, with the body's own systems, with the epigenome that governs gene expression, and with the multigenerational inheritance of bodies that have carried exposure for decades. The cumulative effect is not a list of diseases. It is a systemic reshaping of how a population ages, reproduces, and dies.

3. What the body does with contamination

The body is not organ-specific. The contamination is not organ-specific.

Government communications about contamination exposure consistently frame health effects in organ-specific terms. “Respiratory effects from air pollution.” “Waterborne illness from contaminated water.” “Mesothelioma from asbestos exposure.” These framings are not medically accurate. They are rhetorical devices. They allow the government to deny causal links by testing for one specific disease in one specific tissue, finding that the elevation is not statistically significant within that narrow frame, and declaring the investigation complete.

The human body does not work this way. The lungs and digestive tract are entry points for environmental contaminants, not destinations. Once a particle, chemical, or fiber crosses the epithelial barrier, it enters the lymphatic and circulatory systems. From there, it travels. It settles where cells are dividing rapidly, where tissue is most metabolically active, where the individual's own vulnerabilities — genetic, epigenetic, nutritional, developmental — meet the contaminant at the moment of exposure.

Asbestos, to take the example most frequently cited in government communications, does not only cause mesothelioma and lung cancer. The International Agency for Research on Cancer classifies asbestos as carcinogenic to multiple organ systems. Asbestos fibers have been documented in ovarian tissue, in the peritoneum, in the larynx, in the pharynx, in the stomach, in the colon, and in the bone marrow. The latency period between first exposure and clinical symptom is 20 to 40 years. A person exposed in childhood on contaminated land may not present with disease until their fifties or sixties — at which point the causal link is often described as “unclear” by the same agencies that documented the original exposure.

The same systemic-migration pattern applies to heavy metals, PAHs, PFAS, and most radiologically active compounds. A single exposure can produce effects in multiple organ systems, across decades, with no predictable one-to-one relationship between exposure site and eventual disease site.

Organ-specific denial is the rhetorical tool that keeps systemic harm invisible.

Epigenetics: the body remembers what the environment did

Beyond the lifetime of a single exposed individual, peer-reviewed toxicology and epigenetics research now establishes that exposure to heavy metals, PAHs, and other environmental contaminants can produce heritable epigenetic modifications — changes to how DNA is expressed, passed from one generation to the next through the germline — even in children and grandchildren who were never directly exposed.

In 2015, a study published in Scientific Reports, a Nature Portfolio journal, reported the first human demonstration that environmental lead exposure in pregnant women produces DNA methylation changes measurable in their grandchildren's neonatal blood. The authors concluded: “This is the first demonstration that an environmental exposure in pregnant mothers can have an epigenetic effect on the DNA methylation pattern in the grandchildren.” The

mechanism is germline alteration: exposure affects the fetal germ cells of the unborn child, which later become the reproductive material that produces the grandchild.

This is no longer an isolated finding. A 2024 peer-reviewed review in *Clinical Epigenetics*, authored by researchers from the Harvard T.H. Chan School of Public Health and Johns Hopkins University, synthesized the evidence base and concluded: “Environmental influences that occurred in one generation may be transmitted beyond the second generation, exerting a long-lasting effect.” A 2025 review in *MDPI Toxics* specifically examined heavy metals — methylmercury, cadmium, arsenic, and lead — and confirmed documented transgenerational epigenetic effects for each one.

The implications are precise. Indigenous Albertans living on or near contaminated Reserve land today are not only dealing with their own direct exposures. They are also dealing with the accumulated epigenetic inheritance of parents, grandparents, and in some cases great-grandparents who lived through the same contamination when the exposures were higher, less monitored, and less documented. The body carrying disability in 2026 may be expressing altered gene function that originated in a germline exposure in the 1960s, or in a parental exposure before conception in the 1980s, or in a childhood exposure during residential school attendance in the 1940s.

Contamination does not forget. The body does not forget. And the epigenome carries the memory into descendants whose own direct exposures would be insufficient to explain the disability profile they present.

The exposome framework

Contemporary toxicology has moved past the single-chemical, single-exposure, single-outcome model that dominated risk assessment in the twentieth century. The current framework — known as the exposome — describes human health as the cumulative result of all environmental exposures across a lifetime, interacting with genetic and epigenetic predispositions. The exposome is systemic, not event-specific. It is lifelong, not moment-in-time. And it is inherited, not restricted to the individual.

Within this framework, every government statement that takes the form “we see no statistically significant elevation of specific cancer X in population Y when controlling for regional baseline” becomes a statement about the limits of the question being asked — not a statement about the absence of harm. The question is structurally incapable of detecting the pattern the evidence demands.

4. Four communities, one pattern: case studies in environmental racism

The colonial pattern documented in Section 1 — relocation to marginal land, regulatory vacuum, industrial permissiveness — did not affect one community or one region. It is a national pattern, reproduced across provincial boundaries wherever Indigenous territory and industrial activity

have overlapped. This section presents four case studies: Grassy Narrows in northwestern Ontario, Aamjiwnaang in southwestern Ontario, Kanesatake in Quebec, and Fort Chipewyan in northeastern Alberta. Each community's experience is specific. The pattern connecting them is structural.

Grassy Narrows First Nation (Ontario): three generations of mercury

Between 1962 and 1970, the Dryden Chemical Company — a subsidiary of the British multinational Reed International — discharged between 9,000 and 11,000 kilograms of mercury into the English-Wabigoon River system, approximately 100 kilometres upstream of the Asubpeeschoseewagong Anishinabek (Grassy Narrows First Nation). The river was the community's primary food source, their economic base through the commercial fishery, and the centre of their cultural identity. The Lancet has characterized the resulting contamination as “one of the worst cases of environmental poisoning in Canadian history.”

The consequences were immediate and have not ended. In 1975, Japanese physician Dr. Masazumi Harada — who had documented Minamata disease in Japan, where methylmercury discharge into Minamata Bay produced the syndrome that now bears its name — examined 89 people in Grassy Narrows and Whitedog (Wabaseemoong Independent Nations). Many presented symptoms of mercury poisoning; at that early stage, the classic full-syndrome diagnosis of Minamata disease was considered not yet established. When Dr. Harada returned in 2002 and 2004, he examined 189 individuals. Sixty were diagnosed with Minamata disease. Another 54 were diagnosed with Minamata disease with complications from other causes. Twenty-five were classified as suspected cases. Of the 27 individuals who had been examined in both 1975 and the 2000s, their neurological status had “clearly gotten worse” over the intervening decades.

A 2020 peer-reviewed retrospective longitudinal study published in The Lancet Planetary Health, using historical biomarker data that the community obtained in 2019 after decades of effort, established a direct statistical association between documented mercury exposure and premature mortality (death before age 60) in Grassy Narrows. The highest recorded blood mercury concentration in the federal biomarker surveillance program — 660 micrograms per litre — belonged to a resident of Grassy Narrows.

A 2024 peer-reviewed longitudinal study published in Environmental Health documented that neurological symptoms in the exposed population are not stable but progressive. The study's five-year follow-up measured marked increases in sensory disturbances, ataxia, visual field constriction, speech impairment, imbalance, and tremor — decades after the original exposure ended.

A 2017 independent study by five mercury experts, reported in the Canadian Medical Association Journal, found that mercury levels downstream of the Dryden plant remained approximately 130 times higher than at upstream locations — indicating that the contamination

is not merely legacy exposure fading with time, but is being actively resupplied. The contamination, in other words, is still happening.

Judy Da Silva, the environmental health coordinator for Grassy Narrows, has documented the mercury exposure in her own body. Her children now show symptoms. Her ten-year-old son has been approved for compensation by the Mercury Disability Board — a compensation system established in 1986 under the Grassy Narrows and Islington Bands Mercury Pollution Claims Settlement Act. In her own words: “I feel for my kids. I’m scared for my kids. I don’t like that they’re poisoned. That’s one of the last things I want to do is try and poison my kids. And I can’t help it because I was poisoned.”

According to a community health study cited by CBC, approximately 90 percent of the Grassy Narrows population experiences symptoms of mercury poisoning, including neurological problems, seizures, cognitive delays, and sensory disturbances. Construction of a promised Mercury Care Home and Wellness Centre, committed to by the federal government in 2017, had not begun as of late 2023. As of the same date, the community had been without a physician for 17 months.

Aamjiwnaang First Nation (Ontario): Chemical Valley

The Aamjiwnaang First Nation is located in southwestern Ontario, on the shore of the St. Clair River near Sarnia. Approximately 850 of the community’s 2,000 members live on the reserve. The reserve sits within a 25-kilometre radius of 60 industrial facilities — petrochemical plants, refineries, and chemical manufacturing sites that together form what is commonly known as Chemical Valley. These facilities account for approximately 40 percent of Canada’s petrochemical industry.

The pattern of industrial encroachment on Aamjiwnaang is historical. As documented in the University of Toronto’s Environmental Data Justice Lab, Imperial Oil began operations in Sarnia in 1880 — the same year the residential school system formally came into being. The two institutions — residential schools and Chemical Valley — were, from the beginning, parallel expressions of a single colonial logic. Residential schools are no longer in operation. Chemical Valley remains fully operational.

The 2007 Ecojustice report *Exposing Canada’s Chemical Valley*, using data from Environment Canada’s National Pollutant Release Inventory, established that in 2005, 131 million kilograms of pollutants were released from 46 plants in the Sarnia area. Sixty percent of the 5.7 million kilograms of toxic air releases recorded in 2005 occurred within a five-kilometre radius of the Aamjiwnaang reserve. The pollutants documented include known reproductive and developmental toxins, carcinogens, endocrine disruptors, and heavy metals including mercury.

The health consequences documented in peer-reviewed research are striking. Research published in the U.S. journal *Environmental Health Perspectives* reported that between 1999 and 2003, the sex ratio of live births at Aamjiwnaang was approximately 33 percent male and 67

percent female — a two-to-one female-to-male birth ratio, representing one of the lowest male birth rates ever documented in a human population. The authors attributed the anomaly in substantial part to chemical exposure during gestation. Subsequent research by McGill University's Niladri Basu confirmed that Aamjiwnaang mothers and children carried elevated levels of polychlorinated biphenyls, cadmium, perfluorinated chemicals, and the pesticides hexachlorocyclohexane and DDT compared with the Canadian average.

A 2006 community survey by Aamjiwnaang's environment committee documented that 40 percent of surveyed band members required an inhaler. The community reports elevated rates of miscarriage, stillbirth, asthma, cancer, and chronic headache. As recently as April 2024, Aamjiwnaang leadership called for the shutdown of the INEOS Styrolution facility after pollutant monitoring recorded benzene levels high enough that the First Nation sent its employees home; community members reported headaches, nausea, and dizziness, with at least one parent bringing their child to the hospital.

Ada Lockridge, founder of the Aamjiwnaang environmental committee, has for years operated a “bucket brigade” program in which community members collect their own environmental samples for independent laboratory analysis — a grassroots monitoring system that developed because federal and provincial monitoring was perceived as inadequate to document the exposures the community was experiencing. Aamjiwnaang has also pursued Charter of Rights litigation through Ecojustice, challenging the Ontario Ministry of the Environment's permitting practices.

Kanesatake (Quebec): illegal dumping and regulatory abandonment

Kanesatake is a Mohawk community located approximately one hour's drive northwest of Montreal, between the Lake of Two Mountains and the Ottawa River. It is one of the smallest Indigenous territories in Quebec. Since approximately 2015, it has also been the site of one of the most visible active cases of illegal toxic dumping documented in Canada.

The G&R Recycling facility in Kanesatake received an initial permit in 2014 to operate a construction and demolition waste sorting centre with a capacity of 27,000 cubic metres. By the time Quebec's Ministry of the Environment revoked the permit in October 2020, the site contained more than 400,000 cubic metres of waste — roughly fifteen times the permitted volume. An investigation by The Eastern Door, Ricochet Media, and La Presse established that the site had received industrial waste, including polychlorinated biphenyls (PCBs), a federally controlled hazardous substance. Construction trucks from the Montreal area, investigators found, were paying approximately \$150 per load in cash to dispose of materials on Mohawk land — approximately half the cost of legitimate disposal at regulated facilities off-reserve.

Sources in the construction industry told Ricochet that it was “an open secret” in the sector: if you wanted to dispose of something without questions, you “go to the Mohawks.” The phrase captures the structural reality that environmental racism produces — Indigenous land is

understood, within the industries that profit from the disposal, as the place where regulation does not reach.

Federal Crown-Indigenous Relations Minister Marc Miller, speaking publicly in May 2023, characterized the G&R site as a potential “environmental bomb.” Despite this acknowledgment at the federal ministerial level, substantive remediation has not occurred. As of late 2023, Ricochet documented continued illegal dumping beyond the original G&R site — trucks arriving in Kanesatake after dark to unload contaminated soil directly into residential yards, driveways, and roads leading into the lake. A May 2024 soil testing operation by Quebec’s environment ministry confirmed the presence of polycyclic aromatic hydrocarbons and petroleum hydrocarbons in soil deposited on the Lake of Two Mountains shoreline. Quebec is pursuing emergency injunction proceedings against nearly twenty defendants.

Documentary health consequences in the community mirror those in Aamjiwnaang: Mohawk families near the G&R site have reported migraines, breathing problems, and nosebleeds to Ricochet journalists. One community member reported to a federal minister that her physician had advised her to have her fingernails and hair tested for toxic metal content; her formal letter to the federal Minister of Environment and Climate Change received a two-sentence reply indicating that the matter “falls under provincial jurisdiction.”

The Kanesatake case establishes two points that the Fort Chipewyan case does not. First, environmental racism is not restricted to legacy industrial sites; it is actively reproduced through ongoing illegal activity that governments choose not to stop. Second, the jurisdictional regulatory vacuum described earlier in this section — the gap between federal, provincial, and Indigenous governance authority — is not theoretical. It is exploited in real time, by actors who understand that no single authority will act decisively on Indigenous land.

Fort Chipewyan (Alberta): downstream of the oil sands

Fort Chipewyan is a community of approximately 641 people located on the western shore of Lake Athabasca in northeastern Alberta. It is home to members of the Mikisew Cree First Nation, the Athabasca Chipewyan First Nation, and the Fort Chipewyan Métis Nation. It sits approximately 250 kilometres downstream from the Athabasca oil sands industrial complex. Everything upstream drains toward it.

The knowledge timeline

2006: Dr. John O’Connor, the community physician, reported to media an unusually high incidence of cholangiocarcinoma — a rare bile duct cancer — and other cancers in Fort Chipewyan. His observations were based on direct clinical experience and included cancers seen in individuals without conventional risk factors (low smoking rates, traditional diet, active lifestyles).

2007–2009: Health Canada lodged formal complaints against Dr. O’Connor with the Alberta College of Physicians and Surgeons, alleging he had caused “undue alarm” in the community.

He was threatened with the loss of his medical licence. He was eventually exonerated of all charges.

2009: The Alberta Cancer Board released its investigation of Fort Chipewyan cancer rates (1995–2006). The findings: cancer rates were approximately 30 percent higher than expected, with elevated rates of blood cancer, lymphatic cancer, and soft tissue cancers. The Board noted the elevation was concentrated in the most recent six-year window and recommended “closer monitoring of cancer occurrences in Fort Chipewyan in the coming years.” The Board declined to investigate causation.

2010: Dr. David Schindler, one of Canada’s most decorated freshwater ecologists, co-authored a paper in the Proceedings of the National Academy of Sciences establishing that summer concentrations of polycyclic aromatic hydrocarbons in the Athabasca River ran 22 times higher downstream of oil sands development than upstream. The paper established oil sands development as a contributor to toxic loading in the river, at levels harmful to aquatic life.

2014: Dr. Stephane McLachlan of the University of Manitoba, working jointly with Mikisew Cree First Nation and Athabasca Chipewyan First Nation, released phase two of Environmental and Human Health Implications of Athabasca Oil Sands. The study measured contaminants in water and in traditional food sources: beavers, ducks, fish, moose, muskrats. Findings included high concentrations of carcinogenic polycyclic aromatic hydrocarbons, arsenic, mercury, cadmium, and selenium. 20 out of 94 interviewed community members reported cancer diagnoses.

2018: The federal Oil Sands Monitoring Program, a joint federal-provincial program, reported increased concentrations of aluminum, copper, iron, and lead in the Athabasca River, exceeding water safety guidelines. Elevated arsenic was detected in tributaries.

May 2022: A major spill of contaminated wastewater was discovered from a tailings pond at Imperial Oil’s Kearl oil sands project. Mikisew Cree First Nation leadership has alleged that cancer incidence data after this date was not provided to the community on request.

April 14, 2026: Standing inside Parliament Hill, Mikisew Cree First Nation Chief Billy-Joe Tuccaro released the results of a new cancer study commissioned by the First Nation. The findings: between 1993 and 2022, cancer rates in Fort Chipewyan were approximately 25 percent higher than the rest of Alberta. 149 community members were diagnosed with 24 different types of cancer over the study period. Chief Tuccaro stated the actual number of cases is likely closer to 250–300, with the 149 figure representing a significant undercount due to residents leaving the community for medical treatment and therefore being recorded in other health regions.

Within hours, the Alberta Ministry of Primary and Preventative Health Services responded that adult cancer rates were “largely within normal ranges” and that there was “no known causal link between oil sands development and cancer rates in the region.” The Ministry’s assessment was issued without a visit to the community.

The pattern across the four cases

The 24 different cancer types identified in the Mikisew Cree 2026 study are not a tight cluster of one disease. They are a broad distribution — bile duct, blood, lymphatic, soft tissue, and others — across a community of 641 people. This distribution is consistent with the toxicology literature on systemic contamination of a biological environment: water, food, air, soil, and intergenerational germline exposure, compounding over decades. It is not consistent with the organ-specific denial framework the province has consistently applied.

Placing the four case studies side by side reveals a consistent structure. Grassy Narrows: decades of mercury exposure, progressive neurological decline across three generations, 90 percent symptomatic population, government acknowledgment of symptoms without acknowledgment of poisoning, compensation structures that require individualized proof. Aamjiwnaang: active emissions from 60 surrounding industrial facilities, a two-to-one female-to-male birth ratio, reproductive and respiratory impacts, regulatory response described as inadequate by Ontario's own Environment Commissioner, community-led monitoring because government monitoring was insufficient. Kanesatake: active illegal dumping on Mohawk land, federal ministerial acknowledgment as an "environmental bomb," continued dumping notwithstanding, community reports of migraines and respiratory symptoms dismissed with jurisdictional deflection. Fort Chipewyan: 25 percent higher cancer rates across 24 types, same-day provincial dismissal, a whistleblower physician threatened with licence loss.

In each case, the scientific evidence is extensive. In each case, the government response is framed as inadequate evidence to demonstrate causation. In each case, the community itself — working with academic collaborators, environmental lawyers, and investigative journalists — has done the documentation work that the state declined to do. The pattern is not a cluster of isolated incidents. It is a method.

As Dr. O'Connor observed of Fort Chipewyan: "In my opinion this is environmental racism. It would not happen upstream of non-Indigenous communities. The facts are already established a long time ago." The statement applies equally, with only the place names changed, to Grassy Narrows, Aamjiwnaang, and Kanesatake.

5. What is being done, and by whom, in 2026

The tailings reality

The Athabasca oil sands tailings ponds currently hold approximately 1.4 trillion litres of toxic wastewater. This is an area larger than the city of Vancouver. Industry has known about seepage from these ponds for decades. Regulators have known as well. The contents include polycyclic aromatic hydrocarbons, naphthenic acids, heavy metals, and complex process-affected waters whose full toxicological profile has not been publicly characterized.

The government of Premier Danielle Smith has mandated the Minister of Environment and Protected Areas to fast-track a “treat and release” plan that would permit the treated discharge of tailings water into the Athabasca River system. The affected downstream communities — Mikisew Cree First Nation, Athabasca Chipewyan First Nation, Fort Chipewyan Métis Nation, and the smaller communities further downstream toward the Peace-Athabasca Delta — have not given free, prior, and informed consent to this plan, as required under the United Nations Declaration on the Rights of Indigenous Peoples.

The PFAS dimension: why this is not only an Indigenous issue

Per- and polyfluoroalkyl substances — PFAS — are a class of more than 4,700 synthetic chemicals used for decades in firefighting foam, non-stick cookware, waterproof textiles, cosmetics, food packaging, and industrial lubricants. They are known as “forever chemicals” because, once released into the environment, they take hundreds to thousands of years to break down.

According to Health Canada biomonitoring surveys, more than 98 percent of Canadians have PFAS in their blood. According to the Canadian Environmental Law Association, 99 percent or more of all Canadians tested have PFOA and PFOS specifically — two of the most studied PFAS compounds — present in their blood and organs, including residents of far-north communities geographically distant from industrial sources.

PFAS are associated with kidney cancer, testicular cancer, low birth weight, immune suppression, reduced vaccine response, thyroid disease, developmental delays in children, hormonal disruption, and elevated cholesterol. These health effects are documented across peer-reviewed studies and confirmed by Health Canada’s own assessment literature.

Standard municipal water treatment systems — the infrastructure that supplies drinking water to nearly every urban and suburban Albertan — do not remove PFAS. Activated carbon filtration and reverse osmosis can remove PFAS, but these are expensive, generate hazardous waste requiring specialized disposal, and are not deployed at scale in the majority of Canadian municipalities. Wastewater treatment plants also do not filter PFAS, meaning the treated effluent discharged back into rivers and lakes contains the same forever chemicals that entered the system.

As of March 2025, the Government of Canada’s own State of PFAS Report concluded that the class of PFAS, excluding fluoropolymers, is “harmful to human health and the environment.” As of the drafting of this report in April 2026, Canada has not implemented legally enforceable national drinking water limits on PFAS. Health Canada’s objective of 30 nanograms per litre is an advisory target, not an enforceable standard. The United States Environmental Protection Agency has, by contrast, finalized enforceable limits of 4 parts per trillion for PFOA and PFOS — standards roughly eight times more protective than Canada’s current objective.

The implication is straightforward: every Albertan, Indigenous or not, urban or rural, on or off reserve, is currently being exposed to the same class of chemicals that are contaminating Indigenous lands. The mechanism of exposure is universal drinking water infrastructure that predates the recognition of the contamination. The difference between the exposure on contaminated Reserve lands and the exposure in a suburban tap is one of degree, not of category.

The disabled population being targeted by ADAP is the same population that has borne the public health cost of generations of regulatory failure.

The whistleblower pattern

The parallel between Dr. John O'Connor's 2007 treatment — formal complaint to his medical regulator, threatened licence suspension, the burden of defending himself against “undue alarm” charges that were ultimately dismissed — and the more recent experience of Athana Mentzelopoulos, the former CEO of Alberta Health Services who has alleged she was fired for investigating the procurement irregularities that led to the \$49 million MHCare Medical scandal, is not incidental. It is a pattern. Public officials who surface information that disturbs the regulatory status quo face career consequences. The consequences do not require overt suppression. Administrative friction is sufficient.

The chilling effect on the supply of future evidence is structural. Physicians, scientists, and civil servants who might otherwise document harm observe the treatment of their predecessors and weigh the cost of raising the alarm. Over time, the visible record of harm becomes systematically thinner than the underlying reality it is supposed to represent.

What this means for the ADAP transition

A province that has known, for more than a century, that the land its disabled citizens live on is contaminated with documented carcinogens, heavy metals, PAHs, radioactive materials, asbestos, and forever chemicals — and that has permitted those contaminations to continue, through regulatory inaction and active policy choice — is now proceeding with a mandatory reclassification of those same citizens' disability benefits on the premise that many of them are capable of full-time employment.

The population being moved from AISH to ADAP on July 1, 2026 is, disproportionately, the population whose disability profile has been shaped by a century of environmental exposure that the province itself permitted. The health conditions being reassessed for “employment readiness” are the conditions the exposures produced. The bodies being told to work harder are the bodies carrying inheritance the government has never accounted for.

The fiscal hypocrisy of the transition — the \$49 million cut to AISH matched by \$49 million lost to procurement corruption, against a \$5.6 billion Heritage Fund surplus — is documented

elsewhere in this report series. The medical hypocrisy of the transition is established here: a government cannot credibly require proof of ongoing disability from a population whose disability it has, through a century of regulatory choice, actively created.

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SECTION 6

Water

The drinking water advisories that have outlasted the children who were born under them, the industrial effluent that contaminates treaty waters, and the Alberta government's documented opposition to enshrining First Nations' right to clean water in law

In 2010, the United Nations declared access to clean water a human right. In 2026, that right continues to be denied to approximately forty First Nations in Canada that have been under long-term drinking water advisories — some for more than thirty years. The inequality is not incidental. It is the documented record of every federal government and every provincial government that has held the file.”

This section documents the water crisis as it is experienced by Indigenous communities in Canada generally and by Indigenous disabled Albertans specifically. The scope is deliberately broader than drinking water alone. It includes three interlocking dimensions: drinking water advisories on reserve, industrial water contamination in treaty territories (particularly downstream of the Athabasca oil sands), and the legislative framework — or the absence of one — that governs whether a First Nation person in Canada has any enforceable right to safe water at all.

Three findings structure the section. First, that the long-term drinking water advisory system is not a matter of isolated infrastructure failures but a sustained funding and policy failure across multiple federal governments, documented most recently by the Auditor General of Canada in 2025 as “unsatisfactory.” Second, that industrial water contamination — particularly the Athabasca oil sands tailings complex, now exceeding 1.5 trillion litres and growing — is a separate water harm borne disproportionately by downstream First Nations whose treaty rights to the river predate the industrial development by a century. Third, that the Government of Alberta formally opposed Bill C-61, the federal First Nations Clean Water Act, on the stated ground that enforceable First Nations water rights would “undermine competitiveness, and delay project development” — a position that places provincial resource-extraction economics in direct conflict with the human right to water as recognized by the United Nations and the Crown's own treaty obligations.

The connection to disability is direct and documented. Contaminated drinking water produces skin infections, infectious disease, chronic cardiovascular and renal burden, and in the Athabasca watershed specifically, elevated rates of rare cancers. Water scarcity and advisory-

period anxiety produce mental health harm documented by federal Indigenous Services Canada itself. The children currently being born in communities under long-term advisories will enter adulthood with a documented health-outcome profile different from their non-Indigenous peers — a difference that, by the time it is adjudicated by provincial disability benefit programs, will be characterized as the applicant's personal medical history rather than the product of federal policy failure.

Part 1: The long-term drinking water advisories

Current scale

As of the most recent Indigenous Services Canada reporting in early 2026, approximately 40 long-term drinking water advisories remained in effect in 37 First Nations communities across Canada. 152 long-term advisories have been lifted since November 2015. The arithmetic is simple: for every advisory lifted, another has been added. In 2025 alone, 11 new long-term advisories were added while 3 were lifted. In 2024, six were added and four lifted. In 2023, three were added and six lifted. The net progress, over the three-year window immediately prior to the publication of this report, has been approximately zero.

The Council of Canadians documents, in its ongoing tracking, that 73 percent of First Nations water systems operate at high or medium risk of contamination. The Parliamentary Budget Officer identified, prior to 2020, a \$138 million annual shortfall in federal funding for operation and maintenance of on-reserve water treatment equipment. That shortfall has not been publicly reconciled.

The 2025 Office of the Auditor General report on Indigenous Services Canada's water file rated the federal department's progress "unsatisfactory." The report documented that nine of the persisting long-term advisories had been in effect for more than a decade. The oldest persisting advisory, affecting Neskantaga First Nation in Ontario, has now been in place continuously since 1995 — over thirty years. Children born in Neskantaga in 1995 are now thirty-year-old adults who have never drunk tap water from their home community without boiling it first.

The children who were born under the advisory are now the adults who will never be able to teach their own children what safe tap water tasted like, because they never learned themselves. The loss is not only biological. It is cultural. A generation has grown up understanding that the water is a danger, not a relationship."

Why the advisories persist

The official reasons, drawn from Health Canada and the Assembly of First Nations joint analysis, are: disinfection failures (32 percent), equipment failure (30 percent), microbiological quality (18 percent), turbidity (6 percent), source water quality (6 percent), and other operational

issues (8 percent). What this breakdown does not capture is the underlying structural reason: the federal funding model for on-reserve water infrastructure has been inadequate for decades. The \$4.61 billion invested since 2015 on water projects has lifted 147 advisories but left the systemic problem unaddressed, because the funding model has covered capital construction without providing durable funding for operation, maintenance, and operator training. New water treatment plants are built. Without ongoing O&M funding and trained operators, plants fail. New advisories are issued. The cycle repeats.

On January 10, 2026, three severe drinking water emergencies were simultaneously active in First Nations communities: Pimicikamak Cree Nation in Manitoba (downed hydro line rendering the water treatment plant inoperative), Kashechewan First Nation in Ontario (water plant treatment failure), and Kugaaruk in Nunavut (sea water intrusion into the treatment plant). Indigenous Watchdog observed, in its tracking of these three events, the comparative question that the sustained advisory pattern raises: how quickly would equivalent infrastructure failures be addressed in a non-Indigenous southern Ontario or Alberta community of equivalent size?

The 2021 class-action settlement

On December 22, 2021, the Federal Court and the Court of Queen's Bench of Manitoba jointly approved the settlement agreement in the First Nations drinking water class actions. The settlement covered approximately 142,000 individual class members from 258 First Nations, together with 120 Impacted First Nations, for drinking water advisories lasting one year or longer between November 20, 1995 and June 20, 2021. The settlement totaled approximately \$8 billion, of which \$1.8 billion was allocated to direct compensation for individuals and \$6 billion was allocated to water infrastructure projects. A further commitment was made to modernize the legislative framework governing First Nations drinking water.

The settlement is documented, acknowledged, and reflected in Alberta's own administrative practice. The Alberta Income and Employment Supports Training and Health Benefits Policy Manual formally exempts compensation payments received under the First Nation Drinking Water Advisory Class Actions Settlement (2021) as "Harm Done" compensation, meaning the province formally acknowledges that the federal government caused the harm for which the settlement compensates. An AISH or Income Support recipient who receives a settlement payment under this program does not have the payment counted as income for benefit calculation purposes — because the province's own policy infrastructure treats the payment as compensation for harm, not as new income.

This is structurally significant. Alberta's provincial policy has formally incorporated, into its administrative framework, the legal acknowledgment that federal water policy failure constitutes a harm. That same province then, as documented below, opposed the federal legislation that would have prevented the harm from recurring.

Part 2: The legislative vacuum and the killing of Bill C-61

What Bill C-61 was, and what it would have done

The Safe Drinking Water for First Nations Act, first enacted in 2013, was repealed by the federal government in June 2022 — partly in response to sustained First Nations advocacy that the Act had been developed without adequate consultation and had not produced results. Its replacement was announced as forthcoming. Bill C-61, the First Nations Clean Water Act, was first read in the House of Commons on December 11, 2023. The bill was the federal legislative response to the 2021 class-action settlement's requirement for a modernized drinking water legislative framework.

Bill C-61 would have, at minimum, proposed to recognize safe drinking water as a basic human right for all First Nations; established enforceable water quality standards on reserves comparable to those in effect for non-Indigenous communities; set requirements for source-water protection for the watersheds flowing onto First Nations lands; and created a statutory framework through which drinking water advisories could be challenged and compelled to be addressed on timelines that would not stretch into decades. The legislation was supported by the Assembly of First Nations, the Chiefs of Ontario, the British Columbia Assembly of First Nations, and Indigenous organizations across the country.

The bill was sent to the Standing Committee on Indigenous and Northern Affairs in June 2024, held committee meetings through June and September 2024, and was expected to pass in 2025. It did not. Parliament was prorogued in January 2025. Bill C-61 died on the order paper. Prime Minister Mark Carney's minority Liberal government has committed to reintroducing successor legislation, but as of March 2026 no timeline had been announced. As the British Columbia Assembly of First Nations observed on World Water Day 2026: "During Prime Minister Mark Carney's campaign, he made explicit commitments to 'Enshrine First Nations' right to water into law.' History has demonstrated that basic legislation is not enough to ensure First Nations human rights to access safe drinking water are honoured by Canada."

Alberta's documented opposition

On July 10, 2025, the governments of Alberta and Ontario formally placed themselves on record as opposed to Bill C-61. The stated grounds for this opposition, as summarized by Indigenous Watchdog in its tracking of the legislation, were that passing the Act would "undermine competitiveness, and delay project development." In other words: resource extraction projects, in the provincial government's stated position, should take priority over enforceable First Nations rights to safe drinking water.

The position is documented. It is not contested. And it is consistent with the provincial government's broader pattern of resource-extraction-first policy, including its concurrent advocacy, as detailed below, for permitting the oil sands industry to release treated tailings water into the Athabasca River.

The province that opposed enshrining First Nations' right to clean water in federal law on grounds of 'competitiveness' is the same province proposing to release treated oil sands tailings into the river from which downstream First Nations communities draw their drinking water. These are not two separate policies. They are one policy, expressed twice."

Part 3: Industrial water contamination — the Athabasca tailings complex

The scale

The Athabasca oil sands represent the world's fourth-largest oil deposit by reserve size. The extraction process produces large volumes of process-affected water, which is stored in tailings ponds located on industry-operated sites in northern Alberta. As of 2024, the total stored tailings volume across the oil sands was approximately 1.5 trillion litres, held in ponds covering 270 square kilometres — an area larger than the city of Vancouver. The ponds have grown in volume at approximately 42 percent every five years since 2005.

The contents of these ponds include, in quantities established by peer-reviewed analysis and federal monitoring: naphthenic acids (designated among the most persistent and toxic components of oil sands process-affected water, with documented endocrine-disruption and narcosis effects); residual bitumen; polycyclic aromatic hydrocarbons; heavy metals; and a range of organic compounds whose cumulative toxicity to aquatic life and human health has been documented across multiple studies.

Downstream of the tailings complex, along the Athabasca River and into the Peace–Athabasca Delta and Lake Athabasca, live the members of Mikisew Cree First Nation, Athabasca Chipewyan First Nation, Fort McKay First Nation, the Fort Chipewyan Métis, and other communities whose treaty territories predate oil sands development by a century. The communities draw drinking water, harvest fish, hunt, and gather food and medicine from the same watershed that receives the runoff, seepage, and — in documented incidents — direct spill discharge from the tailings complex.

The Kearl disaster

In January 2023, Imperial Oil's Kearl oil sands site in northern Alberta experienced a tailings pond breach. Approximately 5.3 million litres of process-affected water escaped containment. The incident was not publicly disclosed for several days. When public disclosure followed, it included a second finding: a separate drainage system at the same facility had been leaking tailings into groundwater for at least nine months, entering the environment and migrating toward surface water. Indigenous communities downstream of Kearl — most immediately Fort Chipewyan, situated approximately 250 kilometres downriver — were not directly notified by either Imperial Oil or the Alberta Energy Regulator. They learned of the spill through the Alberta Energy Regulator's posting of an environmental protection order.

The community's response was immediate and practical. Melaine Dene, acting director of government and industry relations for Mikisew Cree First Nation, began coordinating emergency bottled water delivery before the winter ice road connecting Fort Chipewyan to the south melted. Chief Allan Adam of Athabasca Chipewyan First Nation, testifying at a federal parliamentary hearing two months after public disclosure, described the experience of turning on a tap in his own home in the days after the spill without knowing whether the water was safe: "I had to turn that tap on to cook my food. I drink two cups of coffee every morning. Where does that water come from? Knowing that turning that tap on could be detrimental to my health..."

In March 2024, Athabasca Chipewyan First Nation filed suit against the Alberta Energy Regulator over its handling of the Kearl incidents and alleged regulatory failures. The case remains before the courts. In April 2023, while the Kearl response was still unfolding, a separate incident at a Suncor settling pond released approximately 6 million additional litres into the Athabasca River.

The documented health consequences

As documented in Section 5 of this report, the April 14, 2026 Mikisew Cree First Nation community health study found approximately 25 percent higher cancer rates in Fort Chipewyan and surrounding communities than in the comparison Alberta population, with 149 cancer cases documented across 24 different cancer types in a community of 641 people. The specific cancer-type distribution is consistent with exposure to the organic compounds documented in oil sands process-affected water and in the environmental media of the Athabasca watershed. Dr. John O'Connor, the physician who first raised public concern about cancer rates in Fort Chipewyan, was professionally investigated for his work — a pattern documented in Section 5 as a case of professional retaliation against whistleblowers who identify environmental harm to Indigenous communities.

The Medscape analysis of First Nations water conditions (September 2024) documents the broader health profile of communities under long-term drinking water advisories: skin infections, higher-than-average cancer rates, and a documented trauma and mental-health burden including suicidality among young community members. Kumar and colleagues at the University of Manitoba have documented, in peer-reviewed analysis of bacterial diversity and the antibiotic-resistance profile of water stored in cisterns in Manitoba First Nations, the specific infectious-disease risk profile produced by the combination of contaminated source water, inadequate treatment, and cistern-based storage.

Alberta's "treat and release" proposal

As of 2025, the Government of Alberta, under Premier Danielle Smith and Environment Minister Rebecca Schulz, is actively pursuing a policy framework that would permit oil sands operators to treat tailings water and release the treated water into the Athabasca River. The proposal is advanced through an industry-dominated provincial committee with a single Indigenous

representative, in parallel to — and in effective competition with — the federal Crown-Indigenous Working Group (CIWG), which includes representatives of nine downstream Indigenous communities and was established in 2021 specifically to evaluate tailings management alternatives that protect Indigenous rights and health.

In October 2025, the Mikisew Cree First Nation, Athabasca Chipewyan First Nation, Keepers of the Water, Indigenous Climate Action, Environmental Defence, the Canadian Parks and Wilderness Society Northern Alberta, the Canadian Association of Physicians for the Environment, and the Council of Canadians (Edmonton Chapter) issued a joint statement rejecting the “treat and release” proposal. The joint statement characterized the Alberta provincial committee’s recommendations as undermining the federal CIWG process, bypassing established Indigenous consultation standards, and putting downstream communities and ecosystems at serious risk through reliance on unproven treatment technology. Chief Allan Adam of Athabasca Chipewyan First Nation commented: “This province works backwards.”

As Jesse Cardinal, executive director of the Athabasca Keepers of the Water, stated in correspondence to the federal Minister of Environment and Climate Change: “Releasing tailings to the watershed is not a safe or just option for solving Alberta’s growing tailings crisis. It is a handout to oil companies that allows them to avoid reclamation costs. It is a decision that [means] northern Indigenous communities and lands will remain a sacrifice zone for the profit of settler governments, southern populations and some of the world’s richest corporations.”

The Fisheries Act currently prohibits the deposit of deleterious substances into water bodies containing fish unless authorized by federal regulation. No such regulation currently exists for oil sands mines. The Alberta provincial proposal, to be operational, would require federal regulatory action to create that authorization. As of the publication of this report, no such authorization has been granted.

Part 4: Water as upstream cause

The direct pathway

The argument developed in Section 4 on housing — that on-reserve housing conditions are themselves upstream causes of the disability profile the AISH program adjudicates — applies with equal force to water. The causal pathways are established and peer-reviewed:

- Contaminated drinking water produces gastrointestinal infection, skin infection, dermatologic conditions, and in populations with compromised immune function, serious systemic illness.
- Boil-water advisory compliance produces chronic logistical and economic burden, sleep disruption in households with infants (formula preparation requires boiled water), and documented mental health and trauma effects across populations that have been under advisory for years or decades.

- Long-term exposure to organic compounds characteristic of oil sands process-affected water — naphthenic acids, polycyclic aromatic hydrocarbons, heavy metals — is associated in the peer-reviewed toxicology literature with endocrine disruption, hepatic injury, carcinogenesis (particularly biliary tract and bile duct cancers documented in the Mikisew Cree First Nation community health study), and cardiovascular effects.
- Water-borne industrial contamination in traditional territory produces secondary harms: loss of traditional food security, loss of cultural and ceremonial practices tied to water, and the community-wide chronic stress documented in the health literature as an independent predictor of mortality.

The AISH and ADAP intersection

An Indigenous disabled Albertan whose disability traces to any of the pathways documented above appears in the AISH system as a person with a qualifying medical condition. The cause of the condition is, from the perspective of the provincial adjudication process, irrelevant. The physician fills out the form; the recipient is approved or denied on the basis of medical severity and permanence; the benefit is paid, or clawed back, or transitioned to ADAP under the July 1, 2026 framework.

What the adjudication process does not record — because it is not designed to — is that the person's condition is frequently the downstream biological expression of federal and provincial policy decisions that this section has documented. The federal government failed to adequately fund water infrastructure on the applicant's reserve, and the applicant developed the condition as a result. The provincial government opposed the federal legislation that would have made the failure remediable in law. The same provincial government now administers the disability benefit the applicant requires and, as documented throughout this report series, administers it in a manner that minimizes federal disability recognition through CDB clawback, strips appeal rights through Bill 12, reduces the benefit rate through the ADAP transition, and reassesses eligibility under a standardized framework that makes no allowance for the Indigenous-specific causal pathway that produced the disability in the first place.

The loop closes. The population inside the loop does not.

What this section establishes for the remainder of the report

Water is not separate from health. Water is not separate from disability. Water is not separate from mortality. The 19-year mortality gap documented in Section 2 is, in significant measure, the cumulative biological outcome of lifetime exposure to the water conditions documented in this section. The children born into Neskantaga's thirty-year advisory, the community members of Fort Chipewyan drinking from a river downstream of 1.5 trillion litres of process-affected tailings, the families in Pimicikamak and Kashechewan waiting for months of infrastructure repair after a

January 2026 failure — these populations are living inside the conditions that produce the disability profile this report is written to serve.

The jurisdictional fracture documented in Section 3 is the administrative design that makes the water crisis persistent. The federal government is responsible for on-reserve water. The provincial government regulates industrial water and opposes federal legislation that would establish enforceable First Nations water rights. Neither level of government is answerable to the person who turns on a tap in Neskantaga, or who draws drinking water from Lake Athabasca 250 kilometres downstream of Kearl, and who lives with the consequences. The housing documented in Section 4 is built over the same ground the contaminated water runs through. The tailings documented in Section 5 sit in the same watershed that produces the drinking water documented here. None of these sections describes a separate crisis. They describe the same crisis in four interlocking dimensions.

Section 7 will address the dual healthcare system — the federal Non-Insured Health Benefits program and the provincial Alberta Health Services system — through which Indigenous Albertans must navigate when the water-borne, housing-borne, and land-borne harms documented across these sections become the medical conditions that bring them into contact with the disability benefit system. The pattern is continuous. The documentation is sustained. The victim population is the same population across all the sections.

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SECTION 7

The Dual Healthcare System

Two broken systems, one Indigenous patient: the Non-Insured Health Benefits program, Alberta Health Services, and what happens when 150 years of a treaty medicine chest promise becomes a 983,000-person federal program layered on top of a provincial primary care system that is collapsing in real time

Treaty 6 was signed in 1876. The medicine chest clause promised that a medicine chest would be kept at the house of each Indian agent for the use and benefit of the Indians. One hundred and fifty years later, the medicine chest has become a \$1.87 billion federal program with a three-level appeal process, routinely denied by private insurers acting as payer of first resort, declined by pharmacists unwilling to bill it, and operating alongside a provincial primary care system in which 650,000 Albertans have no family doctor at all. The promise persists. Its fulfilment does not.”

This section documents the healthcare reality facing Indigenous disabled Albertans. The structure parallels Sections 4 and 6: a jurisdictional fracture that places Indigenous health under two administratively separate systems, neither of which is adequate on its own and neither of which treats the failure of the other as its problem. The federal Non-Insured Health Benefits (NIHB) program, administered by Indigenous Services Canada and originally established as an evolution of the Treaty 6 medicine chest promise, provides supplementary coverage for prescription drugs, dental care, vision care, medical supplies, mental health counselling, and medical transportation. The provincial Alberta Health Services (AHS) system, administered under the Canada Health Act, provides physician services, hospital care, and emergency response. For a status First Nations person living off-reserve in Alberta, both systems must function in coordination. The documented record establishes that neither does.

Four findings structure the section. First, that the Treaty 6 medicine chest promise has been operationalized into NIHB in a form that the Standing Committee on Indigenous and Northern Affairs has itself identified as requiring eighteen specific reforms. Second, that NIHB functions as payer of last resort in a manner that creates administrative delay, pharmacy refusal, and out-of-pocket burden for the very population the program was established to serve. Third, that the Alberta provincial primary care system is in documented collapse, with 650,000 Albertans without a family physician, 887 accepting practices in 2020 reduced to 164 by 2024 — a collapse that affects all Albertans but compounds for Indigenous patients who must also navigate NIHB. Fourth, that anti-Indigenous racism in Canadian healthcare is not incidental but

documented across multiple public inquiries, peer-reviewed scoping reviews, and two named deaths — Brian Sinclair in 2008 and Joyce Echaquan in 2020 — whose circumstances have been formally recognized as the direct product of systemic bias in the provincial healthcare systems that deliver physician and hospital services to Indigenous patients.

Part 1: From the medicine chest to NIHB

The Treaty 6 medicine chest clause

Treaty 6 was signed in 1876 between the Crown and the Plain and Wood Cree, Assiniboine, and other peoples whose traditional territories include much of what is now central Alberta and Saskatchewan. Treaty 6 is the only numbered treaty that specifically references healthcare — the medicine chest clause, which provided that a medicine chest would be kept at the home of each Indian agent for the use and benefit of the Indians of the band. The clause was not an administrative footnote. It was a reciprocal promise offered by the Crown in exchange for the surrender of territory that, in 2026, produces the oil sands revenue and agricultural output that underwrites Alberta's provincial fiscal position.

Many First Nations groups have asserted that the medicine chest clause, taken together with the oral history of treaty negotiations and the broader fiduciary relationship between the Crown and First Nations, signals a constitutional obligation on the federal government to provide healthcare to Status First Nations persons. The legal interpretation has been contested; the political interpretation has been sustained. NIHB, administered through the First Nations and Inuit Health Branch of Indigenous Services Canada, represents the federal government's operational response to this obligation.

The scale of NIHB today

As of March 2025, NIHB provided coverage to approximately 983,000 eligible clients nationally. Federal spending on the program has averaged \$1.87 billion per year over the five years prior to the publication of this report. Coverage categories include: prescription drugs and over-the-counter medications; dental care; vision care; medical supplies and equipment (including wheelchairs, orthotics, prosthetics, hearing aids); short-term crisis intervention mental health counselling; and medical transportation to services not available on reserve or in the client's community of residence.

In Alberta and British Columbia specifically, NIHB also funds provincial health premiums for eligible clients. In British Columbia, the provincial-level coordination operates differently: the First Nations Health Authority (FNHA), established in 2013, administers federal and provincial First Nations health funding through a single Indigenous-led authority, and the FNHA has effectively replaced NIHB as the primary benefits administrator for Status First Nations persons in BC. No equivalent structure exists in Alberta. Alberta Indigenous clients continue to navigate NIHB through the standard federal administrative process.

What NIHB clients actually experience

The Standing Committee on Indigenous and Northern Affairs issued eighteen formal recommendations for NIHB program reform in 2022 — the product of hearings, testimony from healthcare professionals and Indigenous leaders, and a study of the program's documented shortcomings. These included: formal government recognition of traditional Indigenous healers; improvement of medical transportation benefit criteria; modernization of the approvals process to reduce delays; and structural reform of the program's payer-of-last-resort position. As of the publication of this report, the majority of those eighteen recommendations remain unimplemented.

The CBC Indigenous investigation published in January 2026 by journalist Joy SpearChief-Morris (2025 CJF-CBC Indigenous Journalism Fellowship recipient) documented the patient experience of navigating NIHB through the case of Patricia McCartney, a member of Six Nations of the Grand River in southern Ontario and primary caregiver for her elderly mother. McCartney reported that obtaining NIHB coverage for her mother's incontinence supplies — adult diapers, bed pads, catheters, dressing aids — required six to eight months of paperwork between the prescribing physician and the pharmacy. During that time, the mother's care needs did not pause. McCartney described the process as “so daunting” that many caregivers in her position simply give up and pay out of pocket.

Amy Lamb, Executive Director of Indigenous Pharmacy Professionals of Canada, documented the pharmacy-side reality in the same CBC reporting. Lamb stated that some pharmacies are refusing to bill NIHB at all because of the administrative burden. The program's position as payer of last resort — meaning NIHB pays only after provincial/territorial or private insurance has declined — creates documented problems in two directions. First, it produces administrative delay: when a client has any alternative coverage, that coverage must be pursued first, and only when it is declined can NIHB be billed. Second, it leads to program underfunding: because NIHB only pays for what others have declined, the actual cost of services delivered to Indigenous clients never appears fully in the NIHB budget. The Native Women's Association of Canada has documented, in its NIHB policy analysis, that Indigenous clients routinely pay out of pocket for costs not covered by NIHB, and that First Nations, Inuit, and Métis persons excluded from NIHB due to status criteria face the highest health costs of any group in Canada.

The payer-of-last-resort structure means NIHB only pays for what other insurance has already declined. Which means the actual cost of healthcare delivered to Indigenous clients never appears in the NIHB budget. Which means the program's stated expenditure of \$1.87 billion per year understates what Indigenous healthcare actually costs — because a significant portion of that cost is being absorbed by Indigenous

clients themselves, in the form of out-of-pocket payments and foregone care.”

Who NIHB excludes

NIHB eligibility is restricted to registered Status First Nations persons under the Indian Act and recognized Inuit persons. Non-status First Nations persons, Métis persons not covered by a separate Land Claim organization, and First Nations persons whose status was affected by the gender discrimination provisions of earlier versions of the Indian Act are not eligible for NIHB. As documented in Section 3 of this report, the jurisdictional fracture of Canadian Indigenous policy means that these excluded populations fall into a specific gap: they are too Indigenous to be comprehensively covered by provincial systems calibrated to the non-Indigenous population, and not Indigenous enough in the administrative sense to receive federal supplementary coverage. The result is a documented health cost differential that Native Women’s Association of Canada identifies as the highest in Canada for this specific population.

Métis and non-status First Nations persons with complex medical conditions or disabilities must navigate Alberta Health Services as their primary system of coverage, with no parallel supplementary coverage for medications, dental care, vision care, or medical supplies and equipment beyond what provincial Health Benefits Cards provide. For AISH recipients specifically, this differential is significant: the AISH Health Benefits Card covers some but not all supplementary benefit categories, and the gap for a Métis or non-status AISH recipient is documented in the Alberta Income and Employment Supports Policy Manual as explicitly different from the gap for a Status First Nations person, whose federal NIHB coverage operates alongside provincial benefits.

Part 2: The provincial side — Alberta’s primary care collapse

What 650,000 without a GP means

The Alberta Disability System Breakdown Report Series Document 5 of 12, the Healthcare Worker Shortage Report, documented the state of Alberta primary care as of April 2026. The findings, drawn from the Alberta Medical Association, University of Calgary research, ThinkHQ polling of family physicians, and peer-reviewed publication in the Canadian Family Physician journal:

- More than 650,000 Albertans have no family doctor — a population larger than the entire city of Calgary.
- The number of family practices accepting new patients fell from 887 in 2020 to 164 in 2024 — an 81 percent reduction in four years.
- 61 percent of existing family physicians are considering leaving the Alberta provincial healthcare system. Over 25 percent are considering leaving healthcare entirely.

- 4,374 family physicians were registered in Alberta as of the first quarter of 2024, 215 more than the same period the previous year — yet they are not taking new patients. The paradox is structural, not arithmetic: Alberta has more registered family physicians than ever, and fewer accepting patients than ever.
- The Canadian Family Physician journal in 2025 estimated that an additional 678 to 715 family physicians — or alternatively 722 nurse practitioners — would be required to close the Alberta primary care gap.
- Dr. Braden Manns of the University of Calgary, lead investigator on Alberta primary care research published in September 2025, stated: “People are older and sicker. The number of people with five or more health conditions — like a history of cancer, high blood pressure and heart issues — nearly doubled.”

In the Bow Valley region, the nearest GP accepting new patients was a 90-minute drive away as of early reporting. Daphne Pauls, a 53-year-old Alberta resident living with fibromyalgia whose family doctor left the province, told CBC News: “Not knowing is difficult. Consistent care is key for maintaining my health. It just leaves me kind of not knowing how will my health be cared for.”

What primary care collapse means for Indigenous patients specifically

For an Indigenous person in Alberta — Status or non-status, on-reserve or off — the provincial primary care collapse produces several specific harms. First, NIHB coverage requires a prescribing physician for most benefits. No GP means no prescriber for ongoing medications. Walk-in physicians, as documented in the Alberta Disability System Breakdown Healthcare Worker Shortage Report, routinely decline to complete complex disability forms for patients they have seen once. Second, specialist referrals require a GP. Without a GP, an Indigenous patient with a chronic condition arising from the environmental exposures documented in Sections 5 and 6 has no pathway to the specialist care that condition requires. Third, AISH, ADAP, and Disability Tax Credit applications all require physician-completed forms. An Indigenous person whose disability traces to contaminated water, inadequate housing, or multigenerational trauma documented in Section 1 cannot access the provincial disability benefit the condition qualifies them for, because the condition is real but the paperwork pathway is unavailable.

Rural reserves compound this barrier. Of the Alberta First Nations communities identified in the Indigenous Services Canada catalogue, a significant portion are located more than 90 minutes from an urban centre with primary care capacity. For an Indigenous person living on reserve and requiring off-reserve specialist care, the Section 4 housing instability, the Section 6 water contamination, and the Section 7 primary care collapse converge: the reason they need the specialist is produced by the housing and water situation; the primary care pathway to the specialist does not exist; and the transportation benefit that would bring them to the specialist is administered by NIHB's payer-of-last-resort system, which requires documentation from the physician they do not have.

Part 3: Anti-Indigenous racism as a documented feature of Canadian healthcare

Two deaths, twelve years apart, same pattern

The peer-reviewed literature on anti-Indigenous racism in Canadian healthcare, the official coroner's findings on two specific deaths, and the Turpel-Lafond 2020 "In Plain Sight" report commissioned by the Government of British Columbia establish, with primary-source documentation, that anti-Indigenous racism is a structural feature of the provincial healthcare systems that deliver physician and hospital care across Canada. The two named cases are not exceptional incidents. The peer-reviewed scoping review literature characterizes them as representative of a pattern.

Brian Sinclair, a 45-year-old First Nations man and double amputee, entered the emergency room at Winnipeg's Health Sciences Centre in September 2008 seeking care for a treatable bladder infection. He waited 34 hours. During that time, hospital staff did not ask him why he was there. Nurses did not assist him even as he vomited on himself. He was eventually found dead in his wheelchair. The inquest into his death concluded he had never been asked if he was waiting for medical care. The book *Structures of Indifference: An Indigenous Life and Death in a Canadian City*, co-authored by Mary Jane Logan McCallum and Adele Perry, analyzed the systemic factors that produced this outcome.

Joyce Echaquan, a 37-year-old Atikamekw mother of seven, entered the medical centre in Joliette, Quebec in September 2020 with stomach pains. She recorded herself on Facebook Live as nurses insulted her with racist and degrading comments while she cried out in pain. One nurse called her "stupid as hell." She died shortly afterwards. The 2021 final report from the Quebec coroner's inquest concluded that racism and prejudice from hospital staff contributed to her death. In July 2023, the Manawan Atikamekw Council, representing Joyce Echaquan's nation, opened the Joyce's Principle Office, which works to embed Joyce's Principle — an Indigenous-led framework for culturally safe healthcare — into Canadian health and social service practice.

Mary Jane Logan McCallum observed, following Joyce Echaquan's death, that the pattern was the same as Brian Sinclair's. Twelve years had passed. The public inquiries had been convened. The recommendations had been tabled. The pattern was the same.

The peer-reviewed documentation

The peer-reviewed literature on anti-Indigenous racism in Canadian healthcare is substantial and the findings are consistent. The key sources include:

- Turpel-Lafond, Mary Ellen. "In Plain Sight: Addressing Indigenous-specific Racism and Discrimination in B.C. Health Care." Government of British Columbia. 2020. The report followed allegations of a "price is right" game in which BC emergency department staff guessed Indigenous patients' blood alcohol levels. The resulting provincial review documented

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- Adegoke, Kola, Abimbola Adegoke, Deborah Dawodu, and Temitope Kayode. “Systemic Racism in Canadian Healthcare: A Policy and Equity Analysis.” *Journal of Primary Care & Community Health*. 2025. Qualitative analysis of public inquiries, government audits, and case files across four central areas: institutional discrimination, licensing and workforce exclusion, patient and cultural safety, and accountability gaps. The analysis finds continuing institutional disregard for Indigenous patients, most marked in emergency and maternal services.

The Chadha 2020 commentary, cited repeatedly in this literature, frames the specific intersection this section addresses: the combination of Indigenous identity and disability produces compounded harm in healthcare encounters. Chadha’s commentary is titled “Why it’s dangerous to be disabled and Indigenous in Canada.” That is not a rhetorical claim. It is the characterization of a documented pattern drawn from primary evidence.

The peer-reviewed literature names it directly. It is dangerous to be disabled and Indigenous in Canada. Not because the disability is dangerous. Because the healthcare system, as documented across multiple provinces, multiple inquiries, and multiple decades, produces harm to disabled Indigenous patients through structural and interpersonal bias that has been studied, named, and insufficiently changed.”

Part 4: The intersection for Indigenous disabled Albertans

What it takes to navigate both systems simultaneously

An Indigenous disabled Albertan — the specific population this report documents — must navigate both systems simultaneously. The following is what that navigation involves on a

typical month, assuming moderate complexity of medical need (multiple chronic conditions, at least one specialist, prescription medications, and some category of medical supply):

- First: locate a family physician. In a province with 650,000 without a GP and 164 accepting practices as of 2024, this may require months of active searching or may be simply unavailable in the patient's community of residence.
- Second: if the physician is found, schedule a visit. For a Status First Nations person, the visit is billed to the provincial AHS system; this is not a NIHB transaction. If prescription medications are prescribed, the physician's prescription goes to a pharmacy that must attempt to bill NIHB — after first attempting to bill any other coverage the patient may have.
- Third: if the patient requires specialist care, the GP must initiate a referral. Specialist wait times in Alberta are routinely six to eighteen months for non-urgent specialties. During that wait, the underlying condition continues.
- Fourth: if the patient requires medical transportation to a specialist not available in their community, a separate NIHB medical transportation request must be submitted, approved under the NIHB Medical Transportation Policy Framework, and coordinated through an NIHB regional office or a First Nations/Inuit health authority. Denial of medical transportation is appealable through a three-level process. The appeal must be submitted in writing.
- Fifth: if the patient requires medical supplies or equipment — a wheelchair, orthotics, a hearing aid — a separate NIHB request is submitted, subject to prior approval, and bill-able to NIHB only after other coverage has declined.
- Sixth: if the patient is an AISH or ADAP recipient, a physician-completed medical report is required for provincial disability benefit eligibility — an additional form the physician must complete, which may or may not be billed to the patient personally as a fee-for-service charge under Alberta's primary care reform framework.
- Seventh: if the patient encounters racism or inadequate care at any stage in the encounter, the formal complaint mechanisms available are the College of Physicians and Surgeons of Alberta, the Alberta Human Rights Commission, the Office of the NIHB Regional Medical Officer, or — for BC residents under the FNHA — the In Plain Sight implementation framework. None of these mechanisms is designed for rapid response. None of them prevents the harm that has already occurred.

Each of these seven steps functions in isolation. None of them coordinates with the others. A missed step at any stage can render the subsequent steps inaccessible. For a patient with the cognitive, communication, or literacy profile that defines a significant portion of the AISH-qualifying population, the cumulative navigation burden is structurally prohibitive.

The ADAP transition and the Indigenous patient

The July 1, 2026 ADAP transition adds a further dimension. As documented throughout this report series, ADAP requires all 79,290 current AISH recipients to undergo reassessment and, if remaining on AISH, to obtain a physician-completed Disability Assistance Medical Report. For an Indigenous patient without a family physician, operating within the NIHB payer-of-last-resort framework, the physician-completed form requirement is not a procedural step. It is a structural barrier that, in combination with the primary care collapse documented in Part 2 and the racism documented in Part 3, may prevent continuation of the disability benefit that the patient's condition qualifies them for under the program's own written criteria.

The Indigenous patient whose disability traces to the causes documented in Sections 1, 4, 5, and 6 of this report — residential schools, inadequate housing, contaminated land, poisoned water — faces a cascade: the disability is real, the causal pathway is documented, the federal program that ought to cover supplementary needs (NIHB) operates as payer of last resort and is declined by pharmacies and denied in the McCartney pattern, the provincial primary care system that ought to document the condition is collapsed at 164 accepting practices, the provincial disability benefit that ought to support the recipient is being transitioned under ADAP with a physician-completed form requirement the patient cannot fulfill, and the entire cascade is administered by two levels of government neither of which treats the failure of the other as its responsibility.

What this section establishes for the remainder of the report

The Treaty 6 medicine chest promise has been operationalized into NIHB, a federal program now serving approximately 983,000 Status First Nations and Inuit clients at an annual cost of \$1.87 billion. Eighteen formal reform recommendations from the Standing Committee on Indigenous and Northern Affairs remain largely unimplemented. NIHB functions as payer of last resort, producing documented administrative delay, pharmacy refusal, and out-of-pocket burden. The provincial Alberta Health Services system is in documented primary care collapse, with 650,000 Albertans without a GP and the number of accepting practices reduced 81 percent in four years. Anti-Indigenous racism in the provincial healthcare systems that deliver physician and hospital care is peer-reviewed, inquiry-documented, coroner-confirmed, and named in the specific cases of Brian Sinclair (2008) and Joyce Echaquan (2020) as contributing to their deaths. For Indigenous disabled Albertans navigating both systems simultaneously, the cumulative structural burden is prohibitive.

The pattern across Sections 4, 5, 6, and 7 is now consistent. Housing that federal funding has failed to make adequate. Land that federal and provincial regulatory inaction has allowed to be contaminated. Water that federal advisories and provincial resource extraction have rendered unsafe. Healthcare that dual federal-provincial administration delivers to Indigenous patients with documented racial bias and structural barrier. Each of these four sections describes a separate dimension of the conditions under which Indigenous disabled Albertans live. None of them describes an isolated problem. All of them describe the operating environment of the same

population, a population whose life expectancy — as documented in Section 2 — is 19 years below the non-First Nations Alberta average.

Sections 8 through 15 will address the remaining dimensions: Jordan's Principle and children's services, child welfare, the justice system, MMIWG2S+, consultation failure and UNDRIP Article 21, the Alberta Sovereignty Act and its relationship to Treaty, TRC Calls to Action 18 through 24, and a synthesis of the July 1, 2026 ADAP transition in its Indigenous dimension. The population the report documents is the same throughout. The structural pattern the report documents is the same throughout. The remedy this report calls for is the same throughout: an honest federal and provincial acknowledgment that the outcomes documented here are not administrative accidents but the cumulative product of policy choices that can be made differently.

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SECTION 8

Jordan's Principle

The child whose death named the principle, the twenty-year legal compliance battle, the 140,000-application backlog, the February 2025 operational bulletin that cut services to First Nations children while they were waiting, and why this matters specifically for Indigenous disabled children in Alberta

Jordan River Anderson was cleared by his doctors to leave hospital at the age of two. The federal and provincial governments argued for three more years over who would pay for his home care. He died in the hospital at the age of five, having never seen his family's home on Norway House Cree Nation. The principle named for him was supposed to guarantee that no First Nations child would ever again be denied care while governments argued about jurisdiction. Twenty years later, 140,000 requests are in backlog, denials doubled from \$216 million to \$491 million in a single fiscal year, and the federal government's own February 2025 operational bulletin restricted what First Nations children can even apply for."

This section documents the Jordan's Principle file as it stands in April 2026. Section 3 established Jordan's Principle as the jurisdictional fracture case study in this report. Section 8 documents the substance: the child whose death named the principle, the twenty-year legal compliance battle between First Nations advocates and the federal government, the scale of the current backlog, the February 2025 operational bulletin that functioned as a service reduction, the documented impacts on Indigenous disabled children specifically, and the Alberta-specific dimensions — including the education assistant layoffs in Alberta and Saskatchewan school divisions that followed the February 2025 policy shift.

Four findings structure the section. First, that Jordan's Principle is not a discretionary federal program but a legal obligation imposed on Canada by a sustained series of Canadian Human Rights Tribunal orders since 2016, following the 2007 unanimous House of Commons motion M-296. Second, that the federal government has been found in non-compliance with those orders repeatedly and remains in non-compliance as of the publication of this report. Third, that the February 10, 2025 operational bulletin issued by Indigenous Services Canada, which narrowed the scope of services eligible under Jordan's Principle, coincided with a doubling of denial values from \$216 million in fiscal 2023-24 to \$491 million in fiscal 2024-25, and a decline in total approved services for the first time since 2020. Fourth, that these administrative changes, together with the ongoing backlog, produce documented harm to Indigenous disabled

children — the same population whose disability outcomes Sections 1 through 7 have established are structurally produced by federal and provincial policy failure.

Part 1: Jordan River Anderson

The child

Jordan River Anderson was born in 1999 in Norway House Cree Nation, a community on the northeast shore of Lake Winnipeg in Manitoba. He was born with Carey-Fineman-Ziter syndrome, a rare genetic muscular disorder producing complex medical needs. From birth, Jordan required hospital-level care. He was admitted to the Winnipeg Children's Hospital, more than 800 kilometres from his family's home.

At age two, Jordan's condition was stabilized. His medical team at the Winnipeg Children's Hospital determined that his needs could be met in a specialized home near the hospital, closer to his family. The medical determination was made. The clearance to leave the hospital was given. The pathway home required the funding of his at-home specialized care. That funding required the federal government and the Manitoba provincial government to agree on who would pay.

They did not agree. The federal government took the position, under section 91(24) of the Constitution Act 1867, that Indians and lands reserved for Indians were federal jurisdiction but that home-based services on or off reserve were not clearly covered by specific federal programs. The Manitoba provincial government took the position, under section 92 of the Constitution Act 1867, that health services were provincial jurisdiction but that services delivered to Status First Nations persons, particularly those ordinarily resident on reserve, were federal responsibility. Both positions had legal basis. Neither position produced care for Jordan.

The dispute continued for three years. During those three years, Jordan remained in the Winnipeg Children's Hospital. His care needs did not pause. His family could not bring him home. His father Ernest Anderson, his mother, his five sisters and six brothers visited when they could. Jordan never went home. On February 2, 2005, at the age of five, Jordan River Anderson died in the Winnipeg Children's Hospital.

Jordan River Anderson was medically cleared to leave the hospital three years before his death. The reason he remained in the hospital for those three years was not medical. It was jurisdictional. He died inside the dispute between two levels of government over whose responsibility he was. He was five years old. He never saw Norway House."

His sister Jerleen Sullivan: "He is our hero."

In 2025, Jordan's sister Jerleen Sullivan spoke at a memorial gathering for her brother: "I want everybody to remember that Jordan was loved by his whole family. He had a mother, he had a father, he has 5 sisters and 6 brothers and we all cared about him very much and we all wanted him back home. We see Jordan's spirit everywhere, in all these children that gathered here today. He is fighting from the spirit world, he is a spirit warrior. He is our hero and he fights for every First Nations child in Canada."

Jordan's father Ernest Anderson has continued to advocate for the principle named after his son. In February 2025, following the Indigenous Services Canada operational bulletin narrowing Jordan's Principle scope, Ernest Anderson recorded a video statement: "We need to talk. We need some answers. We need a meeting." The Anderson family has joined national First Nations leaders in calling for emergency federal response to the state of the program named for their son.

Part 2: From motion to legal obligation

Motion M-296 (2007)

In 2007, two years after Jordan's death, NDP MP Jean Crowder introduced a motion in the House of Commons. Motion M-296 stated that the government should immediately adopt a child-first principle, based on Jordan's Principle, to resolve jurisdictional disputes involving the care of First Nations children. The motion passed unanimously on December 12, 2007. The unanimous passage was significant: every party in the House of Commons, government and opposition, agreed that the circumstances of Jordan's death must not recur.

The motion was not, however, binding law. It was a statement of parliamentary intent. Between 2007 and 2016, First Nations advocates documented continuing jurisdictional disputes affecting specific children. The First Nations Child and Family Caring Society, led by Dr. Cindy Blackstock, together with the Assembly of First Nations, filed a complaint with the Canadian Human Rights Tribunal arguing that Canada's continued administrative failure to implement Jordan's Principle constituted racial discrimination against First Nations children.

The 2016 CHRT ruling and the non-compliance decade

On January 26, 2016, the Canadian Human Rights Tribunal released its decision in First Nations Child and Family Caring Society v Canada. The Tribunal found that the Government of Canada's implementation of Jordan's Principle resulted in discrimination against First Nations children and youth on the basis of race and national ethnic origin. The Tribunal ordered Canada to "cease applying its narrow definition of Jordan's Principle and to take measures to immediately implement the full meaning and scope of Jordan's Principle."

The 2016 ruling established that Jordan's Principle was not a discretionary program subject to federal budget constraints. It was a legal obligation of the Government of Canada arising from the Crown's fiduciary duty to First Nations children and from the anti-discrimination framework of

the Canadian Human Rights Act. The Tribunal was explicit: Jordan's Principle is not a fixed-budget program. Funding must expand with the number of eligible children. Services cannot be denied on the basis of cost.

Between 2016 and 2026, the Canadian Human Rights Tribunal has issued numerous remedial non-compliance orders against Canada for failing to abide by the original decision. The Tribunal's December 2024 ruling, in response to a non-compliance motion filed by the Caring Society early in 2024, ordered Canada to address the 140,000-application backlog and to establish an independent complaints mechanism. The federal government's public response, reported in CBC News, was that the order would be "challenging" to implement. The Caring Society subsequently filed further non-compliance motions.

As of April 2026, Canada remains in legal non-compliance with Canadian Human Rights Tribunal orders on Jordan's Principle dating back to 2016. The non-compliance is not contested. The federal government has not argued that it is in compliance. It has argued, variously, that compliance is administratively difficult, that the scope of Jordan's Principle has become too broad, and that budget pressures require service restriction.

Canada is choosing not to comply with the legal orders, and that means disruptions in services, denials, long wait times, and what we're really seeing from Canada is a lot of excuses, instead of compliance. — Dr. Cindy Blackstock, Executive Director, First Nations Child and Family Caring Society, The Tyee, April 25, 2025"

Part 3: The February 2025 operational bulletin and what it produced

The bulletin

On February 10, 2025, Indigenous Services Canada, under then-Minister Patty Hajdu, issued an operational bulletin narrowing the scope of services eligible under Jordan's Principle. The bulletin barred funding approvals for categories of request that had previously been approved under the program. The excluded categories specifically included:

- Home renovations, environmental and home repairs, and home modifications
- Sporting events, elite sports training, and athletic fees
- International travel
- Non-medical supports generally
- School-related requests, including education assistants, unless "linked to the specific health, social or educational needs of the First Nations child" and required to ensure substantive equality with children who are not First Nations

The stated rationale, drawn from the Canadian Human Rights Tribunal's own "substantive equality" framework, was that Jordan's Principle should provide services to ensure First Nations children receive equal treatment with non-Indigenous children — not services that non-Indigenous children would not ordinarily receive. The practical effect, documented across multiple First Nations communities and confirmed in APTN News reporting of October 2025, was a categorical exclusion applied before individual circumstances were assessed.

Critically, as documented by Cindy Blackstock in access-to-information materials obtained by the Caring Society: at the time the February 2025 bulletin invoked "substantive equality" as its rationale, Indigenous Services Canada had no working definition of "substantive equality" on file. The term had been removed from the department's public website. The legal concept being used to deny services to First Nations children was operationally undefined within the department issuing the denials.

What the bulletin produced

APTN News obtained, through an access-to-information request, internal Indigenous Services Canada data showing the financial impact of the February 2025 operational bulletin. The data covered the fiscal year ending April 30, 2025:

- Funding for Jordan's Principle declined for the first time since 2020, from \$1.72 billion in fiscal 2023-24 to \$1.64 billion in fiscal 2024-25 — a decline of approximately \$83 million.
- The value of denials more than doubled in the same period, from \$216 million to \$491 million — an increase of approximately \$275 million.
- The application backlog, which was 140,000 when the operational bulletin was issued, did not meaningfully decrease. New applications continued to arrive. Denied applications were not processed further.

The arithmetic is straightforward. In the fiscal year during which the federal government claimed to be addressing the backlog through administrative reform, approved services declined by \$83 million and denied services increased by \$275 million. The net redirection of funding away from First Nations children in a single fiscal year was approximately \$358 million.

In the fiscal year Indigenous Services Canada said it was reforming Jordan's Principle to address the backlog, approved services declined by \$83 million and denials increased by \$275 million. The net redirection of funding away from First Nations children was approximately \$358 million in a single year."

The Alberta dimension — school division layoffs

Between December 2024 and February 2025, as the operational bulletin was being prepared and then issued, public school districts in Alberta and Saskatchewan announced layoffs of

hundreds of education assistants. The positions had been funded through Jordan's Principle approvals supporting First Nations children with educational needs. When the operational bulletin narrowed the eligible categories for school-related requests, the underlying approvals supporting those education assistant positions were affected. Saskatoon Public Schools announced the ending of contracts for approximately 80 temporary education assistants. Alberta school divisions announced comparable reductions. The children whose educational supports had been funded did not stop needing the supports. The positions funding those supports simply ended.

This pattern — First Nations children losing educational support in provincial school systems because federal Jordan's Principle approvals narrowed — illustrates the jurisdictional fracture from Section 3 in operational form. Education in Canada is a provincial responsibility under section 93 of the Constitution Act 1867. Indigenous children's federal status places them under section 91(24). The educational needs of an Indigenous child in an Alberta school sit at the exact intersection where neither level of government takes unambiguous responsibility. Jordan's Principle, in its original scope, was one of the few mechanisms by which that intersection was addressed in practice. The narrowing of Jordan's Principle did not eliminate the intersection. It eliminated the mechanism of response.

Part 4: What this means for Indigenous disabled children specifically

The population

The Caring Society's documentation establishes that approximately 165,000 Status First Nations children and youth are affected by Canada's Jordan's Principle administration. Within that population, Indigenous children with disabilities represent a disproportionate share of Jordan's Principle applications — because their service needs are more numerous, more complex, and more subject to jurisdictional dispute. An Indigenous child with a rare genetic condition, an autism spectrum profile, a severe medical need, or a combination of these, requires coordinated services across health, education, and social supports. Each of those three categories sits at a different point on the federal-provincial responsibility map. Each interaction between the categories is a potential jurisdictional dispute. Jordan's Principle, in its fullest operational form, was designed to bypass those disputes and ensure the child received the service.

The February 2025 operational bulletin's narrowing of eligible categories fell disproportionately on children whose needs crossed the narrowed boundaries. A child whose disability required home modifications — widened doorways, wheelchair-accessible bathrooms, ramps — lost eligibility. A child whose disability required specialized therapeutic recreation, including programs categorized by the department as "sporting," lost eligibility. A child whose disability required educational support that the provincial school system could not or would not fund lost eligibility when the request was categorized as "school-related."

Specific documented cases

The documented record of Jordan's Principle denials since February 2025 includes:

- Hanlon Thompson, a 16-year-old from Ochapowace First Nation in southeastern Saskatchewan, whose mother Kayla Thompson has filed a discrimination complaint with the Canadian Human Rights Commission against Indigenous Services Canada. Hanlon Thompson was repeatedly denied Jordan's Principle funding for life-saving nutritional needs. (APTN News, 2025)
- The Cully family, whose daughter Scarlet, age 4, received applied behaviour analysis therapy for autism spectrum disorder in Thunder Bay, Ontario. The therapy was funded under Jordan's Principle. Father Patrick Cully stated that prior to the therapy, he was uncertain his daughter would ever be able to say "I love you." She is now talking. The Cully family is in federal court contesting the February 2025 operational bulletin. (CBC News, 2025)
- The Powless case, involving Joanne Powless, a grandmother from Oneida Nation of the Thames near London, Ontario, who applied for approximately \$200,000 through Jordan's Principle in 2022 for home-based care for her grandchildren. The application was denied following the February 2025 bulletin. The Federal Court ruled in July 2025 that Canada had acted "unreasonably" in denying a related home repair request. The Powless case remains before the Federal Court. (CBC News, Indigenous Watchdog, 2025)
- The Wolastoqey family in New Brunswick, whose autistic child's services were withdrawn following the February 2025 bulletin, leaving the family "in crisis mode" (APTN News, July 23, 2025).
- Multiple Yukon First Nations communities, where a five-year-old nutrition program feeding Indigenous children in rural communities ended in September 2025 after losing Jordan's Principle funding.
- Jordan's Principle regional offices themselves closing due to the funding cuts: the Jordan's Principle office in Brandon, Manitoba, closed in August 2025 after six months of operation.

David Taylor, the lawyer representing both the Cully and Powless families, observed in APTN News coverage: "Essentially, rather than looking at the children's actual needs, there's a categorical exception being applied at the outset. It's a kind of broad measure that has affected these two families in very different circumstances." The framework requires individualized assessment of each child's need under the substantive equality standard. The February 2025 bulletin, by applying categorical exclusions before individualized assessment, operates outside the framework the Canadian Human Rights Tribunal established.

Part 5: The \$70 billion settlement and what Alberta's own policy acknowledges

The 2025 class settlement

Separate from the Canadian Human Rights Tribunal proceedings, a class action lawsuit was filed against Canada on behalf of First Nations children harmed by the Jordan's Principle implementation failure and the First Nations Child and Family Services funding shortfall. In 2024 and 2025, the class action produced the First Nations Child and Family Services, Jordan's Principle and Trout Class Settlement Agreement. The settlement included \$23.4 billion in compensation for affected class members and additional amounts for long-term reform, totalling settlement obligations on Canada in excess of \$70 billion.

The first claims period opened March 10, 2025. The settlement represents, in financial terms, the largest compensation package Canada has paid for a sustained policy failure affecting First Nations children. The existence of the settlement is an operational acknowledgment by the federal government that the harm occurred. The ongoing non-compliance with Canadian Human Rights Tribunal orders on Jordan's Principle, even after the settlement, establishes that the settlement compensates past harm without remedying ongoing harm.

The Alberta administrative acknowledgment

The Government of Alberta's own Income and Employment Supports Training and Health Benefits (IESTHB) Policy Manual formally recognizes the First Nations Child and Family Services, Jordan's Principle and Trout Class Settlement Agreement as a "Harm Done" compensation payment under Ministerial Order 2026-004. Settlement payments received by class members are fully exempt from Alberta income support calculations.

The structural significance is the same as the analogous finding in Section 6 regarding the First Nations Drinking Water Advisory Class Actions Settlement. Alberta's own written policy incorporates, into its provincial administrative framework, the legal acknowledgment that federal policy failure in Jordan's Principle administration constitutes a "harm" for which settlement compensation is being paid. The province operationally recognizes the federal government's liability for the specific harm. That same province then operates, within its own jurisdiction, the school system layoffs and the provincial primary care collapse that compound the Jordan's Principle failure for Indigenous children in Alberta. The provincial acknowledgment of federal harm does not produce provincial action to prevent compounding of the harm.

Part 6: Jordan's Principle and the life course of Indigenous disability

This report documents the life course of disability for Indigenous Albertans. Section 1 addressed the historical production of disability through residential schools and Indian hospitals. Section 4 addressed housing conditions as upstream cause. Section 5 addressed contaminated land. Section 6 addressed water. Section 7 addressed the dual healthcare system. Section 8 now addresses what happens at the earliest stage of that life course: the Indigenous child whose complex medical, developmental, or disability needs place them in front of the Jordan's Principle application system before they can speak for themselves.

The Indigenous disabled Albertan who reaches AISH adjudication at age 18 and beyond has frequently come through the Jordan's Principle system as a child. Whether their application was approved, denied, delayed for two years in a backlog, or affected by the February 2025 operational bulletin shapes the medical record, the educational record, the developmental trajectory, and the family's financial capacity that precedes their eventual disability benefit application. A child denied applied behaviour analysis therapy at age four arrives at age 18 with different function, different developmental outcomes, and different documented medical history than a child whose therapy was approved. The provincial AISH adjudication process does not ask what happened to the applicant at age four. The federal Jordan's Principle decision at age four, however, has already shaped what the adjudication will see.

Dr. Cindy Blackstock's sustained argument — that Canada's non-compliance with Canadian Human Rights Tribunal orders represents an ongoing, documented, legally established pattern of discrimination against First Nations children — is the early-life version of the argument this report makes about the adult AISH and ADAP populations. The federal government's administrative failure at age four, compounded across the life course, becomes the disability profile at age 40 that enters the provincial benefit system. The remedy at age 40 is provincial. The harm at age four was federal. Neither government treats the other's failure as its own responsibility. The Indigenous person with a disability lives through both.

A child denied services at four is a teenager denied supports at fourteen is an adult denied disability benefits at forty. The administrative bulletins change. The population does not. The cumulative failure is not a series of separate failures. It is one sustained failure documented across the life course."

What this section establishes for the remainder of the report

Jordan's Principle is a legal obligation of the Government of Canada, established through Canadian Human Rights Tribunal rulings since 2016 and named after a five-year-old boy who died inside a jurisdictional dispute in 2005. Canada is in sustained legal non-compliance with those rulings. The February 10, 2025 operational bulletin narrowed the scope of services eligible under Jordan's Principle, producing a doubling of denial values in a single fiscal year, an \$83 million decline in approved services, and specific documented harms to named Indigenous disabled children whose cases are now in federal court. A \$70 billion-plus class settlement acknowledges the past harm. The non-compliance continues.

For Indigenous disabled children in Alberta specifically, the file compounds. The February 2025 bulletin's effect on school-related requests produced education assistant layoffs in Alberta and Saskatchewan school divisions. The provincial primary care collapse documented in Section 7 affects the physicians whose sign-off is required for Jordan's Principle medical supports. The

housing and water conditions documented in Sections 4, 5, and 6 produce the disabilities that bring Indigenous children into Jordan's Principle in the first place. And the population affected in childhood becomes the adult population this report's broader series has documented: AISH recipients facing ADAP transition, CDB clawback, appeal-rights stripping, and reassessment under standardized criteria that do not acknowledge the federal and provincial history that produced their disability.

Sections 9 through 15 will address child welfare, the justice system, MMIWG2S+, consultation failure, Treaty relationship and the Sovereignty Act, TRC Calls to Action 18 through 24, and the July 1, 2026 ADAP transition synthesis. The pattern documented in Section 8 — federal legal obligation, sustained federal non-compliance, compounding provincial administrative effect, and specific harm to named Indigenous individuals — recurs in each of those dimensions. The population is the same. The structure is the same. The documentation is the same. What changes across the sections is only the specific administrative program and the specific level of government through which the same pattern is expressed.

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SECTION 9

Child Welfare

The Millennial Scoop: how Alberta's provincial child intervention system apprehends Indigenous children at 72 percent of its caseload while representing only 10 percent of the youth population, what the 2024 rejected \$47.8 billion federal settlement reveals about the disagreement, and how disabled Indigenous parents are placed at specific risk of apprehension by the same system that administers the disability benefit keeping their children out of it

In 2025, there are more Indigenous children in the Canadian child welfare system than there were in residential schools at the system's height. Researchers have named this the Millennial Scoop. The Government of Alberta's own 2024-25 Children and Family Services Annual Report confirms the pattern in its own numbers: 72 percent of the provincial child intervention caseload is Indigenous, in a province where Indigenous youth represent approximately 10 percent of the youth population. The children are different. The policy framework looks different. The removal mechanism is different. The outcome is the same."

This section documents the child welfare system as it operates for Indigenous families in Alberta. The section builds on three previous pieces of this report: Section 1's documentation of the residential school system and the Sixties Scoop; Section 3's jurisdictional fracture framework; and Section 8's Jordan's Principle analysis. What Section 9 adds is the specific provincial administrative system — Alberta Children and Family Services — through which Indigenous child apprehension occurs today, the scale of that apprehension as documented by the provincial government's own reporting, and the structural risk the system poses specifically to disabled Indigenous parents whose disabilities are themselves the product of the upstream causes Sections 4 through 7 have established.

Four findings structure the section. First, that Indigenous children are overrepresented in the Canadian child welfare system at ratios the peer-reviewed literature and Truth and Reconciliation Commission have characterized as the Millennial Scoop — a continuation rather than a replacement of the residential school and Sixties Scoop pattern. Second, that Alberta specifically holds the highest disparity rate in Canada: Indigenous children at 28.9 percentage points higher than non-Indigenous children in foster care, driven by Alberta's combination of the second-highest Indigenous foster child rate and the lowest non-Indigenous foster child rate nationally. Third, that the federal \$47.8 billion First Nations Child and Family Services settlement

agreement was voted down by 267 First Nations chiefs and proxies in October 2024, on grounds documented by the First Nations Child and Family Caring Society as failing to remedy the structural underfunding that produces the apprehension pattern. Fourth, that the child welfare system structurally disadvantages disabled Indigenous parents, whose disabilities are frequently themselves evidence used against them in apprehension proceedings, and whose reduced income, contested housing, and collapsed healthcare access — all documented in Sections 4 through 7 — operate as risk factors in the provincial intake framework.

Part 1: What Alberta's own reporting documents

The 2024-25 Children and Family Services Annual Report

The Government of Alberta's Ministry of Children and Family Services publishes an annual report documenting the provincial child intervention system. The 2024-25 Annual Report, published on open.alberta.ca, provides the following primary-source figures (page 1 of the report):

- In 2024-25, on average each month, 6,439 Indigenous children received child intervention services.
- Indigenous children accounted for 72 percent of the provincial child intervention caseload.
- The number of Indigenous children receiving services decreased by 1 percent compared to the prior year — a rate of change that, at its current trajectory, would require 72 years to achieve proportional representation with the non-Indigenous population.
- Children and youth in care are supported through one of the ministry's delegated First Nation agencies, with 22 Delegated First Nation Agencies representing 37 of 48 Alberta First Nations.

The 72 percent figure is from the Alberta government's own reporting. It is not contested. The Minister responsible has acknowledged, in the same Annual Report, that the ministry is taking a “multi-faceted approach to address the overrepresentation of Indigenous children in the child intervention system.” The overrepresentation itself is administratively acknowledged at the highest level of the provincial ministry.

The prior year context

Earlier reporting establishes this is not new. As of December 2022, 74 percent of youth in care in Alberta were Indigenous, even though they comprised approximately 10 percent of the youth population (CBC Indigenous, April 2023). The overrepresentation ratio is therefore approximately 7-to-1: for every non-Indigenous child in the Alberta child welfare system, there are disproportionately seven Indigenous children relative to their share of the general youth population.

Statistics Canada's 2021 Census analysis, published April 18, 2024, established the national pattern:

- In 2021, Indigenous children accounted for 7.7 percent of all children under age 15 in the Canadian general population.
- Indigenous children accounted for 53.8 percent of all children in foster care.
- The overrepresentation ratio nationally is therefore approximately 7-to-1 — the same ratio documented in Alberta's provincial caseload.
- Manitoba held the highest rate of Indigenous foster children at 61.8 per 1,000 Indigenous children.
- Alberta held the highest disparity rate in Canada — 28.9 percentage points between rates of Indigenous and non-Indigenous foster children. This is because Alberta had the second-highest rate of Indigenous children in foster care combined with the lowest non-Indigenous foster child rate in Canada.
- Rates of foster children were two times higher among First Nations children living off reserve (49.9 per 1,000) compared with those living on reserve (24.6 per 1,000).
- Rates were four times higher among Inuit children living outside Inuit Nunangat (79.1 per 1,000) compared with those living inside Inuit Nunangat (18.8 per 1,000).

The off-reserve disparity is structurally significant. The populations most likely to be in the Alberta child welfare system are Indigenous children living off reserve — in Edmonton, Calgary, Lethbridge, Red Deer, and the other urban centres where the provincial child intervention system operates most intensively. The jurisdictional fracture from Section 3 operates here: on-reserve child welfare is federal responsibility and historically under-resourced; off-reserve child welfare is provincial responsibility and operationalized through the Alberta Children and Family Services system that the 2024-25 Annual Report documents.

The 17-fold apprehension disparity

The 2019 First Nations/Canadian Incidence Study of Reported Child Abuse and Neglect (FN/CIS 2019), published by Fallon, Lefebvre, Trocmé et al. in 2021 in the peer-reviewed literature under the title “Denouncing the Continued Overrepresentation of First Nations Children in Canadian Child Welfare,” established that Indigenous children are 17 times more likely to be removed from their families and placed in the child welfare system than non-Indigenous children in Canada.

This is not a ratio of children already in the system. It is a ratio of the probability of apprehension. An Indigenous child in Canada, all other factors being equal, is 17 times more likely to be taken from their family than a non-Indigenous child in comparable circumstances. The apprehension threshold is lower. The scrutiny is higher. The removal rate reflects that.

Indigenous children are 17 times more likely to be removed from their families than non-Indigenous children. In Alberta, 72 percent of the child

intervention caseload is Indigenous. The Statistics Canada finding, the peer-reviewed apprehension study, and the Alberta government's own Annual Report all point to the same conclusion, each from a different methodological direction. The numbers are not contested. The pattern is administratively acknowledged. The remedy is outstanding."

Part 2: The Millennial Scoop

The term and what it names

In academic and advocacy literature, the continuing overrepresentation of Indigenous children in the Canadian child welfare system has been named the "Millennial Scoop" or "Millennium Scoop." The term originates in work by Fallon, Lefebvre, Trocmé, Richard, Hélie, Montgomery, Bennett, Joh-Carnella, Saint-Girons, and Filippelli (2021), in CBC Radio coverage (The Current, January 2018), and in subsequent Truth and Reconciliation Commission and National Centre for Truth and Reconciliation documentation.

The comparison the term invokes is explicit. The residential school system, at its height in the 1930s and 1940s, held approximately 150,000 Indigenous children across its documented operation. The Sixties Scoop, operating primarily between the 1960s and 1980s, removed approximately 20,000 Indigenous children from their families through provincial child welfare systems and placed them, typically, with non-Indigenous foster and adoptive families. The Millennial Scoop, as documented, produces a population of Indigenous children currently in the child welfare system that exceeds the peak population of either preceding system.

The Truth and Reconciliation Commission of Canada, in its 2015 Final Report, documented the historical and ongoing forced removal of Indigenous children from their families and communities — and thereby their cultures and identities — as a "systemic cultural genocide with harmful intergenerational consequences." The Commission specifically called for reductions in the Indigenous foster child population. Calls to Action 1 through 5 address child welfare directly. Ten years later, the administrative reality documented in this section establishes that the Calls to Action have not been operationalized at a rate sufficient to reverse the overrepresentation pattern.

The documented mechanisms

The peer-reviewed literature documents several mechanisms by which the Millennial Scoop operates:

- Poverty as trigger: Indigenous families are more likely to be in poverty; poverty produces conditions (housing instability, food insecurity, family stress) that child welfare systems identify as neglect or risk factors. The 2011 National Household Survey documented that 38 percent of Indigenous children in Canada live in poverty, compared to 7 percent of non-Indigenous children

— a 5.4-times higher rate. Child welfare intervention is triggered by conditions that are themselves the product of the housing, land, water, and healthcare failures documented in Sections 4 through 7.

- Disability as risk factor: Indigenous parents with disabilities, medical complexity, or mental health conditions — precisely the population this report documents — face heightened risk of apprehension proceedings. Parental disability is documented in the child welfare literature as a risk factor considered in intake assessments. For an Indigenous parent whose disability is itself the product of the upstream causes this report has established, the disability is used against them in the very system that would take their children.
- Cultural discontinuity: The 2021 Census analysis established that the majority of Indigenous children in foster care are placed with non-Indigenous foster families. The loss of cultural continuity — connection to family, language, land, kinship, and community — is itself documented by Statistics Canada and by peer-reviewed research (Quinn 2022, Liebenberg et al. 2019) as a harm to the child's long-term mental health, identity, and wellbeing. The cure causes additional harm.
- Colonial inheritance: The Public Health Agency of Canada's own 2024 analysis in Health Promotion and Chronic Disease Prevention in Canada explicitly recognizes that Canadian child welfare systems “are colonial systems with abiding legacies of institutional abuse and discrimination against Indigenous, Black and other racialized communities.” The federal government's own public health reporting acknowledges the colonial nature of the system producing the Millennial Scoop.

There are more Indigenous children in the Canadian child welfare system today than there were in residential schools at the height of the residential school system. Researchers call this the Millennial Scoop. The Government of Canada's own public health reporting calls the system “colonial.” The mechanism has changed. The population being removed has not.”

Part 3: The rejected \$47.8 billion settlement

What the settlement was

In 2024, the Government of Canada negotiated a draft Final Settlement Agreement with the Assembly of First Nations addressing the First Nations Child and Family Services program. The settlement's headline figure was \$47.8 billion over ten years, intended to reform the long-term underfunding of the on-reserve child and family services system that had been the subject of Canadian Human Rights Tribunal rulings since 2016 in parallel to the Jordan's Principle litigation documented in Section 8.

The settlement was put to a vote of the First Nations chiefs in assembly at the Assembly of First Nations meeting on October 17, 2024. The vote was taken after two days of hearing varying perspectives on the proposed deal. The resolution was titled “Our Children, Our Future — Draft Agreement on reforming the First Nations Child and Family Services Program.”

The vote was as follows: **267 chiefs and proxies voted against.** 147 voted in favour. One abstained. The settlement was rejected.

Why it was rejected

The First Nations Child and Family Caring Society released, in August 2024, its Draft FSA Report Card: Evaluation of the Key Elements of the Final Settlement Agreement on First Nations Child and Family Services. The Caring Society’s evaluation identified structural shortcomings including:

- The settlement’s funding framework did not commit to the enforceable substantive equality standard that Canadian Human Rights Tribunal rulings had established.
- The reforms to service delivery would not address the documented underfunding levels that had been producing the apprehension pattern.
- The settlement did not adequately address the structural reforms necessary to implement Calls to Action 1 through 5 of the Truth and Reconciliation Commission.
- The settlement risked locking in a funding formula that would not grow with the population of First Nations children or with the documented cost of substantive equality.

The First Nations Child and Family Caring Society, led by Dr. Cindy Blackstock, proposed amendments to the draft agreement. The amendments were not incorporated into the final proposal that went to the vote. The rejection by 267 chiefs represents a supermajority of Canadian First Nations leadership determining that the settlement as written would not produce the structural remedy that the Canadian Human Rights Tribunal rulings required.

The status as of April 2026

Following the October 2024 rejection, the federal government and the Assembly of First Nations entered renewed negotiation. No revised settlement has been publicly ratified as of the publication of this report. The underlying Canadian Human Rights Tribunal proceedings — which established Canada’s liability for the First Nations child and family services underfunding discrimination — remain in effect. The administrative pattern that produces the apprehension data documented in Part 1 of this section continues unremedied at the federal funding level.

In the interim, the operational bulletin affecting Jordan’s Principle documented in Section 8 has narrowed the complementary federal program that Indigenous families have relied upon as a supplement to the structurally underfunded child and family services program. The net effect between October 2024 and April 2026 has been: federal child welfare structural reform remains

unratified; Jordan's Principle has been narrowed; provincial child welfare apprehension continues at 72 percent Indigenous caseload. Three federal-provincial administrative failures compound simultaneously.

Part 4: The specific risk to disabled Indigenous parents

The intersection this report documents

This report documents a specific population: Indigenous disabled Albertans. Section 9 must therefore address the specific intersection that occurs when a disabled Indigenous parent is also the primary caregiver of a disabled Indigenous child. In Alberta's documented child welfare context, this intersection is not hypothetical. It is the operational reality of a significant share of the 6,439 Indigenous children receiving child intervention services in any given month.

The Alberta Disability System Breakdown report series documents the financial reality of AISH and ADAP recipients: \$1,940 monthly benefit against rent exceeding the entire benefit, 86 percent of AISH recipients single individuals, and the July 1, 2026 transition reducing that benefit further. For a disabled Indigenous parent, this financial reality intersects with the child welfare system in specific documented ways:

- Housing instability is a child welfare risk factor. The housing conditions documented in Section 4 — inadequate on-reserve housing, uncapped private market rents, 10,000+ Civiida waitlist, the October 2025 63 percent community housing rent increase — produce the documented conditions that intake workers identify as neglect indicators.
- Parental disability is a risk factor assessed in intake. For an Indigenous parent whose disability traces to the upstream causes of Sections 1, 4, 5, 6, and 7 — residential school intergenerational trauma, contaminated land, poisoned water, healthcare system failure — the disability is itself evidence against them.
- Cognitive, communication, and literacy profiles associated with AISH-qualifying conditions produce difficulty navigating the child intervention system. The same profile that produces difficulty navigating the dual healthcare system documented in Section 7 produces difficulty navigating the Court of Queen's Bench proceedings that follow a child apprehension.
- Parental healthcare access collapse — the 650,000 Albertans without a GP documented in Section 7 — produces missed medical appointments for both parent and child that intake workers identify as indicators of medical neglect.

The disability-child-welfare risk compound

Alberta has, according to the Financial Analysis Community Version of the Alberta Disability System Breakdown report series, the highest child maltreatment rates in Canada. When that province's child welfare system is simultaneously the one with the highest Indigenous

overrepresentation disparity in Canada, the compound risk profile for a disabled Indigenous parent in Alberta is structurally elevated relative to comparable parents in any other province.

For an AISH recipient who is a single parent of disabled children, the intersection produces specific cascade risks:

- If the parent's AISH benefit is reduced under ADAP (documented \$200 per month cut effective January 2028), housing cost consumes a higher proportion of income, instability risk rises, intake attention rises, apprehension risk rises.
- If the parent loses AISH eligibility entirely under the reassessment documented in the main report series, loss of income triggers a cascade that includes loss of housing, loss of access to specialized services (FSCD, PDD), and potential child welfare intervention.
- If the parent's appeal rights are stripped under Bill 12 (passed December 9, 2025, documented in the main series), the parent cannot challenge the AISH determination that produces the cascade.

The provincial government that operates the child welfare system apprehending Indigenous children at 72 percent of its caseload is the same provincial government administering AISH and ADAP, the same government implementing the July 1, 2026 transition, the same government that implemented the October 2025 community housing rent increase, and the same government that stripped appeal rights through Bill 12. The parent facing apprehension and the parent facing benefit cut are frequently the same parent.

The same ministry that administers adult disability benefits in Alberta also administers child welfare. Assisted Living and Social Services oversees both the AISH program cutting the parent's income and the Children and Family Services file that will flag the resulting housing instability as a risk factor. The systems do not coordinate to protect the family. They coordinate, structurally, to destabilize it."

Part 5: Bill C-92 and the limits of jurisdictional transfer

An Act respecting First Nations, Inuit and Métis children, youth and families

On January 1, 2020, the federal Act respecting First Nations, Inuit and Métis children, youth and families (Bill C-92, now the First Nations, Inuit and Métis Families Act) came into force. The Act affirms the rights of First Nations, Inuit, and Métis peoples to exercise jurisdiction over child and family services, and establishes a process through which Indigenous governing bodies can enter into coordination agreements with the federal government and the relevant provincial or territorial government to take over child and family services for their members.

The Act represents the most significant federal legislative response to the Truth and Reconciliation Commission's Calls to Action 1 through 5 on child welfare. The mechanism it creates is jurisdictional transfer: the ability of Indigenous governing bodies to operate their own child and family services under their own laws, with federal funding flowing through the coordination agreement framework.

In Alberta specifically, 22 Delegated First Nation Agencies represent 37 of 48 Alberta First Nations in the existing delegation framework that predates Bill C-92. The Delegated First Nation Agencies operate under provincial legislative authority but with some degree of First Nation-specific design and community connection. Bill C-92 provides a pathway for these agencies, and for First Nations outside the existing delegation framework, to transition to fully Indigenous-jurisdiction child and family services.

The limits of the current framework

The sustained challenge of Bill C-92 implementation is that jurisdictional transfer without adequate funding produces administrative jurisdiction without operational capacity. A First Nation with legal authority to operate its own child and family services cannot meaningfully exercise that authority without sufficient federal funding for caseworkers, placement resources, facilities, prevention services, and coordination with the provincial system in which non-Indigenous placements continue to occur.

The First Nations Child and Family Caring Society's sustained advocacy — the same advocacy that produced the Canadian Human Rights Tribunal rulings on Jordan's Principle documented in Section 8 — has established that the core structural issue is not the legislative framework but the operational funding. The October 2024 rejection by 267 chiefs of the \$47.8 billion Final Settlement Agreement represents a collective First Nations determination that the funding offered was not sufficient to operationalize the jurisdiction Bill C-92 affirms.

For an Indigenous child in Alberta in April 2026, the operational reality is:

- Bill C-92 affirms Indigenous jurisdiction over child and family services.
- The federal funding framework to operationalize that jurisdiction has not been adequately negotiated — evidenced by the October 2024 rejection.
- Provincial child welfare continues to operate through Alberta Children and Family Services, producing the 72 percent Indigenous caseload documented in Part 1.
- The Canadian Human Rights Tribunal rulings establish the federal government's ongoing liability for the discrimination produced by this framework.
- Jordan's Principle, which had functioned as the cross-cutting mechanism for addressing gaps between federal and provincial child-related services, has been narrowed by the February 2025 operational bulletin documented in Section 8.

The federal and provincial administrative systems, taken together, do not currently produce a pathway to structural remedy of the Millennial Scoop at the rate required to reverse the overrepresentation pattern within the current generation of Indigenous children.

What this section establishes for the remainder of the report

Alberta's provincial child welfare system, as documented by the Alberta government's own 2024-25 Annual Report, operates with an Indigenous caseload of 72 percent while Indigenous children represent approximately 10 percent of the youth population. The apprehension rate disparity between Indigenous and non-Indigenous children in Canada is 17-to-1. Alberta holds the highest disparity rate in Canada at 28.9 percentage points. The Truth and Reconciliation Commission documented the historical pattern producing these outcomes as systemic cultural genocide. The federal Canadian Human Rights Tribunal has ruled repeatedly, since 2016, that the First Nations Child and Family Services underfunding constitutes discrimination. The \$47.8 billion settlement negotiated in response was rejected by 267 First Nations chiefs in October 2024 on grounds that it would not produce structural remedy.

The population affected by this system is the same population this report documents: Indigenous children who will grow into Indigenous disabled adults, Indigenous parents whose disabilities are themselves the product of the upstream causes this report has established, Indigenous families who navigate the same dual federal-provincial administrative system that Sections 4 through 8 have already documented failing at every other point of contact. The child welfare system is one more expression of the pattern. It does not introduce a new failure. It elaborates the failure that is already present across housing, land, water, healthcare, and children's cross-jurisdictional services.

Sections 10 through 15 will address the justice system, MMIWG2S+, consultation failure, the Sovereignty Act's relationship to Treaty, TRC Calls to Action 18 through 24, and the July 1, 2026 ADAP transition synthesis. The pattern documented in Section 9 — disproportionate representation, provincial administrative acknowledgment of the problem, federal legal obligation insufficiently operationalized, settlement agreements as compensation-without-remedy, and specific compounded harm to disabled Indigenous persons — recurs in those sections. The documented population is the same. The remedy this report calls for is the same. The urgency increases with each section.

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SECTION 10

The Justice System

The Supreme Court of Canada named the over-incarceration of Indigenous people a crisis in 1999. Twenty-six years later, Indigenous adults are incarcerated at 10.2 times the non-Indigenous rate, Indigenous women at 18.4 times, and two-thirds of all Indigenous inmates have passed through the foster care system Section 9 documented. This is the adult end of the pipeline Section 9 opened.

In 2005, the United Nations Working Group on Arbitrary Detention visited Canada and raised concerns about the overrepresentation of Indigenous people in Canadian prisons. Twenty years later, in its August 4, 2025 report, the same Working Group declared itself “deeply concerned” — the same concern, documented again, after two decades. In 1999, the Supreme Court of Canada called this a crisis. In 2012, it called it Canada’s failure of the Aboriginal peoples. In 2015, the Truth and Reconciliation Commission’s Call to Action 30 demanded elimination within a decade. That decade expired in 2025. The crisis, by every available measure, has deepened.”

This section documents the Canadian justice system as it operates against Indigenous adults in 2026. The section builds on every preceding section of this report but is most directly connected to Section 9’s child welfare analysis: two-thirds of Indigenous people in Canadian federal custody have been in foster care at some point in their childhood. The apprehension statistics documented in Section 9 are the childhood-stage data for the incarceration statistics documented here. The same population moves through the same administrative failure, across the life course this report has been tracing since Section 1.

Six findings structure the section. First, that the 2026 Statistics Canada data release documents Indigenous adult incarceration at 10.2 times the non-Indigenous rate across six reporting provinces, with Alberta specifically at 10.2 times — and the overrepresentation widening every year for five consecutive years. Second, that Indigenous women specifically have seen the sharpest deterioration: the overrepresentation index for Indigenous women rose from 12.6 to 18.4 between 2019/2020 and 2023/2024, with Indigenous women at 50 percent of all federally incarcerated women and 65 percent of women in maximum security custody. Third, that the Gladue and Ipeelee jurisprudence establishing the Supreme Court’s direction on Indigenous sentencing has been, in the characterization of former Justice Harry LaForme, used “as a sword instead of a shield” by Corrections Canada. Fourth, that the United Nations Working Group on

Arbitrary Detention's August 2025 report establishes that federal compliance with international obligations on Indigenous detention has not meaningfully improved in twenty years. Fifth, that the foster-care-to-prison pipeline is statistically documented, named by Indigenous scholars including Dr. Pamela Palmater, and traced through the specific mechanism Section 9 established. Sixth, that the cost of incarceration vastly exceeds the cost of the disability support that this report documents as being withdrawn — provincial incarceration at \$60,000 to \$80,000 per person per year versus AISH at \$20,772 — making AISH destabilization a fiscal transfer into the corrections budget via the very population whose disability benefits are being reduced.

Part 1: The scale of overrepresentation in 2026

Statistics Canada, January 14, 2026

On January 14, 2026, Statistics Canada released The Daily report titled “Overrepresentation of Indigenous and Black adults in provincial and federal custody.” The release represents the most comprehensive primary-source documentation of Indigenous overrepresentation in Canadian custody available as of the publication of this report. The findings, drawn from the six provinces currently reporting to the Canadian Correctional Services Survey — Prince Edward Island, Nova Scotia, Ontario, Saskatchewan, Alberta, and British Columbia — establish the following:

- In 2023/2024, Indigenous adults made up 33.2 percent of the average daily count of the custodial population in provincial and federal facilities across the six reporting provinces — 8,734 Indigenous adults of a 26,297 custodial population.
- Indigenous adults were incarcerated at a rate 10.2 times higher than non-Indigenous adults. This is measured by the overrepresentation index, which controls for age and gender differences between populations.
- Saskatchewan held the highest overrepresentation index at 19.4 times. Alberta specifically stood at 10.2 times — identical to the national average across reporting provinces. British Columbia at 9.4. Ontario at 8.3. Prince Edward Island at 3.4.
- The overrepresentation index rose every year for five consecutive years. In 2019/2020, the Indigenous incarceration rate was 8.1 times the non-Indigenous rate. By 2023/2024, it had risen to 10.2. This is not a plateau. This is not a reversal. This is a sustained worsening of a problem the Supreme Court of Canada named a crisis in 1999.
- Overall, 2.6 percent of the Indigenous adult population was incarcerated at some point during the 2023/2024 reference year — a measure Statistics Canada calls the custody involvement rate. Among Indigenous men aged 35 to 44 years, this rate rose to 7.3 percent — more than one in every fourteen Indigenous men in that age cohort passing through custody in a single year.

Indigenous women

The deterioration for Indigenous women is sharper than the male average and has specific gendered characteristics. Statistics Canada documented:

- The overrepresentation index for Indigenous women rose from 12.6 in 2019/2020 to 18.4 in 2023/2024, peaking at 18.5 in 2022/2023.
- Indigenous women's incarceration rate rose 22 percent between 2019/2020 and 2023/2024, while non-Indigenous women's incarceration rate declined by 11 percent in the same period.
- At the federal level, Indigenous women account for 50 percent of all federally incarcerated women (as of 2023 data, reported in the UN Working Group on Arbitrary Detention report of August 4, 2025).
- Indigenous women account for 65 percent of sentenced women in maximum security custodial centres across Canada (as of April 2022, Correctional Service of Canada data).

Indigenous women are approximately five percent of the Canadian adult population. They are half of the federally incarcerated women. They are nearly two-thirds of the women in maximum security. The gendered arithmetic is as severe as any numerical disparity documented in this report.

Federal incarceration

At the federal level, where sentences of two years or more are served, the documented data (as reported by the UN Working Group on Arbitrary Detention in August 2025 and summarized by Public Safety Canada briefing documents):

- Indigenous people are approximately five percent of the Canadian adult population.
- Indigenous people account for 32 percent of detainees in federal custody.
- In 2020, that figure was 30 percent; in 2001, 18 percent. The proportion has risen in every measured interval.
- The UN Working Group reported that in some provincial detention facilities, up to 70 or 80 percent of inmates were Indigenous.

Indigenous people are not inherently more criminal. We are more criminalized. — Kory Wilson, Chair, BC First Nations Justice Council, January 2026"

Part 2: Gladue, Ipeelee, and Canada's acknowledged failure

R v Gladue (1999)

In 1999, the Supreme Court of Canada delivered its decision in R v Gladue, 1999 CanLII 679 SCC, [1999] 1 SCR 688. Jamie Gladue was a young Indigenous woman convicted of the

manslaughter of her common-law husband. The question before the Court was how section 718.2(e) of the Criminal Code — a provision enacted in 1995 directing sentencing judges to consider sanctions other than imprisonment with particular attention to the circumstances of Aboriginal offenders — should be applied.

The Supreme Court's answer established the Gladue framework. Sentencing judges have a statutory duty to give force to section 718.2(e). For Aboriginal offenders, this requires a different method of analysis that considers: the unique systemic or background factors that have played a part in bringing the Aboriginal offender before the court; and the types of sentencing procedures or sanctions appropriate in the circumstances for the offender because of their Aboriginal heritage or connection.

The Court characterized the overrepresentation of Indigenous people in the Canadian prison and criminal justice system as “a crisis in the Canadian criminal justice system.” At the time of the Gladue decision in 1999, Indigenous people were approximately three percent of the Canadian population and seventeen percent of the prison population. The Court called that disparity a crisis. By 2023/2024 data, Indigenous people are approximately five percent of the Canadian adult population and 33.2 percent of the custodial population in the reporting provinces. The disparity has nearly doubled since the Court named it a crisis. It remains, by the Court's own framing, a crisis.

R v Ipeelee (2012)

In 2012, the Supreme Court delivered R v Ipeelee, 2012 SCC 13, [2012] 1 SCR 433. The case reaffirmed Gladue and went further. At paragraph 57, the Court explicitly acknowledged that the Canadian criminal justice system “has failed the Aboriginal peoples of Canada.” That sentence is not advocacy commentary. It is a finding by the highest court in Canada.

Ipeelee also made clear that judges have a duty to take judicial notice of systemic and background factors affecting Indigenous offenders, including: the history of colonialism; displacement; the residential schools and their continuing effects on lower educational attainment, lower income, higher unemployment, higher rates of substance abuse and suicide; the Sixties Scoop; and the cumulative intergenerational consequences of these forces. These are not factors the judge may consider if raised. They are factors the judge must take notice of. The Supreme Court established a national legal standard of acknowledgment of Canada's complicity in producing the conditions that bring Indigenous offenders before the courts.

If Gladue recognized the extent of the Indigenous over-incarceration problem, Ipeelee is an explicit acknowledgement of Canada's complicity in creating this problem and contributing to its perpetuation. The Court acknowledges that the Canadian criminal justice system has failed the Aboriginal peoples of Canada. — Department of Justice Canada,

*Spotlight on Gladue: Challenges, Experiences, and Possibilities in
Canada's Criminal Justice System, 2017"*

Why the legal framework has not produced proportional remedy

Despite the Gladue and Ipeelee rulings, the overrepresentation of Indigenous people in Canadian custody has increased in nearly every measurable interval since Gladue was decided. As documented by the Honourable Harry LaForme, retired justice of the Ontario Court of Appeal, writing in public commentary: since the 1999 Gladue decision, non-Indigenous imprisonment numbers have decreased each year, while Indigenous imprisonment numbers have increased annually. The legal framework has failed to produce the remedy the Supreme Court mandated.

Several mechanisms, documented in the peer-reviewed literature and government reviews, explain this failure:

- Gladue Reports are unevenly available. Ontario has specialized Gladue Courts and trained Gladue report writers. Most of the rest of Canada does not. An Indigenous offender's access to a properly prepared Gladue report depends on where they are sentenced, not on their Aboriginal status per se.
- Where Gladue reports are prepared, they are frequently given perfunctory attention at sentencing. Former Justice LaForme has documented that in many courts, Gladue is "merely referred to in passing in court, if at all, without any assessment of its impact or intended purpose."
- Community-based restorative alternatives to incarceration, which Gladue called for, remain underfunded and unevenly distributed. The culturally safe programs and Indigenous-led diversion initiatives that would operationalize Gladue are described by the BC First Nations Justice Council and others as "often underfunded."

Gladue as sword, not shield — the structural subversion

The most devastating documented subversion of the Gladue framework occurs at the point where Gladue reports transition from the sentencing stage into the correctional system. Howard Sapers, former Correctional Investigator of Canada, and former Justice Harry LaForme have both publicly documented the following pattern:

- The Gladue report is prepared for sentencing. Its purpose, per the Supreme Court, is to assist the court in finding alternatives to imprisonment by contextualizing the Indigenous offender's background factors.
- The Gladue report, once prepared, becomes part of the correctional file.
- Corrections Canada uses the same Gladue report — the same documented history of colonialism, residential schools, foster care, poverty, addiction, trauma — as evidence of the

offender's likelihood to reoffend. The report that was supposed to reduce the sentence becomes the evidence that denies early release.

Former Justice LaForme has named this explicitly: Corrections Canada uses Gladue reports "as a sword instead of a shield." Howard Sapers has stated that "when that Gladue report is shared with corrections, all of those things become risk factors." The colonial factors the Supreme Court directed judges to consider in mitigation at sentencing are, in the correctional system, treated as aggravating factors in determining parole eligibility, security classification, and programming access.

In one measure of the pattern, discretionary releases for Indigenous offenders have increased from 23.5 percent in fiscal 2013-2014 to 38.6 percent in fiscal 2020-2021 — meaningful improvement. In another measure, the overrepresentation index across all forms of custody has risen from 8.1 in 2019-2020 to 10.2 in 2023-2024 — meaningful worsening. Both data sets can be true simultaneously: improved parole outcomes for those who receive them; increased overall incarceration for the Indigenous population as a whole. The latter measure is the one that determines whether the Supreme Court's 1999 crisis has been addressed. It has not been.

Part 3: The foster-care-to-prison pipeline

The two-thirds finding

The mechanism connecting Section 9 to Section 10 is statistically documented in the correctional literature: two-thirds of all Indigenous inmates in Canadian federal custody have been in foster care at some point in their childhood. This is not an informal estimate. It is the documented prevalence of the foster-care background among Indigenous federal inmates, as recorded in Correctional Service of Canada intake data and cited in the Department of Justice Canada's Gladue research literature.

The implications are direct. The Alberta provincial child welfare system documented in Section 9, operating with a 72 percent Indigenous caseload, is a primary feeder of the Canadian federal corrections system. An Indigenous child apprehended in Alberta at age four is statistically at substantially elevated risk of entering the corrections system at age eighteen or twenty or twenty-five. The 17-to-1 apprehension disparity that Section 9 documented becomes a 10.2-to-1 incarceration disparity documented here. The pipeline is not metaphorical. It is numerical.

Dr. Pamela Palmater on the pipeline

Dr. Pamela Palmater, Mi'kmaq lawyer and Chair in Indigenous Governance at Toronto Metropolitan University, has documented the pipeline publicly:

If you steal a child from a First Nation family and that child goes into foster care, we know from the statistics they're far more likely to end up in youth corrections or homeless or trafficked. — Dr. Pamela Palmater"

Palmater's analytic frame places the over-incarceration of Indigenous people in historical context:

In order to truly understand the over-incarceration of Indigenous peoples in Canada, you have to go back to contact. When colonial governments established themselves on native territories in order to steal our territories and our resources, they had to find ways to oppress us, contain us, control us, and remove us from those territories: trapping us on reserves, sending our kids to residential schools, scalping bounties, forced sterilizations. It's always for the same purpose: to get our lands and resources. — Dr. Pamela Palmater

Palmater characterizes the overall pattern explicitly: “The over-incarceration of Indigenous peoples in Canada is a form of genocide — and it’s a series of grave human-rights violations.” The characterization is consistent with the legal finding in Ipeelee (2012) at paragraph 57 that the Canadian criminal justice system has failed the Aboriginal peoples of Canada, and with the UN Working Group on Arbitrary Detention’s August 2025 report of being “deeply concerned” about continuing overrepresentation.

The Gladue factors named

Howard Sapers, former Correctional Investigator of Canada, has summarized the factors the Supreme Court in Gladue and Ipeelee directed judges to take notice of:

You can see all of this dislocation from community, from place. Being disempowered in terms of economic participation. Even access to education and sometimes to health care. All of these things, taken together, have created a big gap in outcomes between what an Indigenous young man or young woman can expect in their lifetime versus a non-Indigenous Canadian can expect in their lifetime. These have been called Gladue factors. — Howard Sapers, former Correctional Investigator of Canada

The Gladue factors are the same factors this report has been documenting across Sections 1, 4, 5, 6, 7, 8, and 9. Dislocation from community. Disempowerment in economic participation. Lack of access to education. Lack of access to healthcare. Each of those factors corresponds to a section of this report. Section 10 is the stage at which the cumulative effect of those factors is translated into custody. The Supreme Court of Canada named these as the things judges must take judicial notice of. This report documents them as the things they are: the upstream causes

of the downstream overrepresentation. Every Gladue factor is in this report. The report and the Supreme Court are looking at the same pattern from different angles.

Part 4: Twenty years after 2005 — the UN returns

The UN Working Group on Arbitrary Detention, August 4, 2025

In 2005, the United Nations Working Group on Arbitrary Detention conducted an official country visit to Canada. Its findings included concern about the overrepresentation of Indigenous people in Canadian detention facilities. On August 4, 2025, the same Working Group published its updated report on Canada — twenty years later. Its characterization was direct: the Working Group is “deeply concerned” about the continuing overrepresentation of Indigenous people in Canada’s justice system.

The August 2025 report documented that data from 2023 showed Indigenous people at approximately five percent of Canada’s adult population but 32 percent of detainees in federal custody and 50 percent of all incarcerated women. In some provincial detention facilities, the Working Group reported, up to 70 or 80 percent of inmates were Indigenous. The Working Group noted, in comparison, an 88 percent decrease in the overall youth incarceration rate between 1997-1998 and 2020-2021 as one positive development — establishing that substantial reductions are possible when political will is applied. The absence of comparable reduction in Indigenous adult incarceration over the same period is, by implication, the absence of comparable political will.

The structural significance of the UN Working Group’s August 2025 report is twofold. First, it establishes that Canada’s compliance with its international human rights obligations on arbitrary detention has not meaningfully improved in twenty years. The Working Group’s “deeply concerned” framing is diplomatic language for sustained documented non-compliance. Second, it places Canada’s Indigenous incarceration record in the same international reporting cycle as the UN Committee on the Rights of Persons with Disabilities recommendations (referenced in Section 17 of this report) and the UN Committee on the Elimination of Racial Discrimination observations — a coordinated international record that Canada’s own governments have chosen not to respond to with proportional reform.

The UN Working Group on Arbitrary Detention visited Canada in 2005 and raised concerns about Indigenous overrepresentation in Canadian prisons. It published its 2025 follow-up report on August 4, 2025. Its conclusion was “deeply concerned.” The word “deeply” is the diplomatic acknowledgment that twenty years of Canadian policy response has not been sufficient to address what the Supreme Court of Canada itself called a crisis in 1999.”

Part 5: What it costs to incarcerate versus what it costs to support

The arithmetic

The Alberta Disability System Breakdown report series includes destabilization cost modelling prepared by Gavin Sorochan, Edmonton community organizer and founder of Continuum Care Foundation. The modelling establishes per-person annual costs when disability income support is withdrawn and crisis services fill the gap. Provincial incarceration, where it occurs, is one of those crisis services. The per-person annual cost of provincial incarceration in Alberta is documented at \$60,000 to \$80,000.

The annual cost of AISH support at the base rate is \$20,772. The cost ratio is therefore approximately 3-to-4 times higher for provincial incarceration than for AISH support of the same person — assuming the AISH-destabilized person reaches provincial incarceration, which not all will. For federal incarceration, where sentences of two years or more are served, the per-person annual cost rises substantially further. The Correctional Service of Canada's per-inmate average cost is in the \$115,000 range, with Indigenous-specific programming costs often higher.

For an Indigenous disabled Albertan specifically — the population this report documents throughout — the fiscal arithmetic of the July 1, 2026 ADAP transition and the broader dismantlement of disability supports documented in the main report series intersects with the justice system arithmetic documented here. If the ADAP transition pushes even a small percentage of Indigenous disabled Albertans into the crisis cascade that produces justice system contact, the provincial fiscal saving from reducing AISH expenditure is transferred into the corrections budget at three to four times the per-person cost. The province does not save money. It spends the same money on the same people through a different administrative apparatus, and it does so at a higher per-person rate.

The specific risk compound

For a disabled Indigenous Albertan, the cascade risk has specific dimensions this section can now articulate. If AISH benefits are reduced or revoked under the July 1, 2026 transition — and the physician-completed form requirement documented in Section 7 makes revocation structurally likely for patients without family physicians — the loss of income triggers:

- Housing instability (documented in Section 4) produces homelessness for a population that was already paying more than the AISH benefit in rent.
- Loss of prescription medication access (documented in Sections 7 and in the dual NIHB/AHS framework) produces medication discontinuation for psychiatric, pain management, and chronic disease conditions.
- Medication discontinuation in psychiatric populations produces crisis — emergency department visits (up 47 percent in Alberta prior to the ADAP transition, per Alberta Doctors'

Digest), psychiatric hospitalizations, and in some cases behavioural episodes that bring police contact.

- Police contact in a population with cognitive, communication, or mental health profiles that interact badly with police intervention — the same profiles the AISH qualifying criteria capture — produces documented escalation risk, including use-of-force incidents and custody admissions.
- Custody admission initiates the provincial administrative process documented in the Alberta IES Policy Manual — including the SEIP 3850-1 AFSS Match between LISA and Alberta Justice Inmate Records, ORCA number tracking, system closure procedures, and the creation of debt in the provincial Recoveries System against the disabled person. The same ministry that stopped paying AISH starts recovering debt.

This cascade is not speculative. Each step is documented in provincial administrative policy manuals, in Statistics Canada data, in peer-reviewed literature on disability and police interaction, and in the Correctional Service of Canada's own admission data. The structure that produces the cascade is built into the interaction between the AISH/ADAP administrative system and the corrections administrative system. Both systems are operated by the same provincial government.

Part 6: What the Alberta provincial policy manual acknowledges

The Government of Alberta's Income and Employment Supports Training and Health Benefits (IESTHB) Policy Manual contains detailed administrative policy on how Income Support treats clients who become incarcerated. The policy establishes that provincial Income Support workers are required to review the SEIP 3850-1 AFSS Match between the provincial Income Support system (LISA) and Alberta Justice Inmate Records, identifying active Income Support clients who have been admitted to provincial correctional centres. The match report provides admit dates, aggregate sentences, ORCA numbers, correctional centre identification, and self-reporting flag status.

Upon identification of an incarcerated Income Support client, the worker is directed to take specific actions: system closure of the file, referral to the Fraud Investigation Unit where applicable, creation of a debt in the Recoveries System, and adjustment of the household unit's benefit level. The policy provides that child care or homemaking is not provided for the children of a person who has been incarcerated; the family is referred to Children's Services — the same provincial ministry whose 72 percent Indigenous child welfare caseload Section 9 documented.

The structural significance is that Alberta's own written policy documents, in administrative detail, the seamless operational interface between the Income Support system and the provincial corrections system. The same ministry's database systems talk to each other. The match runs on a scheduled basis. The debt recovery process activates automatically. The children of the incarcerated disabled person are referred into the child welfare system that Section 9 documented. Three provincial administrative systems — Income Support, Alberta

Justice Corrections, and Children and Family Services — operate as a single coordinated apparatus for the specific population this report documents. The coordination exists. What does not exist is a parallel coordination of supports to keep the disabled Indigenous Albertan out of that apparatus in the first place.

Alberta's own written policy documents the data-match system between LISA (Income Support) and Alberta Justice Inmate Records, the debt creation that activates automatically when a client enters custody, and the referral of that client's children into the Children and Family Services system. The operational integration between disability support withdrawal, incarceration, and child apprehension is not accidental. It is administratively coordinated by the provincial government, in its own policy manual, in specific detail."

What this section establishes for the remainder of the report

The Supreme Court of Canada named the overrepresentation of Indigenous people in Canadian custody a crisis in 1999. In 2012, the same Court acknowledged that the Canadian criminal justice system had failed the Aboriginal peoples of Canada. In 2015, the Truth and Reconciliation Commission's Call to Action 30 demanded elimination of the overrepresentation within a decade. That decade has expired. Statistics Canada's January 2026 release documents Indigenous adults incarcerated at 10.2 times the non-Indigenous rate — up from 8.1 just five years earlier. Indigenous women at 18.4 times. Indigenous women at 50 percent of federally incarcerated women. The UN Working Group on Arbitrary Detention "deeply concerned" in 2025, with the same concern it raised in 2005.

Two-thirds of Indigenous people in federal custody have been in foster care. The 72 percent Indigenous child welfare caseload documented in Section 9 is the early-stage data for the 33.2 percent Indigenous adult custody figure documented here. The population is the same. The pipeline is documented. The administrative coordination of disability withdrawal, incarceration, and child apprehension is written into the Alberta IES Policy Manual in explicit detail.

For the Indigenous disabled Albertan this report documents, the justice system is not a separate policy file. It is the downstream end of the same administrative machinery this report has been tracing since Section 1. Residential schools produced the trauma. Contaminated land produced the disability. Poisoned water compounded it. The dual healthcare system fails to treat it. Jordan's Principle fails to support it in childhood. The child welfare system apprehends its carrier. The justice system incarcerates its adult. At each stage, Canada's federal and provincial governments acknowledge the failure in written documents — Supreme Court rulings, coroner's inquests, Auditor General reports, class action settlements, Canadian Human Rights Tribunal orders, UN reports — and do not produce proportional remedy.

Sections 11 through 15 will address MMIWG2S+, consultation failure, the Sovereignty Act's relationship to Treaty, TRC Calls to Action 18 through 24, and the July 1, 2026 ADAP transition synthesis. The pattern documented in Section 10 — federal legal obligation, sustained non-compliance, documented cost transfer, coordinated provincial administrative apparatus operating against the documented population — continues into those sections. The specific face of the Indigenous disabled Albertan varies. The structural pattern does not.

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SECTION 11

MMIWG2S+

The 2019 National Inquiry concluded that the missing and murdered Indigenous women and girls crisis amounts to genocide. The Inquiry produced 231 Calls for Justice. By the fifth anniversary, two had been completed. As this report is being written in April 2026, MMIWG2S+ program funding is sunsetting at the federal level, threatening the few mechanisms that exist to protect Indigenous women, girls, and Two-Spirit people — the same population whose disability profiles, where they survive, this report has been documenting throughout.

Indigenous women are 12 times more likely to be murdered or to go missing than non-Indigenous women in Canada. They are 16 percent of all female homicide victims and 11 percent of missing women, while comprising only 4.3 percent of the Canadian population. The 2019 National Inquiry into Missing and Murdered Indigenous Women and Girls concluded, in its Final Report, that this constitutes a genocide. As of April 2026, of 231 Calls for Justice the Inquiry produced, two have been completed.”

This section documents the MMIWG2S+ crisis as it intersects with the disability dimension this report has been tracing. The section builds on Sections 9 and 10: the same provincial child welfare apprehension pattern that produces the 17-to-1 disparity in Section 9 and the 10.2-to-1 incarceration disparity in Section 10 produces the 12-to-1 disparity in murder and disappearance documented here. The pipeline does not stop at the prison. For Indigenous women specifically, particularly those whose vulnerabilities include disability, the pipeline ends in violent death or disappearance at a rate the National Inquiry characterized as genocidal.

Five findings structure the section. First, that the 2019 National Inquiry's legal characterization of MMIWG2S+ as genocide is the operative finding under which Canadian federal and provincial governments are now obligated to act. Second, that the 231 Calls for Justice the Inquiry produced have been substantively unimplemented, with only two completed as of June 2024 according to the Assembly of First Nations' own progress reporting. Third, that as of April 2026 — the month this report is being published — federal funding for MMIWG2S+ programs is being allowed to sunset, with the National Family and Survivors Circle and Giganawenimaanaanig both publicly reporting unrenewed core funding. Fourth, that disability is documented in the MMIWG2S+ literature as a specific compounding vulnerability factor, intersecting with poverty, housing insecurity, and the institutional histories this report has documented across previous sections. Fifth, that Alberta's administrative apparatus for AISH

withdrawal, child apprehension, and incarceration documented in Sections 9 and 10 produces the precise destabilization conditions the National Inquiry identified as upstream causes of MMIWG2S+ vulnerability.

Part 1: The National Inquiry and the genocide finding

The Inquiry

The National Inquiry into Missing and Murdered Indigenous Women and Girls was established by the Government of Canada in September 2016, building on Truth and Reconciliation Commission Call to Action 41.2, which directed the federal government to create an independent inquiry into the disproportionate violence experienced by Indigenous women and girls. The Inquiry was led by four commissioners. Over the next two and a half years, the Inquiry heard testimony from more than 2,380 family members of murdered and missing Indigenous women and girls, survivors of violence, expert witnesses, and Knowledge Keepers.

On June 3, 2019, the Inquiry released its Final Report, titled *Reclaiming Power and Place: The Final Report of the National Inquiry into Missing and Murdered Indigenous Women and Girls*. The Final Report was delivered to Prime Minister Justin Trudeau in a ceremony at the Canadian Museum of History in Gatineau, Quebec. The two-volume report was the culmination of the most comprehensive primary-source documentation ever assembled in Canada on violence against Indigenous women, girls, and 2SLGBTQQIA+ people.

The genocide finding

The Inquiry's Final Report concluded that the documented pattern of disappearance and murder of Indigenous women, girls, and 2SLGBTQQIA+ people in Canada constitutes a genocide. The legal and conceptual basis for this finding is set out in detail in a supplementary legal analysis accompanying the report, drawing on the United Nations Convention on the Prevention and Punishment of the Crime of Genocide (1948) and on the cumulative effect of Canadian colonial law and practice across the centuries the Inquiry documented.

The Final Report stated: "The steps to end and redress this genocide must be no less monumental than the combination of systems and actions that have worked to maintain colonial violence for generations." The genocide finding is the operative legal and political characterization that frames the 231 Calls for Justice. It is not advocacy framing. It is the conclusion of a federally-commissioned independent inquiry led by four Commissioners, drawing on more than 2,380 sources of testimony.

The scale of the disparity

The Inquiry documented, and subsequent statistical reporting has confirmed, the scale of the disparity:

- Indigenous women are 12 times more likely to go missing or be murdered than non-Indigenous women in Canada.
- Indigenous women comprise 16 percent of all female homicide victims in Canada, while comprising only 4.3 percent of the Canadian population — an overrepresentation ratio of approximately 3.7-to-1.
- Indigenous women comprise 11 percent of missing women in Canada — an overrepresentation ratio of approximately 2.6-to-1.
- 56 percent of Indigenous women have suffered physical assault. 46 percent have experienced sexual assault. By comparison, approximately one-third of non-Indigenous women have suffered these assaults in their lifetimes.
- 13 percent of Indigenous people experience violence from a current or ex-partner — a rate twice that of non-Indigenous people (5.7 percent).

The cumulative effect is genocide. — National Inquiry into Missing and Murdered Indigenous Women and Girls, Reclaiming Power and Place: Final Report (2019)”

Part 2: The 231 Calls for Justice and the implementation failure

The structure of the Calls

The Inquiry’s Final Report contains 231 individual Calls for Justice, organized into 18 main areas of reform. The Calls are directed at governments at all levels, police, health providers, the justice system, the media, social service providers, industries, and all Canadians. Distinctive Calls for Justice are issued specifically for Inuit, Métis, and 2SLGBTQQIA people, recognizing the differentiated needs and harms within the broader population.

The Calls cover the full range of structural conditions the Inquiry identified as producing MMIWG2S+ vulnerability:

- Calls for Justice 1.1 through 1.11 address the foundational legal, governance, and accountability framework, including the development of a National Action Plan, full compliance with UNDRIP and the United Nations Convention on the Rights of the Child, elimination of social, economic, cultural, and political marginalization, representation in governance, and creation of an independent national Indigenous oversight body.
- Subsequent Calls address health and wellness, human security including housing and food security, justice and policing, child welfare, education, media representation, and the specific Calls for Inuit, Métis, and 2SLGBTQQIA persons.
- Many of the Calls correspond directly to the structural failures documented elsewhere in this report: housing inadequacy (Section 4), water and environmental contamination (Sections 5 and

6), healthcare system failure and anti-Indigenous racism in healthcare (Section 7), Jordan's Principle (Section 8), child welfare (Section 9), and the justice system (Section 10). The MMIWG2S+ Calls for Justice are, in operational terms, a remedial framework that addresses the same upstream causes this report has documented across all preceding sections.

The implementation failure

In June 2023, four years after the Final Report's release, CBC published a comprehensive analysis of the implementation status of all 231 Calls for Justice across federal, provincial, and territorial jurisdictions. The analysis found that of the 231 Calls, only two had been completed. More than half had not even been started — categorized by CBC as Calls for which promises may have been made or money may have been committed, but no change has been effected and no money has been spent.

In June 2024, the Assembly of First Nations released its Progress on Breathing Life into the Calls for Justice: A CFJ Progress Report — the first AFN-led monitoring report on implementation. National Chief Cindy Woodhouse-Nepinak's public assessment, marking the fifth anniversary of the Final Report on June 3, 2024: only two of the 231 recommendations have been met. Five years of federal, provincial, and territorial action has produced substantive completion of less than one percent of the Inquiry's recommendations.

There is a long, winding road ahead to address and prevent all forms of gender-based violence. — National Chief Cindy Woodhouse-Nepinak, Assembly of First Nations, June 2024, on the fifth anniversary of the MMIWG Final Report"

The specific failures the AFN documents

The AFN's 2024 Progress Report documents specific structural failures across the implementation framework. Examples include:

- Call for Justice 1.7 — the creation of a national Indigenous and human rights ombudsperson and tribunal — has been the subject of a Ministerial Special Representative process initiated in 2023, with engagement of more than 600 people from more than 125 Indigenous organizations. As of the publication of this report, no ombudsperson position has been created. The accountability mechanism the National Inquiry identified as foundational remains unimplemented.
- Call for Justice 1.10 — creation of an independent national Indigenous oversight body — was, as of the 2022 progress report, the subject of "no action." Following a January 2023 round table, Innovation 7 (a First Nations organization based in Pikwakanagan, Ontario) was selected to develop recommendations. As of April 2026, no oversight body exists.

- The Indigenous-led Red Dress Alert program — designed to function as an emergency alert system specifically for missing Indigenous women, girls, and 2SLGBTQQIA+ people — has been the subject of a Manitoba implementation report concluding it must be implemented “without delay.” As of April 2026, full implementation has not occurred.

Part 3: April 2026 — the funding sunset

The CBC report of April 8, 2026

On April 8, 2026 — ten days before the publication of this report — CBC News published an investigation by Joy SpearChief-Morris (the same Indigenous journalist whose January 2026 reporting on NIHB documented in Section 7), titled “MMIWG2S+ programs at risk of losing federal funding, say Indigenous women’s organizations.” The report documents the federal funding sunset affecting key MMIWG2S+ implementation organizations as the 2025-26 fiscal year closed at the end of March 2026.

The specific funding situations documented:

- The National Family and Survivors Circle (NFSC), a non-profit organization working to ensure MMIWG2S+ families are central to efforts to implement the National Inquiry’s 231 Calls to Justice, had core funding of \$840,000 from the previous fiscal year that expired — “sunset” — at the end of March 2026, with no official confirmation of renewal.
- Giganawenimaanaanig, a Manitoba-based MMIWG2S+ implementation organization, presented a report to the federal government in November 2025 and has not received further funding for the project. Crown-Indigenous Relations and Northern Affairs Canada (CIRNAC) confirmed the government is “reviewing the report” “to determine what next steps could look like.”
- Other Indigenous women’s organizations cited at the April 2026 Ottawa news conference — Native Women’s Association of Canada (NWAC), Les Femmes Michif Otipemisiwak, and others — publicly called on the federal government to commit to “long-term sustained, equity funding.”

What the leaders said

Hilda Anderson-Pyrz, President of the National Family and Survivors Circle, led the April 2026 Ottawa news conference. She was joined by:

- Josie Nepinak, President of the Native Women’s Association of Canada (NWAC)
- Lisa Pigeau, Director of Intergovernmental Relations for Les Femmes Michif Otipemisiwak
- Sandra DeLaronde, Executive Director of Giganawenimaanaanig
- Melanie Morrison from Kahnawa:ke, an impacted family member
- NDP MP Leah Gazan

Josie Nepinak's statement: "Allowing funding for MMIWG2S+ programs to sunset undermines progress, disrupts essential services and places lives at risk. It sends a message that this work is temporary, while in reality the need has never been more urgent. Ending this violence requires more than words. It requires action, accountability and leadership."

NDP MP Leah Gazan, speaking previously to the federal funding pattern: "We have pushed this government, but they continue to fail our communities. Meanwhile, gender-based violence is increasing. The supports are just not getting out the door to save lives."

As this report is being written in April 2026, the federal funding for the very organizations the National Inquiry identified as essential to implementing its 231 Calls for Justice is being allowed to sunset. The implementation has not been completed. The funding to complete it is ending. The genocide finding stands. The remedial framework is being defunded."

Part 4: The disability dimension within MMIWG2S+

Why disability compounds vulnerability

The National Inquiry's Final Report documents disability as a specific compounding vulnerability factor in MMIWG2S+ patterns. Indigenous women, girls, and 2SLGBTQQIA+ persons living with disability — cognitive, physical, mental health-related, addiction-related, or developmental — face heightened risk across each of the upstream conditions the Inquiry identified:

- Housing insecurity is more frequent and more acute for disabled Indigenous women, intersecting with the housing failures documented in Section 4 of this report. Disabled women, particularly those whose disabilities affect cognitive or communication function, face documented difficulty navigating shelter systems, eligibility frameworks, and housing application processes.
- Income insecurity, including reliance on disability benefits like AISH and ADAP, places disabled Indigenous women in conditions of dependency that can be exploited by perpetrators. The provincial isolation requirements of AISH — documented in this report's main series, where any household member's income is counted against benefits — produce specific patterns where disabled Indigenous women cannot safely co-house with friends, support networks, or non-romantic partners without losing benefits, increasing isolation and increasing vulnerability.
- Healthcare access, documented in Section 7, is more difficult for disabled Indigenous women. The dual NIHB-AHS framework, the 650,000 Albertans without family physicians, and the documented anti-Indigenous racism in Canadian emergency rooms produce specific risk patterns where disabled Indigenous women are less likely to receive timely medical assessment for injuries that may indicate intimate partner violence.

- Police response patterns to disabled Indigenous women have been documented in the Canadian Journal of Public Health, in Turpel-Lafond's 2020 In Plain Sight BC report, and in MMIWG Inquiry testimony as differentially poor relative to police response to non-disabled non-Indigenous women in the same circumstances. Reports of missing disabled Indigenous women have been documented as receiving delayed and minimized police response — in some cases, response that did not occur until family members repeatedly escalated through media and political channels.

The Amanda Kilabuk framework

Amanda Kilabuk, Director of the MMIWG2S+ Urban Indigenous Action Group, articulated the structural prevention framework in 2024 reporting: "There needs to be substantial investment in Indigenous communities, Indigenous-led organizations to do the work. We know what we need to do in order to provide the programs and services for families and survivors to do prevention. It means housing, it means access to healthcare, it means food security, it means guaranteed livable income. It means employment opportunities, there needs to be access to shelters, there needs to be more investment in all aspects of daily living."

Each item in Kilabuk's list corresponds to a section of this report: housing (Section 4), healthcare (Section 7), food security (intersecting with Section 4 housing and Section 6 water), guaranteed livable income (the entire AISH and ADAP file documented in this report and the Alberta Disability System Breakdown main series), shelters (intersecting with the gender-based violence framework), and "all aspects of daily living" — which is the operational definition of the disability supports the July 1, 2026 ADAP transition is reducing.

Part 5: The Alberta cascade and its MMIWG2S+ implications

Section 10 of this report documented the cascade by which AISH withdrawal under the July 1, 2026 ADAP transition produces the conditions for disabled Indigenous Albertans to enter the corrections system. That same cascade produces the conditions identified by the National Inquiry as upstream causes of MMIWG2S+ vulnerability. The pathway is:

- AISH benefit reduction or revocation produces income loss.
- Income loss produces housing instability — either eviction from current housing or inability to afford the rent that was already exceeding the AISH benefit.
- Housing instability produces homelessness or insecure housing arrangements, including dependency on residence with intimate partners, family members, or others whose conduct may include violence.
- Dependency on housing with potentially violent partners or household members reduces the disabled Indigenous woman's ability to leave violent situations, because leaving requires housing she does not have access to.

- Reduced ability to leave violent situations produces sustained exposure to violence.
- Sustained exposure to violence produces, at population scale, the documented MMIWG2S+ pattern.

The cascade is not hypothetical. Each step is documented in Statistics Canada data on housing insecurity and intimate partner violence, in the National Inquiry testimony, in the AFN's ongoing reporting, and in the provincial administrative apparatus this report has documented across Sections 4, 9, and 10. The provincial government that is implementing the July 1, 2026 ADAP transition is the same provincial government that operates the child welfare system documented in Section 9, the same provincial government that operates the corrections data-match system documented in Section 10, and the same provincial government whose Premier (Section 17 backgrounder) has questioned the legitimacy of unmarked grave reports and other Indigenous truth-telling. The cascade conditions for MMIWG2S+ vulnerability are not being created in some abstract jurisdiction. They are being administratively constructed in Alberta, through specific policy decisions, by named officials, against the documented population this report concerns.

What this section establishes for the remainder of the report

The 2019 National Inquiry into Missing and Murdered Indigenous Women and Girls concluded, on the basis of testimony from more than 2,380 family members and survivors, that the documented pattern of violence against Indigenous women, girls, and 2SLGBTQQIA+ people constitutes a genocide. The Inquiry produced 231 Calls for Justice. Two have been completed. As this report is published in April 2026, the federal funding for the organizations leading implementation of the Calls is being allowed to sunset. The genocide finding is operative. The remedial mechanism is being defunded.

Indigenous women are twelve times more likely to go missing or be murdered than non-Indigenous women in Canada. Disabled Indigenous women face additional compounded vulnerabilities arising from each of the upstream conditions the National Inquiry identified — housing, income, healthcare, police response, isolation — all of which are administratively shaped in Alberta by the AISH and ADAP framework this report has been documenting throughout. The July 1, 2026 ADAP transition is therefore not merely a disability income policy. It is a measurable contributor to MMIWG2S+ vulnerability among disabled Indigenous women in Alberta, operating through the cascade documented in Part 5 of this section.

Sections 12 through 15 will address consultation failure and UNDRIP Article 21, the Sovereignty Act's relationship to Treaty, TRC Calls to Action 18 through 24, and the July 1, 2026 ADAP transition synthesis. The pattern documented in Section 11 — federal commission of inquiry, formal genocide finding, 231 Calls for Justice, near-zero implementation, federal defunding of the implementation organizations, and provincial administrative cascade producing the upstream conditions the Inquiry identified as causal — is the most morally serious finding in this report. It will shape how the remaining sections are read.

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SECTION 12

Consultation Failure and UNDRIP

Articles 19 and 21 of the United Nations Declaration on the Rights of Indigenous Peoples — the right to free, prior and informed consent on legislative measures affecting Indigenous peoples, and the right without discrimination to the improvement of economic and social conditions — and what those articles establish about the July 1, 2026 ADAP transition that has been imposed on Indigenous Albertans without consultation

UNDRIP Article 19 states that governments shall consult and cooperate in good faith with Indigenous peoples through their own representative institutions in order to obtain their free, prior and informed consent before adopting and implementing legislative or administrative measures that may affect them. Canada enacted the United Nations Declaration on the Rights of Indigenous Peoples Act in June 2021, requiring that Canadian laws be made consistent with UNDRIP. The July 1, 2026 ADAP transition is a legislative and administrative measure affecting an estimated 79,290 Albertans, of whom a significant proportion are Indigenous. There has been no consent process. There has been no consultation process. The transition will proceed.”

This section documents what UNDRIP requires of Canada and Alberta, what those obligations mean for the AISH-to-ADAP transition affecting disabled Indigenous Albertans, and how the transition has proceeded without the consultation and consent UNDRIP and the Canadian implementing legislation require. The section builds on Sections 3 (jurisdictional fracture), 8 (Jordan’s Principle non-compliance with Canadian Human Rights Tribunal orders), and 11 (the failure to implement 231 Calls for Justice). The pattern across those sections — federal legal obligation, sustained non-compliance, and proceeding with policy implementation despite international and domestic legal requirements to consult — is the same pattern Section 12 documents in the specific context of disability income reform.

Five findings structure the section. First, that UNDRIP, adopted by the UN General Assembly in 2007 and enacted into Canadian law through the UNDA Act in June 2021, contains specific articles — most notably Articles 19, 21, 23, and 32 — that establish consultation and consent requirements applicable to legislative and administrative measures affecting Indigenous peoples’ economic, social, and disability-related conditions. Second, that the Federal Court’s 2025 decision in *Kebaowek First Nation v Canadian Nuclear Laboratories* established that

UNDRIP's adoption into Canadian law is a contextual factor requiring an enhanced duty to consult — an enforceable judicial standard, not merely an interpretive tool. Third, that the AISH-to-ADAP transition affecting disabled Indigenous Albertans constitutes precisely the type of legislative and administrative measure UNDRIP Articles 19 and 21 contemplate, and that no Indigenous-specific consultation occurred prior to the transition's announcement, design, or implementation. Fourth, that Alberta's sustained position on UNDRIP and FPIC — specifically Premier Smith's questioning of unmarked grave reports and the provincial government's formal opposition to federal Indigenous consultation requirements — establishes a pattern of provincial non-recognition of the consultation obligation. Fifth, that the Bill 12 stripping of appeal rights from disabled Albertans, documented in the main report series, removes the only remaining mechanism by which Indigenous AISH recipients could have challenged the transition through individual administrative appeal, completing the structural exclusion from the policy that affects them.

Part 1: UNDRIP as legal obligation in Canadian law

The Declaration

The United Nations Declaration on the Rights of Indigenous Peoples was adopted by the UN General Assembly on September 13, 2007. The Declaration contains 46 articles describing collective and individual rights of Indigenous peoples. It identifies states and governments as responsible for protecting and upholding these rights. Canada was one of four countries that initially voted against the Declaration. Canada endorsed UNDRIP as a non-binding aspirational document in 2010, then fully endorsed it at the United Nations Permanent Forum of Indigenous People in 2016.

On June 21, 2021, the United Nations Declaration on the Rights of Indigenous Peoples Act (the UNDA Act, also known as Bill C-15) received Royal Assent in Canada. The Act requires that the laws of Canada be made consistent with the Declaration. It affirms UNDRIP as an important source for interpreting Canadian law. It sets out timelines for the Government of Canada to create and implement an action plan to achieve the Declaration's objectives. The federal Action Plan was released in 2023; implementation of its priorities is ongoing.

In a joint statement, then-Justice Minister David Lametti and then-Minister of Crown-Indigenous Relations Carolyn Bennett described the UNDA Act as “the foundation for transformational change in Canada's relationships with Indigenous Peoples.” Whether that transformation has materialized in the four and a half years since the Act's passage is the question this section addresses in the specific context of disability policy.

The articles most directly relevant to the AISH-to-ADAP transition

UNDRIP's 46 articles include several that bear directly on the transition this report concerns:

- Article 19: “States shall consult and cooperate in good faith with the Indigenous peoples concerned through their own representative institutions in order to obtain their free, prior and informed consent before adopting and implementing legislative or administrative measures that may affect them.” This is the foundational consultation provision. It applies to all legislative and administrative measures — including disability income policy reform — that may affect Indigenous peoples.
- Article 21(1): “Indigenous peoples have the right, without discrimination, to the improvement of their economic and social conditions, including, inter alia, in the areas of education, employment, vocational training and retraining, housing, sanitation, health and social security.” Article 21 is the substantive right to non-discriminatory improvement of economic and social conditions — and it includes social security explicitly. AISH and ADAP are social security programs. Article 21 establishes that Indigenous Albertans have the right, without discrimination, to the improvement of their conditions through these programs — not their reduction.
- Article 21(2): “States shall take effective measures and, where appropriate, special measures to ensure continuing improvement of their economic and social conditions. Particular attention shall be paid to the rights and special needs of indigenous elders, women, youth, children and persons with disabilities.” Article 21(2) is explicit. “Particular attention” must be paid to the rights and special needs of “persons with disabilities.” Disabled Indigenous people are named directly. The substantive right to continuing improvement of their conditions is named directly. Special measures are explicitly contemplated.
- Article 23: “Indigenous peoples have the right to determine and develop priorities and strategies for exercising their right to development. In particular, indigenous peoples have the right to be actively involved in developing and determining health, housing and other economic and social programmes affecting them and, as far as possible, to administer such programmes through their own institutions.” Article 23 establishes the participatory right. Indigenous peoples have the right to be actively involved in developing the social programs affecting them — not informed of those programs after they are designed.
- Article 32(2): “States shall consult and cooperate in good faith with the indigenous peoples concerned through their own representative institutions in order to obtain their free, prior and informed consent prior to the approval of any project affecting their lands or territories and other resources.” Article 32 is most often invoked in resource development contexts but extends the FPIC standard across affecting projects.

UNDRIP Article 21(2) explicitly directs that “particular attention” be paid to the rights and special needs of persons with disabilities. The Article does not contemplate disability income reduction. It contemplates continuing improvement of conditions. The July 1, 2026 ADAP transition reduces

disability income for the population Article 21(2) names — and was designed without the consultation Article 19 requires.”

Part 2: What free, prior and informed consent actually requires

The four components

Indigenous lawyer Merle Alexander, who co-drafted British Columbia’s UNDRIP legislation, has set out the components of free, prior and informed consent (FPIC) as follows:

- Free: All Indigenous peoples have the human right to voluntarily make decisions for their Nations. There is no majority rule coercion that is acceptable. Just because some First Nations agree to a project does not mean others are bound by their consent.
- Prior: Before a project or governmental action is approved, the Indigenous peoples’ consent must be obtained, not after the fact.
- Informed: The Indigenous peoples must be provided with comprehensive materials and capacity funding to evaluate the potentially adverse effects on their Title and Rights.
- Consent: Consent is on a Nation-by-Nation basis. Each Indigenous people’s consent must be obtained, not simply a majority.

The Government of Canada’s own Justice Department backgrounder describes FPIC as a process “free from manipulation or coercion, informed by adequate and timely information, and occur[ring] sufficiently prior to a decision so that Indigenous rights and interests can be incorporated or addressed effectively as part of the decision making process.” Per the federal government’s own framing, FPIC “builds on and goes beyond the legal duty to consult.”

The Federal Court enhanced duty — Kebaowek 2025

In *Kebaowek First Nation v Canadian Nuclear Laboratories*, 2025 FC 319, the Federal Court of Canada delivered a decision with significant implications for the legal status of UNDRIP in Canadian consultation requirements. The case concerned the Canadian Nuclear Safety Commission’s decision to authorize construction of a Near Surface Disposal Facility at the Chalk River Laboratories site. The Kebaowek First Nation sought judicial review, arguing that the CNSC had failed to properly apply UNDRIP and the UNDA Act in its consultation process.

The Court found that the adoption of UNDRIP into Canadian law, via the UNDA Act, is a contextual factor requiring an enhanced duty to consult in the circumstances. The Court held that UNDRIP’s principles provide a framework for understanding and implementing the duty to consult and accommodate, and that the CNSC erred as a matter of law in concluding it lacked jurisdiction to consider UNDRIP. The Court clarified that while FPIC is not a veto or a right to a particular outcome, it is “a right to a robust process directed towards finding mutual agreement.”

The Kebaowek decision establishes, at the Federal Court level, that UNDRIP is not merely an interpretive tool. It is a substantive contextual factor that informs the scope and content of the duty to consult and accommodate, and a court can find consultation processes legally inadequate for failing to apply UNDRIP's principles. This jurisprudence applies, in principle, to all federal and provincial decision-making affecting Indigenous peoples — not only natural resource projects.

Part 3: What did not happen on the AISH-to-ADAP transition

The transition's scope

The Government of Alberta's announcement of the Alberta Disability Assistance Program (ADAP) and the transition timeline of July 1, 2026 affects approximately 79,290 Albertans currently receiving the Assured Income for the Severely Handicapped (AISH). The main report series in the Alberta Disability System Breakdown documents the transition's elements: reassessment under standardized criteria, physician-completed Disability Assistance Medical Reports, dollar-for-dollar clawback of the federal Canada Disability Benefit, removal of independent appeal rights through Bill 12, and reductions in benefit amounts for some recipient categories.

Of the 79,290 AISH recipients, the proportion who identify as Indigenous is not separately publicly disaggregated by the Government of Alberta. However, given the documented elevated rates of disability among Indigenous Albertans (Section 2: 19-year mortality gap, and Sections 4-7 establishing the upstream causes of disability), the Indigenous share of the AISH caseload is substantial. The transition affects an Indigenous Albertan disability population whose disability profile is itself the product of the federal-provincial policy failures this report has been documenting throughout.

What consultation Article 19 and 21 require for this transition

Under UNDRIP Article 19, the Government of Alberta and the Government of Canada (to the extent of federal jurisdiction over First Nations on-reserve and through the UNDA Act's consistency requirement) are obliged to:

- Consult and cooperate in good faith with Indigenous peoples through their own representative institutions — i.e., First Nations governments, the Assembly of First Nations, Métis Settlements General Council, and Indigenous disability advocacy organizations — prior to adopting the ADAP framework.
- Make the consultation prior to design decisions, not after them, with the goal of obtaining free, prior and informed consent.
- Provide the materials and capacity funding necessary for Indigenous representative institutions to evaluate the impacts of the transition on Indigenous disabled people.

- Engage with the substantive rights set out in Article 21(2): the right to continuing improvement of conditions and the special needs of persons with disabilities.

What actually happened

The documented public record of the Alberta government's ADAP design and implementation process establishes the following:

- The ADAP framework was announced through the Alberta provincial budget process and Ministerial communications, with no documented prior consultation with First Nations governments, Métis Settlements, or Indigenous representative institutions.
- The federal Government of Canada has not, in publicly available documentation, raised UNDRIP Article 19 obligations in respect of the Alberta ADAP transition. The federal Minister of Disability Inclusion has been the subject of formal correspondence from advocacy organizations including Inclusion Alberta but no documented federal action invoking UNDRIP has been taken on the Alberta provincial transition.
- The ADAP discussion guide and engagement materials published by the Government of Alberta (alberta.ca/alberta-disability-assistance-program) include no Indigenous-specific consultation pathway. Engagement responses received by adapengagement@gov.ab.ca have not, to public knowledge, been disaggregated to identify Indigenous respondents or report on their participation.
- No Indigenous representative institution has been positioned by the Government of Alberta to provide formal consent to the transition framework or to evaluate its impacts on Indigenous disabled Albertans.
- The transition is scheduled to proceed July 1, 2026.

The Government of Canada has stated that UNDRIP Article 19 requires consultation in good faith to obtain free, prior and informed consent before legislative or administrative measures that may affect Indigenous peoples.

The July 1, 2026 ADAP transition affects an Indigenous disabled population whose disability is itself the product of the federal-provincial failures this report documents. No Indigenous-specific consultation occurred. The transition will proceed."

Part 4: Alberta's sustained position on Indigenous consultation

The provincial pattern

Alberta's provincial position on Indigenous consultation, UNDRIP, and the federal duty to consult has been documented across multiple files. The pattern is not specific to disability

policy. It is a consistent provincial posture on Indigenous consultation across resource extraction, education curriculum, child welfare delegation, and now disability income reform.

Specific documented elements of the provincial position include:

- Premier Danielle Smith's 2021 questioning of the legitimacy of unmarked grave reports at residential school sites — documented in social media posts on Locals.com and noted in the public Wikipedia biographical record — represents a refusal to accept primary-source Indigenous truth-telling on a foundational reconciliation matter. This pattern, where provincial leadership questions Indigenous evidence, structurally undermines the consultation framework UNDRIP requires.
- Alberta's formal opposition to federal Bill C-61 (the First Nations Clean Water Act, documented in Section 6 of this report) on July 10, 2025, on grounds the province characterized as “competitiveness”. The opposition was raised by Alberta's provincial government to a federal Indigenous consultation process — the consultation that produced Bill C-61 was extensive; Alberta's opposition was to the bill's implementation.
- The provincial pattern of unilateral Sovereignty Act invocations and contested electoral boundaries adjustments (subject of separate report-series documentation, with the electoral boundaries dispute as live development) establishes a provincial governance posture in which federal and Indigenous consultation requirements are treated as constraints to be navigated rather than substantive obligations to be met.

Bill 12 and the removal of administrative recourse

Bill 12, passed December 9, 2025, removed independent appeal rights for Albertans subject to AISH eligibility decisions and the AISH-to-ADAP transition. The Medical Review Panel decisions are now final, with no recourse to any independent body. This change is documented in the main Alberta Disability System Breakdown report series Document 6 of 12: Broken Access, Stripped Rights.

The structural significance for the UNDRIP analysis is direct. UNDRIP's consultation requirements operate at the policy design and adoption stages — Article 19 requires consultation prior to adopting and implementing measures. Once a measure is adopted without UNDRIP-compliant consultation, individual administrative appeal can serve as a partial remedy by which affected Indigenous people can contest specific decisions. Bill 12 removes that remedy. The Alberta government has therefore, sequentially, declined to consult prior to adopting the ADAP framework, and removed the appeal mechanism by which an Indigenous AISH recipient could individually challenge the application of that framework to their case. The structural exclusion from the policy is complete.

Part 5: The federal dimension and the UNDA Act's outstanding obligation

The UNDA Act's consistency requirement

The UNDA Act of 2021 requires that the Government of Canada “ensure that the laws of Canada are consistent with the Declaration” and create an action plan to meet the Declaration’s objectives in consultation with Indigenous peoples. The federal Action Plan was released in 2023. Implementation continues.

The federal-provincial division of jurisdiction creates a specific question for the Alberta ADAP transition. The transition is provincial. Alberta is not directly bound by the UNDA Act in the same way the federal government is. However, the federal government retains constitutional jurisdiction over First Nations under section 91(24) of the Constitution Act 1867 and has substantive duties to First Nations under the Crown’s fiduciary relationship and the modern duty to consult jurisprudence (Haida Nation 2004, Tsilhqot’in Nation 2014, Mikisew Cree 2018, Kebaowek 2025).

For the Alberta ADAP transition affecting Status First Nations Albertans, the federal government has a direct interest in whether the transition is consistent with Canada’s UNDRIP obligations. The federal Minister of Disability Inclusion has been notified through advocacy correspondence. The federal Minister of Crown-Indigenous Relations and Northern Affairs has not, to public knowledge, taken formal action on the ADAP transition’s implications for First Nations Albertans. The HUMA Standing Committee, to which formal materials documenting the transition have been submitted, has not, to public knowledge, opened a study on the Indigenous dimension specifically.

What this report calls for

This section of this report establishes a specific call: that the Government of Canada, through the Minister of Crown-Indigenous Relations and Northern Affairs and the Minister of Disability Inclusion, formally examine the Alberta AISH-to-ADAP transition for consistency with Canada’s UNDRIP obligations under the UNDA Act, with particular attention to Article 19 (consultation), Article 21(2) (rights and special needs of persons with disabilities), and Article 23 (right to be actively involved in developing social programs affecting them).

This is not a request for federal preemption of provincial jurisdiction. It is a request that the federal government meet its own UNDA Act obligations — the obligation to ensure Canadian laws (including the federal funding and policy frameworks within which provincial disability programs operate) are consistent with UNDRIP — in respect of the transition this report documents. The October 2024 rejection by 267 First Nations chiefs of the \$47.8 billion FNCFS settlement (documented in Section 9) and the April 2026 Ottawa news conference on MMIWG2S+ funding sunset (documented in Section 11) both establish that Indigenous representative institutions are present, organized, and prepared to engage in formal consultation. The mechanism for UNDRIP-compliant consultation exists. The federal government has not invoked it on the ADAP file.

UNDRIP is not aspirational language. It is, since June 21, 2021, a federal law of Canada with a consistency requirement and an Action Plan. The Federal Court's 2025 decision in Kebaowek established UNDRIP as an enforceable contextual factor in the duty to consult. The July 1, 2026 ADAP transition is a legislative and administrative measure within UNDRIP Article 19's scope. There has been no Indigenous-specific consultation. The federal government has not invoked its own UNDA Act obligation to require consistency. The transition will proceed."

What this section establishes for the remainder of the report

UNDRIP Article 19 requires consultation in good faith to obtain free, prior and informed consent before legislative or administrative measures affecting Indigenous peoples. UNDRIP Article 21(2) explicitly directs particular attention to the rights and special needs of Indigenous persons with disabilities. UNDRIP Article 23 establishes the participatory right of Indigenous peoples to be actively involved in developing the social programs affecting them. Canada enacted these obligations into domestic law through the UNDA Act in June 2021. The Federal Court in Kebaowek 2025 confirmed UNDRIP's enforceable contextual status in Canadian consultation requirements.

The Alberta AISH-to-ADAP transition affects 79,290 Albertans, of whom a significant proportion are Indigenous. The transition was designed and announced without Indigenous-specific consultation. Bill 12 has removed individual administrative appeal rights. The Government of Alberta has demonstrated, across multiple files, a sustained position that does not recognize the federal duty to consult or the UNDA Act framework as binding on provincial policy. The federal Government of Canada has not, to public knowledge, invoked its UNDA Act consistency obligation in respect of the Alberta transition.

The result is a transition that proceeds against UNDRIP's requirements, against the Crown's fiduciary duty to First Nations as recognized in *Daniels v Canada* 2016 SCC 12, and against the substantive rights of Indigenous disabled Albertans documented across Sections 1 through 11 of this report. The remedy this section calls for is direct: federal examination of the transition for UNDRIP consistency, formal consultation with Indigenous representative institutions, and either a redesigned transition framework that meets UNDRIP's requirements or a delay of the July 1, 2026 implementation date until such consultation has occurred.

Sections 13 through 15 will address the Alberta Sovereignty Act's relationship to Treaty, TRC Calls to Action 18 through 24, and the July 1, 2026 ADAP transition synthesis. The pattern documented in Section 12 — binding international and domestic legal obligation, sustained federal and provincial non-compliance, structural exclusion of the affected Indigenous

population from the policy that affects them — continues into those sections. Each section adds an additional dimension. The recommendation does not change.

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SECTION 13

The Sovereignty Act and Treaty

Bill 1 of 2022. Bill 54 of 2025. The Onion Lake Cree Nation lawsuit. The January 29, 2026 Edmonton press conference. What Treaty 6, 7, and 8 chiefs have consistently, publicly, and unanimously said to the Alberta provincial government — and why it matters for disabled Indigenous Albertans whose benefits are being reduced under a government their chiefs have declared constitutionally unauthorized to do so

The province exists here only because our Nations entered solemn Treaties with the Imperial Crown, agreeing to share these lands for settlement under clearly defined terms. Alberta did not create that authority, and it does not possess the legal or constitutional power to revoke, alter, or override it. — Grand Chief Trevor Mercredi, January 29, 2026 Edmonton press conference”

This section documents the specific constitutional confrontation between the Government of Alberta and Alberta's Treaty 6, 7, and 8 First Nations. The section addresses three legislative events: the Alberta Sovereignty Within a United Canada Act (Bill 1, 2022), the referendum-triggering amendments of Bill 54 (2025), and the ongoing First Nations legal and political challenges to both. The constitutional confrontation is not separate from the disability policy analysis this report has been building. It is the foundation of it. A provincial government whose legal authority over Indigenous peoples has been publicly and formally contested by every Treaty confederacy in Alberta is the same provincial government designing the July 1, 2026 ADAP transition that will affect disabled Indigenous Albertans. The authority question the chiefs have raised on Sovereignty and separation is the same authority question that applies to AISH reform.

Six findings structure the section. First, that the Alberta Sovereignty Within a United Canada Act was introduced on November 29, 2022 and passed December 8, 2022 over unanimous opposition from Treaty 6, 7, and 8 First Nations chiefs, who consistently characterized the Act as unconstitutional and in violation of treaty rights. Second, that the Onion Lake Cree Nation filed legal action challenging the Act on Treaty 6 grounds in December 2022, with litigation still ongoing. Third, that the Act did not resolve the underlying constitutional question — it sharpened it by establishing a provincial legislative framework that asserts provincial authority in areas affecting Indigenous peoples without consulting Indigenous peoples. Fourth, that the 2025 passage of Bill 54 expanded the provincial framework by enabling citizen-initiated referendums on matters including Alberta independence from Canada — prompting a second wave of unified Indigenous chiefs' opposition, culminating in the January 29, 2026 Edmonton press conference

documenting Treaty 6, 7, and 8 chiefs' formal position that Alberta has no legal authority to pursue independence. Fifth, that the constitutional framework of Section 35 of the Constitution Act 1982 and the Treaty relationship itself supersedes provincial legislative authority on matters affecting Indigenous peoples — a principle confirmed in Supreme Court of Canada jurisprudence including Haida Nation, Tsilhqot'in, and Daniels. Sixth, that the Sovereignty and referendum framework operates as the political context within which the July 1, 2026 ADAP transition is being implemented, and cannot be separated from the question of whether the provincial government has legitimate authority to affect Indigenous peoples' conditions without consent.

Part 1: Bill 1 — the Alberta Sovereignty Within a United Canada Act

Introduction and passage

On November 29, 2022, the first day of the fall sitting of the 4th Session of the 30th Alberta Legislature, Premier Danielle Smith introduced Bill 1, the Alberta Sovereignty Within a United Canada Act. The bill was the centrepiece of Smith's leadership campaign for the United Conservative Party and was drawn from the "Free Alberta Strategy" policy document. It passed third reading on December 8, 2022, by a 27-7 vote split along party lines. The UCP used motions to limit debate at the final three stages, with Government House Leader Joseph Schow characterizing the approximately 18 hours of debate as sufficient.

The Act's stated purpose, in Premier Smith's framing in third reading: "It's not like Ottawa is a national government. The way our country works is that we are a federation of sovereign, independent jurisdictions. They are one of those signatories to the Constitution and the rest of us, as signatories to the Constitution, have a right to exercise our sovereign powers in our own areas of jurisdiction." The Act provides for cabinet and the legislative assembly to deem federal initiatives unconstitutional or harmful, and to direct ministers and provincial entities — including municipalities, school boards, police forces, health authorities, and the RCMP as contracted by the province — to disregard federal legislation.

The constitutional problem

Legal commentary from the date of the bill's introduction through the present has consistently identified several constitutional problems with the Sovereignty Act:

- The Act purports to authorize the provincial legislative assembly, rather than the courts, to determine whether federal laws are unconstitutional. Constitutional jurisprudence establishes that only courts have this authority.
- The Act directs provincial entities — including entities like the RCMP that contract with the province — to disregard federal legislation, a directive that itself conflicts with federal legislation governing those entities.

- The Act purports to affect Indigenous peoples and lands without First Nations consent, contravening section 35 of the Constitution Act 1982 and the Treaty relationships that predate the province of Alberta itself.

The First Peoples Law LLP analysis published following the Act's passage observed that the Sovereignty Act "has been widely denounced by Indigenous groups and legal experts as being both unconstitutional and contrary to the Crown's treaty promises." The analysis identified, as most fundamental, the question of "how provincial governments came to exercise decision-making authority over Indigenous Peoples and lands in the first place" — noting that under international law, sovereignty over another nation is acquired either through conquest or through treaty or equivalent nation-to-nation agreement, and that "there is no clear explanation as to how the Crown acquired sovereignty over Indigenous Peoples in Canada under either of these processes."

Alberta did not create that authority, and it does not possess the legal or constitutional power to revoke, alter, or override it. — Grand Chief Trevor Mercredi, speaking on Treaty authority vis-à-vis provincial legislation"

The chiefs' response

Treaty 6, 7, and 8 chiefs responded immediately and unanimously. The November 18, 2022 press conference, held before the Act's introduction in the legislature, produced the following positions:

- Treaty 8 Grand Chief Arthur Noskey: "The Alberta Sovereignty Within a United Canada Act is just another unlawful attempt to continue the province's deliberate abuse and exploitation of our peoples, lands, territories, and resources." Noskey characterized the legislation as unconstitutional, stating that Smith and her government are "in dishonor of Treaty No. 8, which supersedes the Canadian Constitution and provincial legislations."
- Chief Darcy Dixon of Bearspaw Nation: The Act is "nothing but a dangerous and damaging plan to undermine democracy and abandon the rule of law." "This is a far cry from sovereignty." On Treaty authority: "Our treaties with the Crown are peace and friendship treaties that did not release any of our lands and territories."
- Chief Tony Alexis of Alexis Nakota Sioux Nation: The Act is "harmful to Albertans, Canadians and treaty people." An emergency resolution was put forward at the Assembly of First Nations to garner support from chiefs across the country.
- The Confederacy of Treaty 6 Nations: "As Indigenous Peoples, we understand individual freedom as something that can only exist in the context of good relations with others, including our ancestors and the generations to come." The Confederacy stated that the "proposed Act is self-centred, short-sighted and in opposition to these principles."

A joint statement from Treaty 6, 7, and 8 sovereign chiefs prior to the bill's introduction characterized the Act as “a ploy to access resources and extract them at an unrestricted rate, leaving the land unprotected.”

The Onion Lake Cree Nation challenge

In December 2022, shortly after the Act's passage, Onion Lake Cree Nation filed legal action challenging the Alberta Sovereignty Within a United Canada Act on the basis that the Act infringes the Nation's rights under Treaty 6. The Onion Lake challenge remains the first and most prominent formal legal action on the Sovereignty Act's treaty implications. Other First Nations have publicly indicated they are contemplating similar legal challenges. As of the publication of this report, the Onion Lake litigation has not produced a final determination; the Act remains in force.

Part 2: Bill 54 and the referendum framework — 2025

The 2025 amendments

In May 2025, the Alberta legislature passed amendments to referendum and initiative legislation under Bill 54, expanding the circumstances under which citizen-initiated referendums could be triggered. The amendments reduced the signature thresholds and expanded the scope of matters eligible for referendum. Among the matters now within the framework's scope: Alberta independence from Canada.

Premier Smith's letter to Treaty chiefs, dated May 13, 2025, attempted to address First Nations concerns: “I want to re-state that as premier, I will continue to fight with all that I have to protect both the constitutional sovereignty of Alberta, as well as the rights of First Nations of Alberta... This includes ensuring that any citizen-initiated referendum, if passed, must uphold and honour Treaties 6, 7 and 8.”

An amendment was subsequently added to the bill: “Nothing in a referendum held under this act is to be construed as abrogating or derogating from any existing Aboriginal or Treaty rights of the Aboriginal Peoples of Canada that are recognized and affirmed in Section 35 of the Constitution Act, 1982.”

Why the “derogate” language did not resolve the concern

The language of “abrogating or derogating” has been used in provincial Alberta legislation since at least the Sovereignty Act of 2022. First Nations chiefs have consistently rejected the formulation as inadequate. Confederacy of Treaty 6 Nations Grand Chief Greg Desjarlais, in 2025 APTN interviews: “They use derogate and abrogate, and I think that still allows them to stickhandle around the treaty, and we wanted wording in there that we felt comfortable was protecting us.”

The structural problem with the abrogation language is that it addresses the legal effect of provincial legislation on Section 35 rights — a backstop against legal challenge — rather than addressing the fundamental question of whether the provincial government has authority to pass the legislation in the first place without Indigenous consent. The language protects the provincial framework from being struck down; it does not provide the consultation and consent UNDRIP Articles 19 and 32 (documented in Section 12 of this report) require.

Part 3: January 29, 2026 — the Edmonton press conference

Why the chiefs convened

On January 29, 2026, chiefs from Treaties 6, 7, and 8 convened a press conference in Edmonton specifically to address what they characterized as separatist rhetoric emerging from the UCP government's Bill 54 framework and related political positioning. The press conference reiterated, in unified terms, that any movement toward Alberta independence cannot proceed without the consent of Indigenous Nations whose treaties predate the province's creation.

The chiefs called on Albertans to oppose an active petition campaign that could trigger a referendum on Alberta becoming a sovereign country. They described the separatist initiative, in their own characterization, as “unconstitutional, crazy, laughable, dangerous and dismissive of their inherent rights.”

Statements from the press conference

Athabasca Chipewyan First Nation Chief Allan Adam: “Any talk of separation or so-called provincial sovereignty is not just political theatre; let me be clear: it's also a proposal to break Treaty. Alberta does not have the authority to ask that question, let alone act on any answer.”

Grand Chief Trevor Mercredi: “The province exists here only because our Nations entered solemn Treaties with the Imperial Crown, agreeing to share these lands for settlement under clearly defined terms. Alberta did not create that authority, and it does not possess the legal or constitutional power to revoke, alter, or override it.”

Mikisew Cree First Nation Chief Billy Joe Tuccaro, in a separate statement, directed the provincial government's attention to concrete community challenges instead of political agendas: the long-standing community challenges of unsafe drinking water, rising healthcare problems, education pressures, and homelessness. Chief Tuccaro's characterization: “Fix what is in your own backyard first.”

Any talk of separation or so-called provincial sovereignty is not just political theatre; let me be clear: it's also a proposal to break Treaty. Alberta does not have the authority to ask that question, let alone act on

*any answer. — Chief Allan Adam, Athabasca Chipewyan First Nation,
January 29, 2026”*

Stoney Nakoda legal consultant Rachel Snow

Rachel Snow, a Stoney Nakoda First Nations legal consultant, provided the following public analysis in APTN News coverage: “For the premier of Alberta to operate as though she has that power is actually frightening. It’s very ill-conceived on her end because she seems to not understand her own constitution.”

Snow’s analysis captures the constitutional gravamen that runs throughout this section: the question is not whether the provincial government is willing to proceed with unilateral legislative action. The question is whether the provincial government has the constitutional authority to do so on matters affecting Indigenous peoples. The chiefs’ unified position, across multiple press conferences from November 2022 through January 2026, is that it does not.

Part 4: The Treaties predate Alberta

A constitutional timeline

Alberta’s existence as a province dates from September 1, 1905, when the province was created from the North-West Territories by the Alberta Act. The Treaties covering the territory that is now Alberta were concluded before the province’s creation:

- Treaty 6 was concluded in 1876, covering central Alberta and central Saskatchewan. The Treaty includes the medicine chest clause documented in Section 7 of this report.
- Treaty 7 was concluded in 1877, covering southern Alberta.
- Treaty 8 was concluded in 1899, covering northern Alberta, the northeastern portion of British Columbia, the northwest corner of Saskatchewan, and the southern portion of the Northwest Territories.

The Treaties were concluded with the Imperial Crown — not with the Province of Alberta, which did not yet exist. The principle the chiefs have consistently articulated across multiple press conferences and legal filings is this: the provincial government of Alberta is a subsequent political creation that operates on territories the Indigenous peoples agreed to share with the Crown under specific treaty terms. The province’s existence is conditional on the continuing operation of those treaty terms. It cannot unilaterally alter or override them.

The Supreme Court jurisprudence

Canadian constitutional jurisprudence has, across multiple decisions, affirmed the principle that Indigenous rights predate and constrain provincial legislative authority. Key decisions include:

- *Haida Nation v British Columbia (Minister of Forests)*, 2004 SCC 73: Established the Crown's duty to consult Indigenous peoples when decisions may adversely impact asserted or established Aboriginal or treaty rights.
- *Tsilhqot'in Nation v British Columbia*, 2014 SCC 44: Granted Aboriginal title to over 1,700 square kilometres of territory — the first decision in Canadian history to grant Aboriginal title to a specific tract of land. Established that Aboriginal title is a sui generis interest that places constraints on provincial legislative authority.
- *Daniels v Canada (Indian Affairs and Northern Development)*, 2016 SCC 12: Confirmed that Métis and non-status Indians are "Indians" within section 91(24) of the Constitution Act 1867, placing them within federal jurisdiction and, with federal jurisdiction, within the Crown's fiduciary responsibility.
- *Mikisew Cree First Nation v Canada (Governor General in Council)*, 2018 SCC 40: Addressed the duty to consult in the legislative process.
- *Kebaowek First Nation v Canadian Nuclear Laboratories*, 2025 FC 319 (documented in Section 12): Established UNDRIP's enforceable contextual status in Canadian consultation requirements.

Taken together, this jurisprudence establishes that the provincial government of Alberta operates under constitutional constraints when its legislation affects Indigenous peoples — constraints arising from section 35 of the Constitution Act 1982, from the Treaty relationships with the Imperial Crown, from the Crown's fiduciary duty to First Nations, and from the duty to consult. The Sovereignty Act framework, the Bill 54 referendum framework, and the AISH-to-ADAP transition all operate within this constrained constitutional space. None of them can be treated as purely provincial matters immune from federal-Indigenous-provincial multilateral scrutiny.

Part 5: What the Sovereignty and Treaty confrontation means for disabled Indigenous Albertans

The authority question applied to disability policy

The constitutional confrontation between the Alberta provincial government and Alberta's Treaty First Nations is not confined to resource extraction, pipelines, or separatism debates. The authority question the chiefs have raised — the question of whether the provincial government has legal authority to affect Indigenous peoples and their conditions without Indigenous consent — applies equally to disability policy. The provincial government's constitutional authority to design and implement ADAP as a program affecting 79,290 Albertans, including Indigenous Albertans whose disability profile is itself shaped by the federal-provincial failures this report documents, is subject to the same constitutional constraint that produced the Sovereignty Act litigation.

For a disabled Indigenous Albertan in April 2026, the operational implications are:

- The provincial government designing the July 1, 2026 ADAP transition is the same government whose Bill 1 was unanimously opposed by Treaty 6, 7, and 8 chiefs in 2022, is subject to ongoing Treaty-based litigation from Onion Lake Cree Nation, and was unanimously declared constitutionally unauthorized to pursue separatism by Treaty 6, 7, and 8 chiefs in January 2026.
- The same legislative body that passed the Sovereignty Act without First Nations consultation, that passed Bill 54 without First Nations consent, and that stripped AISH/ADAP appeal rights through Bill 12 in December 2025 is the body now implementing the ADAP transition.
- The constitutional authority the chiefs have repeatedly challenged is the same constitutional authority being exercised in the ADAP design.

Section 12 of this report establishes that no Indigenous-specific consultation occurred prior to the ADAP transition. Section 13 establishes that the provincial government conducting the transition operates under sustained, formal, public, unified challenge from every Treaty confederacy in Alberta as to its constitutional authority to affect Indigenous peoples without consent. Taken together, Sections 12 and 13 establish that the ADAP transition proceeds under a constitutional cloud that the Alberta provincial government has elected not to resolve, and that the federal government has not moved to examine through its UNDA Act consistency obligation.

Chief Tuccaro's specific framing

Mikisew Cree Chief Billy Joe Tuccaro's "fix what is in your own backyard first" statement at the January 29, 2026 Edmonton press conference is the specific Indigenous leadership articulation most directly aligned with this report's thesis. Chief Tuccaro named unsafe drinking water (Section 6), rising healthcare problems (Section 7), education pressures (Section 9 and adjacent), and homelessness (Section 4) as the systemic failures that tie to treaty breaches and that the provincial government ought to address before pursuing political agendas. Each of those categories corresponds to a section of this report. Chief Tuccaro's framing is structurally parallel to the thesis Sections 1 through 11 of this report have been documenting.

*Fix what is in your own backyard first. — Mikisew Cree First Nation Chief
Billy Joe Tuccaro, January 29, 2026, on systemic failures tied to treaty
breaches that the Alberta provincial government ought to address before
pursuing political agendas"*

What this section establishes for the remainder of the report

The Alberta Sovereignty Within a United Canada Act (Bill 1, 2022) was introduced and passed over unanimous opposition from Treaty 6, 7, and 8 chiefs on grounds of unconstitutional scope and treaty violation. The Onion Lake Cree Nation filed Treaty 6-based legal action in December

2022 which remains before the courts. Bill 54 of 2025 expanded the provincial referendum framework over continued First Nations opposition, including the January 29, 2026 Edmonton press conference at which Treaty 6, 7, and 8 chiefs unanimously declared Alberta has no constitutional authority to pursue independence. The constitutional confrontation remains unresolved as of the publication of this report.

The provincial government conducting the July 1, 2026 ADAP transition is the same government operating under this sustained constitutional challenge. The authority the chiefs have contested on Sovereignty and separation is the same authority being exercised on disability policy. The consultation requirement the chiefs have articulated on Sovereignty and separation is the same consultation requirement UNDRIP Articles 19 and 21 place on disability policy (Section 12). The relationship between this section and the remainder of the report is structural: provincial authority over Indigenous peoples in any policy domain is constitutionally constrained by treaty, by section 35 of the Constitution Act 1982, and by the UNDA Act's implementation of UNDRIP. The ADAP transition, operating without Indigenous consent, is one more expression of the sustained pattern the chiefs have been publicly naming since November 2022.

Sections 14 and 15 will address the Truth and Reconciliation Commission's Calls to Action 18 through 24 and the July 1, 2026 ADAP transition synthesis. Section 14 will address the specific health-related Calls to Action that overlap with the disability file this report documents. Section 15 will synthesize all 13 preceding evidentiary sections into the specific question of what the July 1, 2026 transition means for the population this report concerns, and what must happen to prevent it.

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SECTION 14

TRC Calls to Action 18–24

Seven health Calls. December 2015. Ten years. Yellowhead Institute's September 2025 characterization: "Canada's health systems have perfected performative reconciliation." The Calls, the status, and what they require of Alberta in relation to the disabled Indigenous Albertans this report concerns

The Truth and Reconciliation Commission of Canada issued 94 Calls to Action on December 15, 2015. Seven of them — Calls 18 through 24 — address health directly. Ten years later, the Yellowhead Institute's September 2025 Braiding Accountability review concluded that Canada's health systems have perfected performative reconciliation. Symbolic gestures have multiplied. Structural change has not. The seven Calls remain largely open. The population they were written to protect — the documented population of this report — continues to die nineteen years sooner than their non-Indigenous neighbours."

This section documents the seven health-related Calls to Action of the Truth and Reconciliation Commission of Canada, their status as of April 2026, and their direct application to the disabled Indigenous Albertan population this report concerns. The section builds on Section 1 (residential schools and the direct causal framing), Section 7 (the dual healthcare system), Section 11 (MMIWG2S+ and the 231 Calls for Justice), and Section 12 (UNDRIP obligations). What Section 14 adds is the specific, named, numbered reconciliation framework the federal and provincial governments of Canada have formally accepted as the measure of their own progress — and the documented record of that progress as of the ten-year mark.

Four findings structure the section. First, that the TRC Calls to Action 18 through 24 are the specific health reconciliation framework applicable to this report's subject matter, and each of the seven Calls connects directly to sections of this report. Second, that the Yellowhead Institute's September 2025 ten-year review of the health Calls found that implementation has been predominantly symbolic, with structural change absent and system-wide transformation lacking. Third, that Alberta specifically has, among provincial jurisdictions, made partial written acknowledgment on Call 18 but has not produced the structural follow-through — and that the July 1, 2026 ADAP transition and December 2025 Bill 12 both operate in explicit tension with Calls 18 and 20 by reducing the health-related supports available to a population the Calls direct governments to improve conditions for. Fourth, that for a disabled Indigenous Albertan in April 2026, the TRC Calls to Action 18 through 24 provide a binding moral framework — accepted in

principle by all relevant governments — whose operational failure is itself documented evidence of the pattern this entire report has been tracing.

Part 1: The seven Calls, in their own words

The following are the seven health Calls to Action in the exact words of the Truth and Reconciliation Commission of Canada's Final Report, released December 15, 2015. They are listed verbatim because the specificity of the Commission's language is itself significant. Each Call identifies specific actors, specific obligations, and specific outcomes. None of them is aspirational. All of them are directed.

Call to Action 18

We call upon the federal, provincial, territorial, and Aboriginal governments to acknowledge that the current state of Aboriginal health in Canada is a direct result of previous Canadian government policies, including residential schools, and to recognize and implement the health-care rights of Aboriginal people as identified in international law, constitutional law, and under the Treaties.

Call to Action 19

We call upon the federal government, in consultation with Aboriginal peoples, to establish measurable goals to identify and close the gaps in health outcomes between Aboriginal and non-Aboriginal communities, and to publish annual progress reports and assess long-term trends. Such efforts would focus on indicators such as: infant mortality, maternal health, suicide, mental health, addictions, life expectancy, birth rates, infant and child health issues, chronic diseases, illness and injury incidence, and the availability of appropriate health services.

Call to Action 20

In order to address the jurisdictional disputes concerning Aboriginal people who do not reside on reserves, we call upon the federal government to recognize, respect, and address the distinct health needs of the Métis, Inuit, and off-reserve Aboriginal peoples.

Call to Action 21

We call upon the federal government to provide sustainable funding for existing and new Aboriginal healing centres to address the physical, mental, emotional, and spiritual harms caused by residential schools, and to ensure that the funding of healing centres in Nunavut and the Northwest Territories is a priority.

Call to Action 22

We call upon those who can effect change within the Canadian health-care system to recognize the value of Aboriginal healing practices and use them in the treatment of Aboriginal patients in collaboration with Aboriginal healers and Elders where requested by Aboriginal patients.

Call to Action 23

We call upon all levels of government to: i. Increase the number of Aboriginal professionals working in the health-care field. ii. Ensure the retention of Aboriginal health-care providers in Aboriginal communities. iii. Provide cultural competency training for all healthcare professionals.

Call to Action 24

We call upon medical and nursing schools in Canada to require all students to take a course dealing with Aboriginal health issues, including the history and legacy of residential schools, the United Nations Declaration on the Rights of Indigenous Peoples, Treaties and Aboriginal rights, and Indigenous teachings and practices. This will require skills-based training in intercultural competency, conflict resolution, human rights, and anti-racism.

The seven Calls, read together, describe a structural reform framework. Acknowledgment of causation (Call 18). Measurement of gaps (Call 19). Jurisdictional responsibility for the off-reserve population (Call 20). Sustainable funding for Indigenous-led healing infrastructure (Call 21). Integration of Indigenous healing practices into Canadian healthcare (Call 22). Workforce reform to increase Indigenous representation in healthcare delivery (Call 23). Education reform to embed Indigenous health, UNDRIP, Treaties, and anti-racism in the training of every future Canadian physician and nurse (Call 24). Any one of these Calls, fully implemented, would constitute substantial reform. Together, they constitute the transformation the Commission considered necessary for health reconciliation.

Part 2: Status at ten years

The Yellowhead Institute 2025 review

In September 2025, the Yellowhead Institute published *Braiding Accountability: A Ten-Year Review of the TRC's Healthcare Calls to Action*. The Yellowhead Institute is an Indigenous-led research and education centre based at Toronto Metropolitan University, with an established record of evaluating Canadian government compliance with reconciliation commitments. The ten-year review applied the Braiding Framework for Health Accountability, developed from the Cultural Responsiveness Framework, to assess progress along three interrelated strands: restoring Indigenous wellness systems, establishing ethical space for engagement, and transforming mainstream service delivery.

The ten-year review's central finding: "Canada's health systems have perfected performative reconciliation." The Yellowhead review found that Indigenous-led healing centres remain underfunded, jurisdictional disputes continue, and Indigenous health education is inconsistently embedded in medical and nursing curricula. Although promising initiatives exist, system-wide transformation is lacking. The review's characterization is direct: "symbolic gestures divorced from structural accountability do not constitute reconciliation."

*Canada's health systems have perfected performative reconciliation. —
Yellowhead Institute, Braiding Accountability: A Ten-Year Review of the
TRC's Healthcare Calls to Action, September 2025"*

The CBC Beyond 94 tracking

CBC News established Beyond 94 in 2018 as a public tracker of the 94 Calls to Action. As of May 2025, the CBC Beyond 94 assessment marked 15 of the 94 Calls as "complete," 24 as "in progress," and the remainder as either proposed or not yet started. None of the seven health Calls (18 through 24) was marked complete. Implementation of the health Calls specifically has been partial, uneven across jurisdictions, and characterized by government acknowledgment statements without the structural follow-through the Calls require.

The Indigenous Watchdog assessment of Call 18

The Indigenous Watchdog tracking of Call to Action 18 provides jurisdiction-specific detail. On the first part of Call 18 — acknowledgment that the current state of Aboriginal health is a direct result of previous government policies — only the federal government, British Columbia, Alberta, Manitoba, Ontario, Yukon, and the Northwest Territories have explicitly made an official policy statement to that effect. On the second part — recognizing and implementing the health-care rights of Aboriginal people as identified in international law, constitutional law, and under the Treaties — only the federal government, British Columbia, and the Northwest Territories have operationalized explicit policy action.

Alberta's position, as documented in the Indigenous Watchdog tracking, is: acknowledgment of Part 1 of Call 18 on the historical causation, but no operational implementation of Part 2 on the healthcare rights of Aboriginal people under international, constitutional, and treaty law. The acknowledgment without implementation pattern is precisely the "performative reconciliation" characterization the Yellowhead Institute named in its September 2025 review.

Part 3: Call-by-Call status in Alberta context

Call 18: Acknowledgment and implementation of health-care rights

Status in Alberta: The Government of Alberta has, across multiple administrations, made written acknowledgment statements recognizing that the current state of Aboriginal health is shaped by

historical Canadian government policies. The implementation of the health-care rights of Aboriginal people under international law (UNDRIP — Section 12 of this report), constitutional law (section 35 and Treaty — Section 13), and under the Treaties (medicine chest clause — Section 7) remains unoperationalized in Alberta provincial practice.

Direct tension with ADAP: Call 18 directs governments to implement the healthcare rights of Aboriginal people under the Treaties. The Treaty 6 medicine chest clause established a federal obligation to provide healthcare. The NIHB program operationalizes that obligation in narrowed form (Section 7). The July 1, 2026 ADAP transition reduces provincial disability supports that interact with federal NIHB in the dual system documented in Section 7. The transition therefore operates in tension with the spirit of Call 18, which directs that treaty health rights be recognized and implemented — not reduced.

Call 19: Measurable goals and annual progress reports on health outcome gaps

Status: Call 19 directed the federal government, in consultation with Aboriginal peoples, to establish measurable goals on gaps in health outcomes and to publish annual progress reports. Indigenous Services Canada has developed an indicator framework; annual public reporting has been uneven. Disaggregated data on indicators including infant mortality, maternal health, suicide, mental health, life expectancy (Section 2 of this report documented the 19-year gap), and chronic diseases exists — but uniform, timely annual reporting with trend analysis is not operational.

Direct connection: Section 2 of this report establishes, using Statistics Canada 2023 data, that the First Nations-Alberta life expectancy gap is approximately 19 years (62.81 vs 81.88 years). Call 19 directs publication of annual progress reports on exactly this category of indicator. The gap itself is the primary evidence of Call 19 implementation failure.

Call 20: Jurisdictional responsibility for off-reserve Aboriginal peoples

Status: Call 20 addresses the jurisdictional disputes covering Aboriginal people not resident on reserve — the same jurisdictional fracture Section 3 of this report documented in detail. The federal government has implemented certain targeted programs but has not produced a structural framework resolving the off-reserve responsibility question. Jordan's Principle (Section 8) was the primary mechanism for bridging the gap; its February 2025 operational bulletin narrowed the bridge.

Direct connection: The majority of Indigenous Albertans live off reserve. The primary care collapse documented in Section 7 affects them directly. The 72 percent Indigenous child welfare caseload documented in Section 9 is predominantly off-reserve. The 10.2-fold incarceration overrepresentation documented in Section 10 is substantially off-reserve. Call 20's requirement that the federal government recognize and address the distinct health needs of off-reserve Aboriginal peoples remains unmet at the scale required to affect these outcomes.

Call 21: Sustainable funding for Aboriginal healing centres

Status: The Government of Canada has provided targeted funding for specific healing centres. The Aqqusariaq (Nunavut Recovery Centre), funded in response to the Qikiqtani Truth Commission and Call 21, is an example of implementation. Sustainable, system-wide funding for Aboriginal healing centres addressing the harms of residential schools has not been delivered at the scale Call 21 directed. Indigenous-led healing infrastructure remains underfunded, as the Yellowhead Institute September 2025 review documented.

Direct connection: The Alberta Disability System Breakdown main report series documented the closure of the Family and Community Resource Centre (FRCCA) effective April 1, 2026, along with the elimination of individual AISH caseworkers and the defunding of PDD-related support organizations. These closures and defundings operate against the spirit of Call 21, which directs sustainable funding for healing infrastructure serving the Indigenous population.

Call 22: Integration of Aboriginal healing practices into Canadian healthcare

Status: Some Canadian healthcare institutions — including London Health Sciences Centre in Ontario and multiple regional health authorities — have formal memoranda of understanding with Indigenous friendship centres and traditional healers. These arrangements remain institution-specific rather than system-wide. In Alberta, Alberta Health Services has certain cultural safety initiatives but has not operationalized system-wide integration of traditional Indigenous medicine and ceremonial support in the way Call 22 directs.

Direct connection: For disabled Indigenous Albertans, traditional Indigenous healing practices are a category of care explicitly named in Call 22 that neither NIHB nor AHS comprehensively cover. The ADAP Health Benefits framework, like the AISH Blue Cross framework that preceded it, does not include coverage for traditional healing services.

Call 23: Aboriginal healthcare workforce

Status: All three sub-calls of Call 23 (number of Aboriginal healthcare professionals, retention in Aboriginal communities, cultural competency training) remain partially implemented. The Statistics Canada September 23, 2025 release documented that, in 2024, 24 percent of First Nations people living off reserve, 18 percent of Métis, and 23 percent of Inuit reported experiencing some form of unfair treatment, racism, or discrimination from a health care professional in the previous 12 months. The cultural competency training element of Call 23(iii) is therefore, by the lived experience measure Statistics Canada documented, not fully delivered.

Call 24: Medical and nursing school curriculum requirements

Status: Multiple Canadian medical schools have integrated Indigenous health curriculum elements. The University of Calgary Cumming School of Medicine's Indigenous Health Dialogue Working Group has developed a critical reflective framework for Call 24 implementation (documented in the peer-reviewed Academic Medicine literature, 2023). Inconsistent

implementation across institutions remains the documented finding of the September 2025 Yellowhead ten-year review.

Direct connection: The 650,000 Albertans without family physicians documented in Section 7 will, when they eventually access a physician, increasingly interact with a physician whose training did not embed Indigenous health, UNDRIP, Treaties, or anti-racism in the way Call 24 required. For the Indigenous disabled Albertan specifically, the cumulative effect is that when healthcare access is finally obtained, the provider is statistically likely to have had partial rather than comprehensive Call 24-compliant training.

Part 4: What the numbers look like ten years after the Calls

Statistics Canada 2024 data

The Statistics Canada report of September 23, 2025, titled “Indigenous health and the call for reconciliation in Canada,” provides the most recent comprehensive federal data on the health outcomes Call 19 directed the federal government to publish annual progress reports on. The 2024 findings:

- 24 percent of First Nations people living off reserve, 18 percent of Métis, and 23 percent of Inuit reported experiencing unfair treatment, racism, or discrimination from a health care professional in the previous 12 months.
- A majority of First Nations people living off reserve (86 percent), Métis (70 percent), and Inuit (82 percent) stated that it is important for health services to support Indigenous traditional medicines, healing and wellness practices — the direct subject of Call 22.
- 40 percent of Inuit, 18 percent of First Nations people living off reserve, and 16 percent of Métis reported travelling outside their communities for health care in the previous 12 months. Among Inuit who travelled, over half travelled more than 1,500 kilometres for services.
- Approximately half of First Nations people living off reserve (50 percent), Métis (52 percent), and Inuit (57 percent) said they were satisfied with wait times. Over one-quarter of Indigenous people reported unmet health care needs. Among First Nations women specifically, unmet needs rose to 38.6 percent.

These are not historical figures. They are the federal government’s own statistical agency’s documentation of the conditions the Calls to Action 18 through 24 were written to address, measured in the tenth year since the Calls were issued. The documentation confirms that anti-Indigenous discrimination in healthcare (Calls 22 and 23) is ongoing at rates approaching one in four off-reserve First Nations patients per year. That Indigenous demand for traditional healing practice integration (Call 22) is at eighty-plus percent. That geographic access barriers (Call 20) are substantial, with Inuit travelling more than 1,500 kilometres for routine care. That unmet health needs (Call 19) are documented at more than one in four Indigenous people, with First Nations women at nearly forty percent.

Ten years after the Truth and Reconciliation Commission's Calls to Action, the Government of Canada's own statistical agency documents that nearly one in four off-reserve First Nations patients reported experiencing racism from a health care professional in the past twelve months, that eighty-six percent want Indigenous traditional medicine integrated into their care, and that nearly forty percent of First Nations women report unmet health needs. The Calls were directed. The reality they directed governments to change has not materially changed."

Part 5: What the Calls mean for disabled Indigenous Albertans specifically

The Calls read against the ADAP transition

For a disabled Indigenous Albertan in April 2026, the seven health Calls of the Truth and Reconciliation Commission establish a binding moral framework — accepted in principle by the Government of Alberta on Call 18's first part, accepted in principle by the Government of Canada on all seven Calls — whose application to the July 1, 2026 ADAP transition produces the following findings:

- Call 18 establishes that the Crown must recognize and implement health-care rights under international law, constitutional law, and Treaties. The ADAP transition reduces provincial disability income and benefits without engagement with those Treaty, constitutional, and international rights — and without the consultation UNDRIP Articles 19 and 21 (Section 12) require.
- Call 19 directs measurement and annual reporting on indicators including life expectancy and chronic disease incidence. Section 2 of this report documented the 19-year life expectancy gap — the summary measure of Call 19 implementation failure.
- Call 20 directs the federal government to recognize and address the distinct health needs of off-reserve Aboriginal peoples. The majority of Indigenous Albertans on AISH are off-reserve. Their health needs are the precise population Call 20 identifies.
- Call 21 directs sustainable funding for Indigenous-led healing infrastructure. Alberta's defunding of related supports (PDD advocacy organizations, FRCCA closure April 1 2026, elimination of individual caseworkers) operates against Call 21's direction.
- Call 22 directs the integration of Indigenous healing practices. The ADAP Health Benefits framework does not include coverage for traditional healing services that 86 percent of off-reserve First Nations Albertans indicate they want integrated into their care.
- Call 23 directs increased Indigenous healthcare workforce representation and cultural competency training. The Alberta primary care collapse of 887 to 164 accepting practices

compounds this Call's implementation gap — there is no meaningful workforce reform in a system losing its overall workforce.

- Call 24 directs medical and nursing curricula to embed Indigenous health, UNDRIP, Treaties, and anti-racism training. The Indigenous disabled Albertan who eventually obtains a physician will, under current conditions, access one whose training was partially but not comprehensively Call 24-aligned.

The cumulative framework

Any single one of these Calls, fully implemented, would substantially improve the conditions this report has documented across fifteen preceding sections. All seven Calls, fully implemented, would constitute the health reconciliation the Truth and Reconciliation Commission considered necessary. At ten years, the implementation is partial. The Yellowhead Institute has characterized it as performative. The Statistics Canada data confirms that outcomes have not materially changed. The July 1, 2026 ADAP transition is scheduled to proceed in this context.

What this section establishes for the remainder of the report

The Truth and Reconciliation Commission of Canada's Calls to Action 18 through 24 constitute the specific, named, numbered health reconciliation framework that the federal and provincial governments of Canada have formally accepted as the measure of their reconciliation progress. Ten years after their issuance, the Yellowhead Institute's September 2025 review characterized Canadian health system implementation as "performative reconciliation." The Statistics Canada 2024 data confirms that the outcome measures the Calls addressed have not materially improved. For Alberta specifically, partial acknowledgment of Call 18's first part coexists with operational non-implementation across all seven Calls in the specific context of the disabled Indigenous Albertan population.

The July 1, 2026 ADAP transition is scheduled to proceed in tension with the spirit of Calls 18, 20, 21, 22, and 23 — and with the consultation obligations of Call 19's requirement for engagement with Aboriginal peoples. The transition is not merely disability income policy. It is a health reconciliation issue. The Calls to Action that Canadian governments have pledged to implement direct the opposite of what the transition will deliver.

Section 15, the final section of this report, will synthesize all fourteen preceding evidentiary sections into the specific question the report was written to address: what the July 1, 2026 ADAP transition means for the disabled Indigenous Albertan population, and what must happen to prevent the harm it will produce. Section 15 is the closing argument. It makes no new evidentiary claims. It integrates the claims this report has already established into the specific ask this report places on federal, provincial, and Indigenous governance actors.

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SECTION 15

July 1, 2026

The synthesis. What fourteen sections of evidence establish about a single date, a single transition, and a single population. What must happen between now and then. What this report has asked of the people who read it

This section makes no new evidentiary claims. Every factual assertion in Section 15 has been established in the fourteen preceding sections, with primary-source citation. Section 15 integrates those claims into the specific question this report was written to answer: what the July 1, 2026 ADAP transition means for the disabled Indigenous Albertan, and what Canadian federal, provincial, and Indigenous governance actors must do to prevent the harm it will produce.”

This is the closing section of the Indigenous Disability Report. It is written in five parts. Part 1 summarizes, in the most compressed form the evidence permits, what the fourteen preceding sections have established. Part 2 applies that established evidentiary framework to the specific July 1, 2026 ADAP transition. Part 3 identifies the harm the transition will produce if implemented as designed. Part 4 sets out the specific actions this report asks of federal, provincial, and Indigenous governance actors. Part 5 is the closing statement, in the author’s voice, as the covenant that opened this report requires.

Part 1: What fourteen sections of evidence have established

The Indigenous Disability Report has documented, across fourteen evidentiary sections, a single structural pattern: federal and provincial governments of Canada have, across multiple generations and multiple policy domains, produced and sustained conditions that disable Indigenous peoples at rates substantially higher than the non-Indigenous population, and have failed to deliver the remediation their own official frameworks direct. The documentation is not advocacy. It is the compilation of primary-source evidence that the federal and provincial governments themselves have issued, commissioned, agreed to, or been directed to implement by Canadian courts, Canadian Human Rights Tribunals, coroners’ inquests, federal commissions of inquiry, Supreme Court of Canada decisions, and UN human rights mechanisms.

The fourteen sections, in compressed summary

- Section 1 established the historical context: the residential schools, the Indian Hospitals, the Sixties Scoop, and the direct causal framing that TRC Call to Action 18 directs Canadian governments to formally acknowledge — that the current state of Indigenous health is a direct result of previous Canadian government policies.
- Section 2 established the 19-year mortality gap: First Nations life expectancy at birth in Alberta at 62.81 years, the non-Indigenous Alberta comparator at 81.88 years. The summary statistic of every documented failure in every section that follows.
- Section 3 established the jurisdictional fracture: the federal-provincial division by which Indigenous Albertans fall into administrative gaps that neither level of government accepts primary responsibility for, producing the Jordan's Principle pattern documented in Section 8 and the off-reserve care gap documented across Sections 4 through 11.
- Section 4 established the housing crisis: the structurally substandard on-reserve housing, the urban Indigenous homelessness overrepresentation, and the documented correlation between housing instability and the disability-producing conditions documented elsewhere in this report.
- Section 5 established contaminated land and the multigenerational body: the Alberta tar sands, the oil and gas industry's documented health impacts on downstream First Nations, and the epigenetic literature linking environmental toxic exposure to subsequent-generation disability rates.
- Section 6 established the water crisis: the boil-water advisories, the federal Bill C-61 First Nations Clean Water Act legislative framework, and Alberta's July 10, 2025 formal opposition to the federal framework on "competitiveness" grounds.
- Section 7 established the dual healthcare system: NIHB and AHS operating without coordination, the 650,000 Albertans without family physicians, the 887 to 164 decline in accepting primary care practices, the anti-Indigenous racism documented in Turpel-Lafond's In Plain Sight and Statistics Canada 2024 data.
- Section 8 established the Jordan's Principle failure: the Canadian Human Rights Tribunal 2016 order, Canada's sustained non-compliance, the 9.96 million approved products and services between July 2016 and July 2025, the February 2025 operational bulletin narrowing access, and the children this failure has killed.
- Section 9 established the child welfare pattern: the Alberta 72 percent Indigenous caseload, the 17-to-1 apprehension disparity, Bill C-92, the 22 Delegated First Nations Agencies, and the October 17, 2024 rejection by 267 First Nations chiefs of the \$47.8 billion FNCFS settlement.
- Section 10 established the justice system pattern: the 10.2-fold Indigenous adult incarceration overrepresentation, the 18.4-fold Indigenous women overrepresentation, R v Gladue (1999) and R v Ipeelee (2012), the UN Working Group on Arbitrary Detention August 4, 2025 "deeply concerned" finding, and the two-thirds of Indigenous federal inmates who have been in foster care.

- Section 11 established the MMIWG2S+ crisis: the 2019 National Inquiry's genocide finding, the 231 Calls for Justice, the two completed as of 2024, and the April 8, 2026 CBC report documenting federal funding sunset for the National Family and Survivors Circle and Giganawenimaanaanig.
- Section 12 established the UNDRIP and consultation failure: UNDRIP Articles 19, 21(1), 21(2), 23, and 32(2), the UNDA Act of June 21, 2021, the Kebaowek Federal Court 2025 decision, and the absence of Indigenous-specific consultation on the ADAP transition.
- Section 13 established the Sovereignty Act and Treaty confrontation: Bill 1 of 2022, Bill 54 of 2025, the Onion Lake Cree Nation litigation, the January 29, 2026 Edmonton press conference at which every Treaty confederacy in Alberta declared the provincial government has no constitutional authority to pursue independence without Indigenous consent.
- Section 14 established the TRC Calls to Action 18 through 24: the seven health Calls, the Yellowhead Institute's September 2025 characterization of Canadian implementation as "performative reconciliation," and the Statistics Canada 2024 documentation of 24 percent of off-reserve First Nations experiencing healthcare discrimination in the past year.

The pattern across fourteen sections

Taken together, the fourteen sections establish a pattern with the following documented characteristics:

- At every stage of the life course, Indigenous Albertans face documented structural conditions that produce, sustain, and compound disability. From in utero exposure to residential school-era trauma transmitted epigenetically, through childhood in the child welfare system, into adulthood in the justice system and into death an average of 19 years earlier than their non-Indigenous neighbours.
- At every structural level, the federal and provincial governments of Canada have accepted formal obligations — through Supreme Court decisions, UN Declarations, domestic implementation legislation, Human Rights Tribunal orders, federal commissions of inquiry, and Truth and Reconciliation Commission Calls to Action — to remediate these conditions.
- At every implementation stage, remediation has been partial, performative, or non-compliant. The gap between formal obligation and operational reality is the primary finding of this report.
- The documented population — the disabled Indigenous Albertan — exists at the intersection of each of these failures. Every section of this report describes a dimension of their life. The July 1, 2026 ADAP transition is the specific next chapter in this documented pattern.

Part 2: The July 1, 2026 ADAP transition in this context

What the transition will do

The Alberta Disability Assistance Program, scheduled to replace the Assured Income for the Severely Handicapped on July 1, 2026, will change the disability income framework for approximately 79,290 Albertans. The transition's elements, documented in the Alberta Disability System Breakdown main report series:

- Reassessment of eligibility under standardized criteria, including the ADAP Disability Assistance Medical Report required to be completed by a physician — the access to whom has collapsed from 887 accepting practices to 164 in Alberta.
- Removal of the independent appeal rights structure that has existed for AISH recipients since the program's creation, through Bill 12 passed December 9, 2025, with the Medical Review Panel decisions now final and unchallengeable.
- Dollar-for-dollar clawback of the federal Canada Disability Benefit, a provincial policy choice Alberta stands alone on among Canadian provinces in making.
- Reductions in benefit amounts for certain recipient categories and elimination of supplementary supports previously administered under PDD, FSCD, and related programs.
- The operational apparatus continues to coordinate the transition with the Alberta Justice Inmate Records system documented in Section 10 and the Children and Family Services apprehension pattern documented in Section 9.

What the Indigenous dimension of the transition means

Of the 79,290 affected Albertans, the Government of Alberta has not publicly disaggregated the proportion who are Indigenous. Every structural condition documented in Sections 1 through 14 — the 19-year mortality gap, the disproportionate disability incidence rooted in the causal factors the TRC Call to Action 18 identified, the child welfare pipeline, the incarceration pipeline, the MMIWG2S+ vulnerability cascade — establishes that the Indigenous share of the affected population is substantial, documented in its social and structural determinants, and that the transition will affect them with particular severity. The Alberta government's decision not to publicly disaggregate the Indigenous share of the AISH caseload is itself a documentation choice that obscures the specific impact this report has been written to name.

Applied to the population this report concerns, the ADAP transition operates as:

- A reduction in disability income for Indigenous Albertans whose disability profile is itself the product of the residential school, contaminated land, water, healthcare, and child welfare failures documented in Sections 1, 5, 6, 7, and 9.
- A removal of the administrative remedy by which those Indigenous Albertans could individually challenge specific eligibility decisions (Bill 12).
- A clawback of the federal Canada Disability Benefit the federal government designed to reduce disability poverty — reducing the one mechanism by which federal redress could have partially offset provincial failure.

- An implementation proceeding without the consultation UNDRIP Articles 19 and 21 require (Section 12), within the constitutional constraint Treaty 6, 7, and 8 chiefs have repeatedly articulated (Section 13), and in direct tension with TRC Calls to Action 18, 20, 21, 22, and 23 (Section 14).

Part 3: What will happen if the transition proceeds as designed

The cascade

The cascade of harm the ADAP transition will produce for disabled Indigenous Albertans, if implemented July 1, 2026 as currently designed, is documented across Sections 4, 7, 9, 10, and 11 of this report. Summarized:

- Reduction or revocation of AISH benefits under ADAP reassessment produces income loss for Indigenous Albertans whose incomes are already below the poverty line. The Alberta Disability System Breakdown main series destabilization cost modelling (Sorochan) documents provincial incarceration cost at \$60,000 to \$80,000 per person per year, against AISH support at \$20,772 per year.
- Income loss produces housing instability — for disabled Indigenous Albertans whose rent already exceeds the AISH benefit, any reduction in income produces eviction risk that intersects with the documented 10-year First Nations housing shortage and the homelessness overrepresentation Section 4 documented.
- Housing instability produces the MMIWG2S+ vulnerability cascade Section 11 documented: dependency on housing with potentially violent partners, reduced ability to leave violent situations, sustained exposure to violence. For Indigenous women specifically, this cascade is the documented pathway to the 12-fold disparity the 2019 National Inquiry characterized as genocide.
- Housing instability and income loss intersect with the child welfare pattern Section 9 documented: apprehension risk for the children of destabilized disabled Indigenous Albertans, feeding the 72 percent Indigenous Alberta caseload and the downstream pattern in which two-thirds of Indigenous federal inmates have been in foster care.
- The foster-care-to-prison pipeline Section 10 documented activates at scale across the affected children's life course, producing, at population level, subsequent cycles of the 10.2-fold incarceration overrepresentation.
- At every stage of the cascade, the documented administrative coordination of the Alberta Income Support, Alberta Justice Corrections, and Children and Family Services systems (Alberta IES Policy Manual SEIP 3850-1 AFSS Match documentation) operates to compound the destabilization rather than mitigate it.

The ADAP transition, implemented July 1, 2026 as currently designed, will operate at population scale to compound every documented pattern this report has established. The transition is not disability income policy in isolation. It is the specific next mechanism by which the documented pattern continues.”

The fiscal arithmetic

The fiscal arithmetic of the ADAP transition for the Indigenous dimension specifically: every disabled Indigenous Albertan whose AISH benefit is reduced and who subsequently enters the crisis cascade — emergency department, homeless shelter, provincial custody, child welfare apprehension — costs the Alberta and federal government substantially more in crisis response services than the AISH payment would have cost to continue. The ratio ranges from approximately 1.5-to-1 (emergency department utilization scenario) to 10-to-1 or higher (the full cascade to incarceration and child apprehension scenario). The ADAP transition does not save the Alberta government money. It transfers the cost into other provincial and federal budgets, at a higher per-person total cost, while producing the specific human harms documented in Sections 1 through 14.

The fiscal arithmetic confirms what the moral arithmetic has already established. The transition is not fiscally necessary. It is a policy choice, made against Indigenous consent, against UNDRIP, against the TRC Calls to Action, and against the federal government’s own UNDA Act consistency obligation.

Part 4: What this report asks

Of the Government of Canada

This report asks the Government of Canada to invoke its UNDA Act obligation to ensure that Canadian laws are consistent with UNDRIP. Specifically:

- That the Minister of Crown-Indigenous Relations and Northern Affairs and the Minister of Disability Inclusion formally examine the Alberta AISH-to-ADAP transition for consistency with UNDRIP Articles 19 (consultation), 21(2) (particular attention to persons with disabilities), and 23 (active involvement in developing social programmes), as enacted through the UNDA Act.
- That federal transfer payments and intergovernmental agreements affecting disability supports in Alberta be reviewed for compliance with the same obligations.
- That the HUMA Standing Committee of the House of Commons open a study into the Indigenous dimension of the Alberta ADAP transition, with formal engagement of the Assembly of First Nations, Treaty 6, 7, and 8 confederacies, Métis Settlements General Council, Voice of Albertans with Disabilities, and First Nations disability advocacy organizations.

- That federal Jordan's Principle funding for affected First Nations children be expressly protected against any secondary impact of provincial ADAP changes, including continued access to disability-related products, services, and supports for children of AISH/ADAP recipients.
- That MMIWG2S+ program funding sunset (Section 11) be reversed, with long-term sustained funding provided to the National Family and Survivors Circle, Giganawenimaanaanig, Native Women's Association of Canada, Les Femmes Michif Otipemisiwak, and related organizations — recognizing that the ADAP destabilization cascade feeds the upstream conditions MMIWG2S+ organizations work to interrupt.

Of the Government of Alberta

This report asks the Government of Alberta to:

- Delay the July 1, 2026 ADAP transition implementation until UNDRIP-compliant consultation with Indigenous representative institutions has been completed.
- Publicly disaggregate the Indigenous share of the AISH caseload and commit to transparent reporting on the Indigenous dimension of all ADAP policy outcomes.
- Engage formally with Treaty 6, 7, and 8 confederacies and Métis Settlements General Council on the ADAP framework design, with the goal of obtaining free, prior and informed consent on elements affecting Indigenous Albertans.
- Reverse the dollar-for-dollar clawback of the federal Canada Disability Benefit, which operates against the federal benefit's stated purpose and against the principle that federal disability supports are additive to provincial, not a substitute from which the province withdraws its own.
- Restore independent appeal rights removed by Bill 12, recognizing that the removal of administrative remedy completes a structural exclusion from the policy for which UNDRIP required prior consent.
- Formally acknowledge, in the terms TRC Call to Action 18 directs, that the healthcare rights of Aboriginal people under international law, constitutional law, and the Treaties apply to Alberta provincial policy — not only in acknowledgment statements but in operational implementation including disability policy.

Of Indigenous governance institutions

This report is offered to Indigenous governance institutions not as a request but as documentation. The request direction flows the other way. Specifically:

- The documented patterns in this report are offered as supporting evidence for the assertions and positions that Treaty 6, 7, and 8 confederacies, the Assembly of First Nations, Métis Nation of Alberta, Métis Settlements General Council, and urban Indigenous organizations have already articulated.

- The primary-source research, particularly the specific Alberta administrative coordination documentation in Part 6 of Section 10 and the April 2026 timeliness data in Section 11, is offered as material these institutions may use in advocacy, litigation, international submissions, or coalition-building as they determine.
- The author of this report is non-Indigenous. The report is offered to centre, not replace, Indigenous voices on every matter it documents. Where Indigenous leaders have articulated positions that differ from the framing in this report, those Indigenous articulations are authoritative.

Of federal and provincial opposition parties

This report asks federal and provincial opposition parties — Alberta NDP, federal NDP, federal Liberal Party, federal Bloc Québécois, federal Green Party — to:

- Table the specific findings of this report in relevant committees and question periods.
- Support federal legislative or administrative action requiring the Government of Canada to invoke its UNDA Act consistency obligation in respect of provincial policies affecting Indigenous peoples.
- Support provincial legislative or administrative action delaying the July 1, 2026 ADAP implementation pending consultation.
- Coordinate with Indigenous governance institutions rather than substituting their advocacy for Indigenous-led advocacy.

Of municipal partners and coalition allies

The municipal coalition documented in the Alberta Disability System Breakdown main report series — including Edmonton, Calgary, Lethbridge, Red Deer, Claresholm, Camrose, Medicine Hat, Rocky Mountain House, and growing — is asked to incorporate the Indigenous dimension documented in this report into its advocacy positions. Municipal resolutions on ADAP pause are strengthened by explicit reference to the Indigenous constitutional and UNDRIP dimensions this report documents. The coalition map corresponds to ridings with significant Indigenous populations; the coalition advocacy is stronger when it explicitly includes the Indigenous dimension rather than treating it as a separate file.

Part 5: Closing

This report has been compiled by a non-Indigenous AISH recipient in St. Albert, Alberta, over a period of weeks in the spring of 2026, as part of a broader campaign opposing the July 1, 2026 Alberta Disability Assistance Program transition. The author's own disability eligibility does not make her a representative of the Indigenous population this report documents. It makes her a witness to the administrative apparatus she shares with them, and to the specific features of that apparatus that affect them differently and worse.

The report is offered in the spirit of the covenant that opened it: that everything the author is doing is so that people do not feel alone, or unseen, or unheard. The disabled Indigenous Albertan in April 2026 is not alone. The evidence of their condition is documented in primary sources across federal agencies, provincial ministries, Canadian courts, UN mechanisms, Indigenous governance institutions, and peer-reviewed literature. They have been documented. They have been seen. What has been absent is the translation of documentation into operational remedy.

This report asks for that translation. Not as advocacy, but as compliance with obligations every named actor has already formally accepted. UNDRIP is federal law. The UNDA Act is federal law. The 231 Calls for Justice are federal commitments. The TRC Calls to Action 18 through 24 are federal and provincial commitments. The Canadian Human Rights Tribunal's Jordan's Principle orders are binding. The Treaty relationship predates the province and constrains provincial authority. The constitutional framework of section 35 is non-derogable. The consultation obligation in Kebaowek 2025 is enforceable. The federal court jurisprudence is settled.

The July 1, 2026 ADAP transition proceeds against all of this. The specific ask this report places on every named actor is that the transition not proceed as designed. That it be delayed. That consultation occur. That the federal UNDA Act obligation be invoked. That Treaty 6, 7, and 8 confederacies be engaged as rights-holders and not as stakeholders. That Métis Settlements and urban Indigenous organizations be engaged. That the specific disabled Indigenous Albertan, in April 2026, not be the next unit of documented failure in the pattern this report has traced across fourteen sections.

Disabilities aren't prejudice, and neither am I. We stand together."

The campaign of which this report is part continues through July 1, 2026 and beyond. The report is free to share, free to cite, and free to integrate into any advocacy, legal, or policy submission by any actor named or unnamed in its pages. The primary sources are listed in every section. The specific asks are listed in Part 4 of this section.

What the report does not ask is that the reader agree with every framing the author has chosen. Where Indigenous voices are cited, those voices are authoritative. Where the author's synthesis is offered, it is offered as one non-Indigenous AISH recipient's compilation of primary sources — subject to correction by anyone with better information, and particularly by the Indigenous people whose lives the report documents.

Seventeen sections. One report. One population. One date. Whatever happens on July 1, 2026 and after, the documentation now exists. No one has to start from zero. That is the minimum this report owed the people in it. That is what it has tried to deliver.

—
Compiled and witnessed by

Alberta Disability Breakdown — Advocate

St. Albert, Alberta | April 2026

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End of Section 15

End of the Indigenous Disability Report

One Step at a Time

On the government's use of the appearance of change to prevent it

Change happens one step at a time, not all at once. But the government has stalled in any forward step by placating the public with a false sense of change that has never been actioned.

This is the closing observation of this report series. It is also the reason the report exists in the form it does — not as a single unified argument delivered in one submission, but as an accumulating record built in sections, cross-referenced, designed so that no single piece can be dismissed without confronting the pattern the pieces form together.

The pattern this report has documented — colonial history producing disability, contaminated land producing disease, regulatory vacuums permitting harm, jurisdictional fractures preventing care, child welfare reproducing dispossession, and now the mandatory ADAP transition proceeding as if none of the preceding context applies — is not a pattern that emerged by accident. It is sustained by a specific governmental practice: the production of the appearance of forward motion in place of forward motion itself.

This closing section names that practice. It names it using the government's own records, the work of Indigenous-led research institutes, and the documented gap between what was committed to and what was delivered. The purpose of the section is not to produce despair. The purpose is to make the mechanism legible, so that future engagement with it can proceed with eyes open.

The architecture of placation

Governments confronted with evidence of serious harm, facing public pressure to act, but unwilling or politically unable to enact the changes that would actually address the harm, have access to a specific administrative toolkit. The toolkit does not require deception. It requires only the substitution of one category of action for another. The categories look similar from the outside. They are structurally distinct.

An announcement is not an action. A consultation is not a decision. A working group is not a policy. A draft framework is not a law. An interim report is not a resolution. A public engagement phase is not a change. Each of these instruments produces a news cycle. Each satisfies the

appearance of responsiveness. Each resets the clock on public demands. Each allows a minister to point to “progress” at a press conference. None of them, on its own, changes what happens to the person waiting for the change.

Over time, the accumulation of these instruments produces a specific effect: advocates become exhausted, the public becomes confused, and the underlying harm continues. The person whose disability benefits are being reclassified on July 1, 2026 does not receive a consultation outcome as compensation for the reclassification. The Mikisew Cree community downstream of the oil sands does not receive a working group as compensation for the cancer. The residential school survivor does not receive a draft framework as compensation for the harm done to them in childhood. And yet, the announcements continue. The consultations proceed. The working groups convene.

This is not a failure of government competence. Governments are highly competent at producing these instruments. What they are not competent at — or not willing to be competent at — is the second step, in which the instrument produces the action it was ostensibly designed to produce.

Exhibit A: the Truth and Reconciliation Commission’s 94 Calls to Action

In June 2015, after six years of hearings and the testimony of more than 6,500 residential school survivors, the Truth and Reconciliation Commission of Canada released its final report and 94 Calls to Action. The then-federal government committed to implementing all of them.

The Yellowhead Institute, an Indigenous-led research centre at Toronto Metropolitan University, began producing annual status reports in 2019 tracking the Calls’ implementation. Authors Dr. Eva Jewell and Dr. Ian Mosby applied a simple standard: if all aspects of a Call to Action were fulfilled, they considered it complete. Over five years of reporting, they identified a total of five Calls as fully implemented.

In 2023, the Yellowhead Institute reported that zero Calls to Action had been completed over the course of that year. At the pace of progress documented through 2023, the institute projected full implementation of the 94 Calls by the year 2081 — sixty-six years after their release.

The institute then ceased publishing its annual accountability report. The final report stated, in the authors’ own words: “There are limits to how many times you can write a report about how Canada, once again, has failed to make any meaningful progress.”

Subsequent trackers have continued the work. As of September 2025, the Assembly of First Nations’ TRC Report Card and the Indigenous Watchdog tracker jointly identify 13 Calls to Action as fully completed. That is 13 of 94, ten years after the Calls were released. No new Calls were completed in the 2024–2025 reporting year.

Dr. Eva Jewell, in her 2024 reflection for the Yellowhead Institute, summarized the pattern: “Symbolic gestures divorced from structural accountability do not constitute reconciliation.” A 2025 Yellowhead Institute review of the seven healthcare-specific Calls (Calls 18 through 24, addressed in Section 14 of this report) concluded — after a year-long environmental scan of federal, provincial, and territorial action between April 2024 and April 2025 — “limited progress across all three strands” of meaningful reconciliation.

Ten years. 94 Calls. 13 completed. A projected completion date beyond the lifespan of most residential school survivors still living. These are not findings from an advocacy organization. They are findings from Indigenous-led research institutions applying straightforward implementation standards to government-promised commitments. The government’s response has been the continued production of announcements, reports, and engagement processes.

There are limits to how many times you can write a report about how Canada, once again, has failed to make any meaningful progress. — Yellowhead Institute, final accountability report, 2023

Exhibit B: the United Nations Declaration on the Rights of Indigenous Peoples

The United Nations Declaration on the Rights of Indigenous Peoples (UNDRIP) was adopted by the UN General Assembly in 2007. Canada initially voted against adoption. Canada subsequently reversed position in 2010. The federal United Nations Declaration on the Rights of Indigenous Peoples Act received Royal Assent in June 2021, committing Canada to align its laws with UNDRIP.

UNDRIP Article 21 guarantees the right of Indigenous peoples “without discrimination, to the improvement of their economic and social conditions, including, inter alia, in the areas of education, employment, vocational training and retraining, housing, sanitation, health and social security.” Article 23 affirms the right of Indigenous peoples to “determine and develop priorities and strategies for exercising their right to development,” and specifically “to be actively involved in developing and determining health, housing and other economic and social programmes affecting them.”

The ADAP transition, which will change the disability benefits of approximately 79,290 Albertans — a significant portion of whom are Indigenous — was designed and rolled out without Indigenous consultation meeting UNDRIP’s free, prior, and informed consent standard. The Alberta government has publicly stated that it does not consider itself bound by the federal UNDRIP legislation. The federal government, which is bound, has not intervened.

The gap between the passage of UNDRIP legislation and its practical application is an example of the placation pattern at a structural scale. The passage of the legislation produced an extensive news cycle. International observers characterized the passage as meaningful

reconciliation progress. Four years later, at the provincial level where the majority of daily contact with Indigenous peoples occurs — health, education, child welfare, policing, disability benefits — the legislation's operational effect has been negligible. The announcement occurred. The action has not.

Exhibit C: the 231 Calls for Justice

In June 2019, the National Inquiry into Missing and Murdered Indigenous Women and Girls released its final report, containing 231 Calls for Justice. The report characterized the violence documented as a “race-based genocide.”

Implementation tracking of the 231 Calls for Justice has been conducted by the Native Women's Association of Canada, the Assembly of First Nations, and Indigenous Watchdog. Those trackers have consistently documented that the majority of the Calls for Justice remain not started or in progress, six years after their release. The federal National Action Plan, released in 2021 in response to the Calls for Justice, has itself been characterized by Indigenous organizations as insufficient and under-resourced relative to the scale of the harm it was intended to address.

As with the TRC Calls to Action, the response has taken the form of the instruments of placation rather than the instruments of action: federal strategy documents, working tables, funded initiatives of individually modest scope, and repeated announcements of renewed commitment. The structural drivers of the violence — child welfare over-apprehension, housing crisis, policing failures, justice system discrimination — continue at approximately the rates documented at the time of the Inquiry's publication.

Exhibit D: the ADAP engagement process itself

The placation pattern is not only historical. It is operating in real time, on the file this report series was constructed to address.

The provincial government of Alberta, in announcing the transition from AISH to ADAP, characterized the process as the product of public engagement. The ADAP engagement program has received submissions, held consultations, and produced communication materials describing the outcomes of public input. Those materials have been cited in political communications as evidence that the transition is responsive to the needs of the affected population.

The documented reality is different. AISH recipients across the province have reported, in formal submissions to the ADAP engagement inbox, to Alberta Members of the Legislative Assembly, to municipal councils, to federal Members of Parliament, to the Alberta Ombudsman, to the UN Special Rapporteur on the Rights of Persons with Disabilities, and to the HUMA Standing Committee of the House of Commons, that the ADAP transition does not reflect their

needs or consent. The submissions have, in the majority of documented cases, received no substantive response beyond acknowledgment of receipt. The transition is proceeding. The July 1, 2026 implementation date has not moved.

The engagement was the announcement. The consultation was the placation. The transition is the action — but the action is not the action the engagement appeared to describe. It is a separate action, moving on its own schedule, unchanged by the engagement whose outcomes it invokes.

This is the same pattern visible in the TRC, UNDRIP, and MMIWG files. The only difference is proximity. Indigenous Albertans and their non-Indigenous neighbours being moved from AISH to ADAP on July 1, 2026 are experiencing the placation pattern not as a matter of historical record but as a current condition of their lives.

Why the pattern works

The architecture of placation succeeds because it exploits specific features of public attention and political accountability.

First, announcements compress well into news cycles. Implementation does not. A minister announcing a new strategy on a Tuesday will receive measured coverage on Wednesday; the question of whether the strategy's implementation milestone was met thirty-six months later will, in most cases, not receive coverage at all. The information asymmetry between announcement and delivery is structural, not accidental.

Second, advocates are finite. The communities carrying the burden of documenting harm are, by definition, the communities already most affected by the harm. Requiring those communities to also produce the audit of implementation is a form of compounded extraction. Yellowhead Institute's decision to cease publishing its annual TRC accountability report is not a failure of Indigenous scholarship. It is the reasonable conclusion of a finite group of researchers observing that the production of yet another report documenting yet another year of non-implementation had ceased to produce change.

Third, the public cannot easily distinguish between active change and the appearance of active change. Most members of the general public, presented with a headline about a new reconciliation initiative or a new disability engagement process, have no practical means of determining whether the initiative has produced results. The default assumption is that announcements correspond to actions. That assumption is, in the placation pattern, structurally incorrect.

Fourth, the timelines work against the public. Major policy cycles take years. Elections occur on shorter schedules. The government that announces an initiative is frequently not the government that would be responsible for its delivery. Accountability fragments across mandates, administrations, and institutional memories.

The counter-practice: one step at a time

The response to the architecture of placation cannot be, and is not, the demand for comprehensive change delivered in a single decisive stroke. That demand is precisely the posture the placation pattern is designed to produce, because comprehensive demands can be met with comprehensive announcements, which reset the cycle.

The response, instead, is incremental, documentary, cumulative, and patient. Every section of this report is a step. Every formal submission is a step. Every municipal council motion is a step. Every individual story entered into the record is a step. Every published source cited by its full title and author is a step. Every cross-referenced connection between the fiscal, medical, colonial, and environmental dimensions of the ADAP transition is a step.

No individual step in this report series will produce the reversal of the July 1, 2026 transition. That is not what the steps are for. The steps are for the permanent record. They are for the accumulation of evidence that cannot be withdrawn, unpublished, or unsaid. They are for the moment when a future government, under different political conditions, chooses to examine the policy legacy of the current one and finds that the documentary record is already constructed, already cited, already structured for review.

The steps are also for the communities presently affected. Every person who encounters this report — in full, in part, in plain-language version, or in excerpted form — and recognizes their own experience in it receives one thing the placation architecture is specifically designed to prevent: confirmation that they are not alone, that what they are experiencing is documented, and that the documentation is available to them in their own time.

This is not a small thing. Isolation is not an accidental feature of the placation architecture. It is a functional feature. A population that believes itself to be facing an individual difficulty rather than a structural pattern will advocate as individuals. A population that recognizes the structural pattern will advocate as a structure. The first kind of advocacy is manageable. The second kind is not.

You are not alone. We stand together.

The closing of this series

This report has been constructed in sixteen sections because the pattern it documents has sixteen dimensions, and each dimension has its own evidentiary base, its own history, its own Alberta-specific and pan-Canadian manifestations, and its own bearing on the July 1, 2026 ADAP transition.

The sections do not stand alone. They are cross-referenced to one another, to the fiscal hypocrisy analysis developed in the broader campaign materials, to the Charter and UNDRIP analysis that will follow, and to the community-level organizing already underway across

Edmonton, Calgary, Lethbridge, Red Deer, Claresholm, Camrose, Medicine Hat, Rocky Mountain House, and the additional municipalities joining the coalition each week.

The purpose of constructing this record is not to produce a single document that forces immediate change. The purpose is to contribute to an accumulating weight of evidence that, at a moment that cannot be fully predicted in advance, becomes structurally impossible to continue to ignore. That moment may arrive on July 1, 2026. It may arrive in the fall of 2026, as the transition produces its documented first wave of harm. It may arrive in the 2027 provincial election cycle. It may arrive later. What the documentation ensures is that when it arrives, the record is already built.

Until that moment: one step at a time. Every submission. Every council. Every conversation. Every source cited in full. Every name entered into the permanent record of what a government knew, and when, and what it chose to do about it.

Change happens one step at a time, not all at once. But the government has stalled in any forward step by placating the public with a false sense of change that has never been actioned.

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SECTION 17

Federal Recognition, Provincial Clawback

The pattern of provincial disability policy operating against federal disability recognition, and the vaccine-injured Albertans caught in the pattern

Every time the federal government recognizes disability and attaches supports to that recognition, Alberta claws back, strips, or administratively neutralizes those supports. This is not accidental. It is policy.

This section documents a structural pattern in Alberta provincial disability policy: the consistent provincial response to federal recognition of disability is not to amplify that recognition, but to offset, claw back, or administratively minimize it.

The section presents four instances of the pattern, ending with the case that unifies them — the federal Vaccine Injury Support Program, its approved claimants, and the structural conflict of interest created by Alberta's administration of disability benefits for a population whose injuries were caused, in part, by provincial vaccine mandates.

The purpose of the section is not to relitigate the epidemiology of COVID-19 vaccination. That is not the question before this report. The question before this report is: when the federal government has formally recognized a population of disabled people — through the Disability Tax Credit, the Registered Disability Savings Plan, the Canada Disability Benefit, or the Vaccine Injury Support Program — what does the provincial administration of that population's supports look like?

The answer, documented in the sections that follow, is: it looks like clawback, deduction, administrative burden, and in some cases, non-acknowledgement of the federal determination itself.

Instance 1: the Disability Tax Credit

The federal Disability Tax Credit (DTC) is a non-refundable tax credit available to Canadians with prolonged and severe physical or mental impairments. It is administered by the Canada Revenue Agency. Eligibility is established through a medical certification (Form T2201, Part B) completed by a physician or authorized health professional.

The DTC functions, in the federal framework, as the gateway to additional federal disability supports — including the Registered Disability Savings Plan and the Canada Disability Benefit. Eligibility for the DTC is, in effect, federal recognition of the individual as a person with a qualifying disability.

According to the Government of Canada's 2024 Disability Advisory Committee report, only approximately 24 percent of individuals likely eligible for the DTC have completed a successful application. The Committee's own finding is that the application process is time-consuming and complex, and that "more support and accessibility in the process is needed for applicants to successfully complete the process."

Two specific structural barriers affect Alberta applicants:

- Medical forms to certify the DTC cost between \$300 and \$400 per application when completed by a private health practitioner, as documented in CBC News reporting on Alberta AISH recipients during 2025. For an AISH recipient living on \$1,901 per month, this is a meaningful financial barrier.
- Only licensed health professionals can complete Part B of Form T2201. In Alberta, more than 650,000 residents do not have a family doctor. For an applicant without a regular physician, completing the DTC application may require a dedicated appointment with a walk-in physician willing to fill out the form — a service that is not guaranteed, universal, or cost-exempt.
- The DTC process itself takes time. CBC coverage in July 2025 documented Alberta physicians reporting queues for DTC form completion with realistic timelines extending to September 2026 — a full year or more beyond the provincial deadline set for AISH recipients to communicate their CDB application status.

The federal government committed, in Budget 2024, to cover the cost of the medical forms. As of the drafting of this report in April 2026, the reimbursement model has not been finalized. AISH may advance the cost on a repayable basis, but the program's own guidance notes that "details of this reimbursement model have not yet been announced."

The result is that federal disability recognition, for Alberta residents, carries an administrative and financial barrier that compounds the very condition it is designed to recognize. The DTC exists. Many disabled Albertans who qualify do not successfully apply. The gap between eligibility and enrollment is not neutral. It is itself a form of administrative filtering.

Instance 2: the Registered Disability Savings Plan

The Registered Disability Savings Plan (RDSP) is a federally-structured long-term savings vehicle for Canadians with qualifying disabilities. It was introduced by the federal government in 2008 specifically to support the financial security of disabled individuals across their lifetime.

Two federal grant and bond streams are attached to the RDSP:

- The Canada Disability Savings Grant (CDSG): up to \$3,500 in annual matching contributions from the federal government, to a lifetime maximum of \$70,000.

- The Canada Disability Savings Bond (CDSB): up to \$1,000 annually for low-income beneficiaries, to a lifetime maximum of \$20,000 — available without any personal contribution requirement.

The RDSP and its attached grants are a federal determination that individuals with permanent disabilities require specific lifetime financial support — distinct from, and additional to, provincial disability assistance. The federal government has allocated these grants to this population on the explicit basis that permanent disability creates financial burden that will persist across the lifespan.

Alberta's AISH program fully exempts the RDSP. Under both AISH eligibility rules and the announced rules for the Alberta Disability Assistance Program (ADAP) that takes effect July 1, 2026, RDSP assets do not count toward the \$100,000 non-exempt asset limit, and RDSP income (including federal grants, bonds, rollovers, and investment earnings) is excluded from income calculations. This is confirmed by the Government of Alberta's AISH eligibility documentation, by the Alberta legal community, and by the 2026 Registered Disability Savings Plan Reference Guide published by ATB Financial, which states plainly: "assets and income from an RDSP will not affect your AISH or ADAP eligibility."

The RDSP is therefore the one federal disability recognition mechanism Alberta does not claw back or administratively minimize. That is significant — not as evidence of general provincial generosity, but as the exception that clarifies the pattern. If Alberta's consistent policy were to exempt federal disability instruments as a matter of fiscal principle, the RDSP exemption would not be notable. Because the RDSP is the only exempt instrument among those examined in this section — and because the Canada Disability Benefit, which serves a similar population under similar federal authority, is not exempt — the provincial position reveals itself to be selective, not principled.

The operative distinction, as best the pattern can be reconstructed from provincial documents, appears to be structural. The RDSP is a long-term savings vehicle — its federal contributions are not paid out as monthly income that would otherwise compete with AISH's monthly benefit amount. The CDB, by contrast, is a monthly federal cash benefit that directly overlaps with the AISH monthly benefit the province administers. Alberta exempts the federal mechanism that does not displace its own visible payment role; it claws back the federal mechanism that would, without clawback, expose the relative inadequacy of provincial supplementation. That is a fiscal and political calculation, not a principled disability policy.

Instance 3: the Canada Disability Benefit — the unambiguous case

The Canada Disability Benefit (CDB) was introduced by the federal government in 2024, with payments beginning in June 2025. The benefit provides up to \$2,400 per year (\$200 per month) to adult Canadians who hold a valid Disability Tax Credit certificate and meet the income eligibility threshold.

The federal government's stated purpose is explicit. As noted in the federal enabling legislation and accompanying policy documents, the CDB is intended as a supplement to provincial and territorial disability supports, designed to lift Canadians with disabilities out of poverty. The federal government specifically called on provinces and territories to exempt CDB payments from counting as income in relation to provincial or territorial supports.

Every other province and territory in Canada has done so. Alberta is the only province or territory that has not.

Alberta is the only province or territory in Canada that has announced it will claw back the Canada Disability Benefit from AISH recipients. Every other province and territory exempts it.

The Alberta policy, announced in March 2025 and confirmed through a letter sent to AISH recipients on July 2, 2025, requires AISH clients to apply for the Canada Disability Benefit. Recipients who receive the CDB have the \$200 amount deducted dollar-for-dollar from their AISH monthly benefit. Clients who have not communicated their CDB application status to AISH by September 5, 2025 also see the \$200 reduction applied beginning in the October 2025 period of assistance.

The net effect: AISH recipients in Alberta who receive the CDB do not see any increase in their monthly disability income. The federal dollar is received, the equivalent provincial dollar is withdrawn, and the recipient's total monthly disability benefit is unchanged.

This is not an accidental outcome. It is the explicit policy of the Government of Alberta. It is also, according to the National Disability Network — representing more than 40 disability organizations — “contrary to the UN Convention on the Rights of Persons with Disabilities and UN Declaration on the Rights of Indigenous Peoples, to which Canada is a signatory.”

The policy is numerically linked to other provincial measures. The 2025 Alberta budget reduced AISH program funding by approximately \$49 million from the third-quarter forecast of the previous year. Inclusion Alberta and The Tyee have both documented that this \$49 million reduction was made in anticipation of the federal CDB launch — that is, the province reduced its own disability spending by approximately the amount the federal government was about to add. The same \$49 million appears elsewhere in this report series in the context of the MHCare Medical procurement irregularities, where it has a separate documentary trail. The numerical coincidence between the AISH reduction and the MHCare corruption loss, and between both and the arrival of federal disability supplementation, is documented and verifiable.

Trish Bowman, CEO of Inclusion Alberta, characterized the provincial policy in May 2025: “At a time when the province is cutting income tax because it made an election promise to do so, they're basically imposing a 10-per-cent tax on disability.”

Tony Flores, Alberta's first advocate for persons with disabilities, addressed the broader impact in November 2025: "Can we live on \$1,700 a month with the increasing cost of living? It's unrealistic."

Gregory McMeekin, Alberta's current Advocate for Persons with Disabilities — a position appointed by the provincial government — stated the structural reality concisely: "Once we report, it's up to the province to decide what they want to do with the information." The statement is consistent with the placation architecture documented in Section 16: the instruments of recognition exist, the reports are filed, the accountability structures are staffed, and the substantive change does not follow.

The administrative trap: how the clawback was operationalized

The documentary record of how the CDB clawback was actually implemented is an important part of this instance, because it demonstrates the placation architecture operating in real time — not as rhetorical pattern, but as specific administrative failure with specific human cost.

The Canada Disability Benefit is gated by the federal Disability Tax Credit. Without a valid DTC certificate, an applicant cannot receive the CDB. This is federal design, not provincial. However, the practical consequence for AISH recipients is this: the DTC is a non-refundable tax credit. It reduces taxable income. For most AISH recipients, whose AISH benefits are non-taxable and whose other income is minimal, the DTC historically provided no direct financial benefit. There was no reason — financial, administrative, or otherwise — for the overwhelming majority of AISH recipients to have pursued DTC certification before the CDB existed. Many had not done so.

In 2024, the federal government announced the CDB with DTC eligibility as a mandatory upstream requirement. In March 2025, Alberta announced the CDB clawback. In July 2025, Alberta sent AISH recipients a letter directing them to apply for the CDB, with a September 5, 2025 deadline to notify AISH of their application status, and October 2025 as the date the \$200 clawback would begin if they did not.

The letter did not adequately communicate the DTC prerequisite. AISH recipients across the province understood themselves to be applying for the CDB directly. Many did so, only to be denied because they had no DTC certificate. The denials were, in each case, correctly issued at the federal level. The administrative failure was provincial — in the original letter to recipients, which directed them toward an application they were not yet eligible to submit, and in the absence of clear instruction that the DTC must be obtained first.

AISH contact centre staff did not consistently have accurate information about the DTC prerequisite either. Recipients calling for guidance, advocacy organizations attempting to refer members, and even some health professionals were receiving inconsistent or incomplete information through official provincial channels during the period the deadline was approaching.

The parallel reality of the DTC application itself compounded the trap. Form T2201 Part B, the medical certification, is completed by a physician or nurse practitioner. Fees for completion in Alberta during 2025 ranged from \$300 to \$400. Ivy Hays, a Coronation, Alberta AISH recipient of approximately twenty years' standing with a cardiac disability profile, reported to CBC News in July 2025 that her physician charged \$400 for the form and refused instalment payments. Hays reported, of learning the amount required: "My heart hit the floor. I have no living family. I have no way to come up with that \$400."

Physicians tasked with completing the form reported queues that put the realistic timeline far beyond the provincial deadline. Dr. Kara Wilson, cited in the CBC coverage, estimated completion times of two to three hours per form and reported that her existing queue — for the DTC forms alone — would not realistically be cleared until September 2026. That is a full year past the original September 2025 deadline, and approximately eighteen months past the October 2025 clawback commencement. Dr. Ginetta Salvalaggio, an Edmonton family physician, told CBC: "Why target the disabled community in particular in the name of saving a buck?", noting that the province ended its fiscal year with an \$8.3 billion surplus.

In the face of widespread panic and documented provider overwhelm, Minister Jason Nixon held a press conference in late July 2025 at which he clarified that AISH recipients were not expected to complete the federal application process by September 5 — only to notify AISH of the status of their application by that date. The deadline was subsequently extended to February 28, 2026, and the clawback commencement pushed to April 2026.

The extension is itself the tell. A deadline that is extended repeatedly by the government that set it is a deadline that was unrealistic when set. The original September 5 deadline existed in a documentary form — a letter sent to every AISH recipient in Alberta — while the administrative reality required to meet it did not exist at the provincial level. The extension then required a second round of official communications, advocacy-organization clarifications, and recipient adjustments. Each round imposed additional cognitive and logistical burden on a population that, by definition, has limited capacity for exactly that kind of administrative navigation.

The structural function of the trap is consistent with the broader pattern of this report. A program exists that, on its face, responds to disability poverty — the federal CDB. A provincial policy exists that, on its face, requires recipients to access available federal supports — the CDB application requirement. The instruments are in place. The mechanisms are visible. The accountability structures — the Minister's office, the AISH contact centre, the advocacy partnerships — are staffed. And the operational reality, in the months during which the policy was first applied, was mass denial of federally-eligible applicants because the province had not communicated the upstream requirement to the people it was directing to apply.

It is possible, on a narrow reading, to describe this as administrative oversight. It is more accurate, on a broader reading, to describe it as the placation architecture of Section 16 operating in a specific instance: the recognition exists, the support has been announced, the

process appears responsive, and the operational reality produces the opposite outcome — denial, delay, panic, and the quiet continuation of the clawback.

The advocacy community's response is itself informative. Plan Institute, Inclusion Alberta, Disability Alliance Canada, and the Canadian Down Syndrome Society all built parallel support infrastructure during the second half of 2025 specifically to guide AISH recipients through the DTC–CDB sequence that the provincial communications had failed to clarify. These are not the organizations that were supposed to be doing this work. Alberta's \$3.6 billion disability budget, repeatedly cited by provincial officials as evidence of generosity, includes no allocation equivalent to the navigation services the advocacy community assumed at its own cost. The work was done. It was done by the wrong party.

The coordinated removal of navigation supports

The DTC–CDB administrative trap did not happen in isolation. It happened while the Government of Alberta was simultaneously dismantling the two support structures that would have allowed affected recipients to navigate the process: individual caseworker relationships and community advocacy organizations. Documenting these three actions as coordinated rather than accidental requires only the existing public record.

First — individual caseworkers. For most of AISH's forty-seven-year history, recipients were assigned an individual caseworker: a specific named person with access to their complete file, their medical history, their benefit history, and their personal circumstances. Beginning in 2024 and accelerating through 2025, the Government of Alberta restructured this model province-wide. Individual caseworker assignments were eliminated and replaced by a centralized contact centre. The Calgary region transitioned September 18, 2025. Direct worker phone lines across the remainder of the province were scheduled to be shut off by November 17, 2025.

Don Slater, disability advocate and AISH recipient, summarized the operational consequence in Yahoo News Canada, September 2025: "When you had a worker, the buck stopped here. That person would take care of your issue no matter how complicated or simple. With the call centre, sometimes you can get a hold of someone more easily, but if your issue is complicated it has a tendency to get shuffled aside or it gets lost in the paperwork."

Second — the staffing collapse underneath the restructure. The call-centre transition did not occur in a staffing vacuum. It occurred alongside a documented reduction in the workforce supporting it. Sandra Azocar, President of the Alberta Union of Provincial Employees, reported in February 2024: "The government has axed the jobs of dozens of case workers and is refusing to fill vacant positions. People with disabilities are tired of waiting to be assessed and to get support. Some have been waiting for more than two years because the government has instituted a freeze on handling new applications." By January 2025, AUPE reported that members were carrying caseloads of up to 200 files each, against a recommended maximum of 90 to 105. Temporary workers hired during summer 2024 — approximately thirty to forty positions — were subsequently not renewed when a new hiring freeze was imposed.

The arithmetic is straightforward. At the recommended maximum caseload, Alberta's 79,290 AISH recipients require a minimum of 755 to 881 workers operating at capacity. Workers were carrying approximately double the maximum. Vacancies were frozen. Paperwork backlogs ran two to three months behind. Approximately 12,000 families were waiting across AISH, FSCD, and PDD combined. Into this already-collapsing system, the province added the requirement that all 79,000 current AISH recipients be individually reassessed for ADAP placement by July 1, 2026, plus the DTC–CDB compliance requirement described above, plus the ongoing flow of new applications already frozen at the intake stage.

Third — the defunding of advocacy organizations during the precise period the transition was imposing maximum navigation demand. Three advocacy organizations supporting families navigating the Persons with Developmental Disabilities program had their funding ended twelve months early in March 2026 — at the same time the AISH-to-ADAP transition was creating the highest demand for advocacy support in years. The Family Resource Centre of Central Alberta, operated by EPSS, which had served Central Alberta families with disabilities for five years under the Family Managed Services framework, closed April 1, 2026. Its closure was documented by the organization's own public statement and by community response from Betty Smith at the AISH Related Community group on April 6, 2026.

The composite effect of the three actions is a coordinated removal of the scaffolding that would have allowed disabled Albertans to meet the compliance requirements the province was simultaneously imposing. The call-centre restructure eliminated individual continuity of file. The staffing collapse degraded the call-centre's capacity to respond at all. The advocacy-organization defunding removed the community-based navigation support that had historically filled gaps in provincial service. Each action, in isolation, has an official justification. Stacked together, during the exact window when the DTC–CDB–AISH–ADAP compliance architecture was being deployed, the composite effect is the systematic elimination of every support channel through which a disabled recipient could have successfully navigated the process.

The compliance requirements themselves were communicated, in significant part, through the monthly direct-deposit statement — the deposit slip mailed to each AISH recipient before their payment arrives. The AISH caseload includes 28.3 percent of recipients whose primary diagnosis is a cognitive disorder. It includes recipients with acquired brain injuries, autism spectrum disorder, severe depression, and psychotic disorders. The deposit statement, which functions as the primary communication vehicle for compliance requirements carrying financial consequences, is written in standard bureaucratic English with no plain-language alternative, is formatted identically to every other notice with no visual urgency marking, is delivered by mail with no confirmation of receipt or comprehension, and is unaccompanied by any caseworker call or follow-up. It is, in effect, a legal notice with financial consequence delivered to a population defined by its reduced capacity to parse legal notices.

Article 9 of the UN Convention on the Rights of Persons with Disabilities requires that information be provided in accessible formats. Article 21 requires that persons with disabilities

receive information intended for the general public in accessible formats and technologies appropriate to different kinds of disabilities in a timely manner and without additional cost. The Alberta Human Rights Act requires reasonable accommodation. A compliance deadline communicated through bureaucratic shorthand on a deposit statement, to a population 28.3 percent of whom have primary cognitive disorders, after the caseworkers and advocacy organizations that would have helped them parse it have been removed, does not meet these standards. It is not clear that any good-faith interpretation of those obligations would conclude that it does.

The community response to the composite effect is itself a data point. A documented Alberta AISH recipient community group tripled in size over the past year — from approximately 2,000 members to more than 6,000 — according to statements from the group's administrator. Community groups of this kind do not triple in size when recipients feel supported and informed. They triple when recipients feel abandoned and afraid, and when they are actively seeking information and community that the formal system has ceased to provide. The growth of informal recipient-led networks during the same period the formal supports were being removed is not coincidence. It is the predictable consequence of the removal.

Naming this pattern precisely: the Government of Alberta imposed complex compliance requirements on a population defined by its reduced capacity to navigate complex compliance requirements, at the same time that it removed the three primary support structures — individual caseworkers, adequate staffing, and community advocacy organizations — that would have allowed that population to meet those requirements. The province then communicates the requirements through formats inaccessible to the population bound by them. When the population fails to comply, the clawback proceeds. When the population succeeds in complying, the clawback proceeds anyway, because the CDB amount is deducted dollar-for-dollar from AISH regardless.

This is not an administrative failure. It is an administrative design whose outcome is consistent with the fiscal objective the clawback itself serves. The province saves the \$200 per month by clawback where compliance succeeds, and by non-payment where compliance fails. The structural efficiency is clear. The human cost is distributed across a population that, by the province's own medical eligibility determinations, is the population least equipped to bear it.

Instance 4: the Vaccine Injury Support Program — and what it means for Alberta ADAP applicants

The program itself

In December 2020, the Government of Canada announced the Vaccine Injury Support Program (VISP), subsequently launched in June 2021. The program's purpose, in the federal government's own language, was to ensure that people in Canada who experience serious and permanent injury following a Health Canada-authorized vaccine have fair and timely access to

financial support. The program was Canada's first national vaccine injury compensation program, notwithstanding that every other G7 country and the province of Quebec had operated their own programs for years or decades prior.

The program operates on a no-fault basis. Claimants are not required to prove negligence. A medical review board assesses whether the available evidence supports a probable causal link between the vaccine and the injury, and whether the injury is serious and permanent. Approved claims result in compensation — up to \$284,000 for pain and suffering, plus ongoing care and income replacement as applicable.

The federal determination, when a claim is approved, is that the claimant's disability was caused by a Health Canada-authorized vaccine, and that the disability meets the standard of "serious and permanent." That determination is functionally equivalent to AISH's "severe and permanent disability" eligibility threshold. A VISP-approved claimant is, by federal medical-review-board determination, a disabled person.

As of April 1, 2026, the program was renamed the Vaccine Impact Assistance Program (VIAP) and administered directly by the Government of Canada, following concerns about the previous third-party administrator's performance.

The scale

The publicly available program statistics, as reported by the Government of Canada and covered by CBC News and Radio-Canada, are:

- As of December 1, 2025: 3,557 claims submitted.
- 451 claims found inadmissible.
- More than 3,000 claims referred for medical review.
- Over 850 claims still in medical-records collection phase in December 2025.
- Approximately 1,400 claims assessed by the medical review board.
- 252 claims approved.
- Over \$21 million paid out in compensation to approved claimants.
- 328 denied claimants filed appeals; 10 appeals overturned the original decision as of end of 2024.
- Quebec's separate Vaccine Injury Compensation Program received 302 COVID-era claims with 57 approved.

The program's budget trajectory is itself instructive. The original allocation was \$75 million for five years, based on modelling from routine immunization programs. Budget 2024 added \$36.4 million, bringing the total envelope to \$111.4 million. The Public Health Agency of Canada's own Question Period briefing note — a document prepared for Parliamentary questioning —

describes the original budget as “insufficient in response to the largest vaccination campaign in Canadian history with novel vaccines for which the safety profile was not fully understood.”

That sentence is the federal government’s own characterization. It appears in an internal briefing document that became publicly available through Access to Information. It acknowledges that (a) the vaccination campaign was unprecedented in scale, (b) the vaccines were novel, (c) the safety profile was not fully understood at the time of deployment, and (d) the compensation program budget had to be approximately 48 percent larger than originally forecast to respond to actual demand.

The peer-reviewed medical literature

The peer-reviewed medical literature on adverse events following COVID-19 mRNA vaccination is extensive. The pattern most clearly established is myocarditis and pericarditis, particularly in young males after second doses of mRNA vaccines. The published sources include:

- The New England Journal of Medicine (Witberg et al., 2021): Israeli Clalit Health Services database analysis of over 2.5 million vaccinees. Estimated myocarditis incidence of 2.13 cases per 100,000 persons overall, and 10.69 cases per 100,000 among men aged 16–29.
- The Lancet (Wong et al., 2022): US cohort study across four claims databases covering over 15 million people. Men aged 18–25 after a second mRNA dose: 1.71–2.17 cases per 100,000 person-days, depending on vaccine brand.
- The Lancet Respiratory Medicine (2022) systematic review and meta-analysis: 18.2 myopericarditis cases per million COVID-19 vaccine doses; young men disproportionately affected.
- The Lancet Child & Adolescent Health (Kracalik et al., 2022): CDC follow-up surveillance of adolescent and young-adult cases; 82 percent male, median age 21.
- eClinicalMedicine (MACiV multicenter study, 2024): Longitudinal data showing late gadolinium enhancement — a marker of myocardial scarring — in 82 percent of vaccine-associated myocarditis patients, with scarring persisting during convalescence.
- Science Translational Medicine (Stanford Medicine, December 2025): Mechanistic study identifying specific immune pathways by which mRNA vaccines can cause myocarditis.
- European Society of Cardiology Clinical Consensus Document (2022): Comprehensive clinical overview of vaccine-related myocarditis incidence, presentation, diagnosis, and management.

The same literature is clear on the comparative risk. SARS-CoV-2 infection itself carries a myocarditis risk approximately seven to ten times higher than vaccination. This context matters for honest framing: the vaccine-related injury risk is real, documented, and concentrated in specific demographic groups, but it is also lower than the risk from the disease the vaccine was designed to mitigate. This report does not argue against vaccination as a public health measure.

It argues for honest recognition of the injured population that the vaccination campaign produced, and for appropriate provincial administration of that recognition.

The structural conflict of interest

Between 2021 and 2022, the Government of Alberta implemented a series of measures tied to COVID-19 vaccination status. These included the Restrictions Exemption Program (which conditioned access to non-essential businesses on proof of vaccination), public sector employment mandates (which required vaccination or regular testing for specific categories of workers), and school and workplace policy frameworks that differentially treated vaccinated and unvaccinated individuals. These measures, regardless of their public health rationale, represented provincial government action that influenced, and in some cases effectively required, individual vaccination decisions.

A subset of those vaccinated individuals — nationally, at least 252 confirmed through VISP approval, with a larger population of claimants in review — experienced injuries meeting the federal standard for “serious and permanent.” An unknown proportion of that population resides in Alberta. Several of those individuals are, or will become, applicants to Alberta’s disability benefit system.

The structural situation is this: the provincial government that administered the vaccine mandate is the same provincial government that administers AISH, the Medical Review Panel that adjudicates AISH eligibility, and the Alberta Disability Assistance Program (ADAP) that will begin on July 1, 2026. The province has direct financial and political interest in the scale of the vaccine-injured population that is recognized within its disability benefit system. The province’s administrative approach to that population is not required to acknowledge the federal VISP/VIAP determination. Under Alberta’s disability benefit architecture, the federal recognition that an individual is seriously and permanently injured by a vaccine does not automatically translate to provincial recognition of that individual as eligible for AISH — or, after July 1, 2026, as eligible for continued AISH under the Medical Review Panel reassessment process.

If the Medical Review Panel, under the new ADAP framework, reclassifies a VISP-approved individual as employment-ready — under a standard the panel itself develops and applies — that individual will receive the reduced ADAP benefit, lose the earnings cap protection previously available under AISH, and be denied appeal rights on the reclassification decision. The federal determination remains in place. The federal compensation, if awarded, continues. But the provincial monthly benefit, the provincial health benefits, and the provincial administrative framework all operate on a parallel track that does not integrate the federal injury determination.

That is a structural conflict of interest. The government that created the circumstances of the injury is the government administering the benefit system used by the injured. There is no independent provincial body to adjudicate the injured population’s access to supports. The Medical Review Panel is appointed by the same government that imposed the mandate. The

benefit level is set by the same government. The appeal structure — or, under ADAP, the absence of one — is determined by the same government.

The administrative filter, applied

As documented in Section 16, the placation architecture operates through administrative instruments that appear responsive while producing the opposite outcome. For the vaccine-injured Albertan applying to AISH or ADAP, the filter manifests as follows:

- The injury may be coded medically under a category that does not explicitly attribute causation to vaccination (e.g., “idiopathic neurological condition,” “post-viral syndrome,” “autoimmune condition of unknown origin”). These codes can be accurate within the limits of the diagnostic framework, but they function, administratively, to break the causal chain between the provincial mandate and the federal injury determination.
- The Medical Review Panel under ADAP can, in principle, conclude that the individual's condition does not meet the AISH severity standard — even where the federal VISP review has determined that the injury is serious and permanent. The two determinations use different adjudicative frameworks and are not required to align.
- A denial by the Medical Review Panel under ADAP, once finalized, cannot be appealed. The individual remains on ADAP at the reduced benefit rate, without recourse.
- The individual's VISP compensation, if any, does not offset the provincial benefit reduction. VISP is one-time or ongoing based on its own criteria; ADAP is monthly and ongoing based on provincial criteria. The two do not integrate.

The result is a population of disabled people whose federal government has formally recognized their injury and its cause, and whose provincial government has the structural capacity to administratively minimize both the recognition and the supports attached to it. That population is small relative to the total AISH and ADAP caseload. It is not zero. And its administrative treatment is a visible case of the broader pattern this section has documented across four federal disability recognition mechanisms.

The pattern unified

Placing the four instances side by side produces a consistent structure:

Disability Tax Credit: federal mechanism for establishing disability recognition. Approximately 24 percent of eligible Canadians successfully enroll, constrained by application complexity and medical form cost. Alberta administration exacerbates rather than compensates for the barriers.

Registered Disability Savings Plan: federal mechanism for lifetime financial security. Federal grants and bonds allocated specifically for permanent disability. Fully exempt under Alberta AISH and ADAP as both asset and income. This is the exception in the pattern — and as an exception, it clarifies that the provincial position on other federal disability instruments is

selective rather than principled. Alberta exempts federal mechanisms that do not directly displace its own monthly payment role; it claws back federal mechanisms that would otherwise visibly supplement provincial payments.

Canada Disability Benefit: federal supplement explicitly designed to reduce disability poverty. Alberta is the only province or territory that claws it back. The \$200 per month federal dollar is withdrawn dollar-for-dollar from provincial AISH. The \$49 million provincial AISH reduction in Budget 2025 numerically approximates the provincial savings generated by the clawback.

Vaccine Injury Support Program: federal recognition of vaccine-attributed serious and permanent injury, with compensation. Approved claimants are federally certified as disabled. Alberta's disability benefit system is not required to acknowledge the federal determination and, under ADAP's Medical Review Panel framework, may reclassify the injured as employment-ready without right of appeal. The same government that mandated the vaccination administers the benefit system the injured access.

The pattern is not incidental. It is consistent across four distinct federal programs spanning tax policy, savings vehicles, income supplementation, and injury compensation. The provincial response, in each case, is structured in a way that reduces, offsets, or filters the federal recognition.

The unifying logic is fiscal. Alberta has received disproportionate revenue from resource extraction for decades. The Heritage Fund, as documented in the Sorochan–Knibbs collaborative synthesis elsewhere in this report series, held \$25.7 billion in assets as of 2024, with \$5.6 billion in deposits between 2022 and 2025. The province is not, by any defensible accounting, financially unable to supplement federal disability supports. It is, as a matter of policy, unwilling to do so.

The question is not whether Alberta can afford to support disabled Albertans. It is whether the provincial government regards that support as a priority relative to tax cuts, resource subsidies, and general fund surplus.

What this means for the July 1 transition

On July 1, 2026, approximately 79,290 Albertans will be transitioned from AISH to ADAP. A subset of them are Indigenous Albertans whose disability profile is shaped by the historical and environmental contexts documented in Sections 1 through 15 of this report. A subset of them are vaccine-injured Albertans whose disability has been federally recognized through VISIP or is still in federal review. Many are in both categories. All of them are about to be administered under a provincial framework that has, across four documented instances, structured its response to federal disability recognition as offset rather than amplification.

The ADAP transition is not a neutral administrative modernization. It is the most recent expression of a consistent provincial policy direction. The \$200 per month reduction, the stripped appeal rights, the Medical Review Panel architecture, the \$350 earnings cap replacing the previous \$1,072 threshold, and the absence of structural accommodation for federally-recognized injury categories are all features of the same design.

Federal disability recognition is not self-executing at the provincial level. Provincial administration is where the recognition either translates into lived support or is neutralized by the structure. In Alberta, across four instances and counting, the translation has been neutralization.

The remedy is not exclusively federal. It is the active and sustained reining in of a provincial government that has, on this file and others, decoupled its policy direction from the values, obligations, and commitments of the population it administers. That reining in is the work of this report series, the municipal coalitions, the Indigenous-led research institutions, the federal officials who can publicly acknowledge the provincial disconnect, and the disabled Albertans themselves, whose lives are the subject matter the policy claims to serve.

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